

**SUBSTITUTE FOR
SENATE BILL NO. 857**

A bill to make appropriations for the department of health and human services for the fiscal year ending September 30, 2027; and to provide for the expenditure of the appropriations.

THE PEOPLE OF THE STATE OF MICHIGAN ENACT:

PART 1

LINE-ITEM APPROPRIATIONS

Sec. 101. There is appropriated for the department of health and human services for the fiscal year ending September 30, 2027, from the following funds:

DEPARTMENT OF HEALTH AND HUMAN SERVICES

APPROPRIATION SUMMARY

Full-time equated unclassified positions	6.0
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Full-time equated classified positions	15,742.2
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1	Average population	852.0	
2	GROSS APPROPRIATION		\$ 40,546,087,300
3	Interdepartmental grant revenues:		
4	Total interdepartmental grants and		
5	intradepartmental transfers	15,318,800	
6	ADJUSTED GROSS APPROPRIATION		\$ 40,525,768,500
7	Federal revenues:		
8	Capped federal revenues	524,885,300	
9	Social security act, temporary assistance for		
10	needy families	556,702,200	
11	Total other federal revenues	27,210,631,000	
12	Special revenue funds:		
13	Total local revenues	186,827,000	
14	Total private revenues	218,112,100	
15	Michigan merit award trust fund	190,768,700	
16	Total other state restricted revenues	4,303,601,000	
17	State general fund/general purpose		\$ 7,339,241,200
18	Sec. 102. DEPARTMENTAL ADMINISTRATION AND		
19	SUPPORT		
20	Full-time equated unclassified positions	6.0	
21	Full-time equated classified positions	996.5	
22	Unclassified salaries--FTEs	6.0	\$ 1,474,500
23	Administrative hearings officers		9,123,400
24	Child welfare institute--FTEs	59.0	10,278,600
25	Coordinated children's healthcare policy and		
26	supports--FTEs	74.0	22,820,800
27	Demonstration projects--FTEs	7.0	6,779,000



1	Departmental administration and management--		
2	FTEs	613.4	106,252,100
3	Legal services		100,000
4	Office of inspector general--FTEs	219.1	31,979,500
5	Property management		35,894,900
6	Terminal leave payments		6,639,100
7	Training and program support--FTEs	24.0	3,729,000
8	Worker's compensation		8,494,200
9	GROSS APPROPRIATION	\$	243,565,100
10	Appropriated from:		
11	Interdepartmental grant revenues:		
12	IDG from department of lifelong education,		
13	advancement, and potential		1,635,100
14	IDG from department of technology, management,		
15	and budget - office of retirement services		600
16	Federal revenues:		
17	Social security act, temporary assistance for		
18	needy families		17,645,300
19	Capped federal revenues		16,924,400
20	Total other federal revenues		72,719,700
21	Special revenue funds:		
22	Total local revenues		86,000
23	Total private revenues		4,052,300
24	Total other state restricted revenues		16,062,600
25	State general fund/general purpose	\$	114,439,100
26	Sec. 103. CHILD SUPPORT ENFORCEMENT		
27	Full-time equated classified positions	187.0	
28	Child support enforcement operations--FTEs	181.0	\$ 30,774,000



1	Child support incentive payments		24,409,600
2	Legal support contracts		129,023,300
3	State disbursement unit--FTEs	6.0	7,402,200
4	GROSS APPROPRIATION		\$ 191,609,100
5	Appropriated from:		
6	Federal revenues:		
7	Capped federal revenues		16,273,100
8	Total other federal revenues		153,463,700
9	State general fund/general purpose		\$ 21,872,300
10	Sec. 104. COMMUNITY SERVICES AND OUTREACH		
11	Full-time equated classified positions	56.0	
12	Bureau of community services and outreach--FTEs	24.0	\$ 3,691,000
13	Community services and outreach administration--		
14	-FTEs	17.0	6,907,000
15	Community services block grant		37,170,600
16	Diaper assistance grant		6,404,400
17	Homeless programs--FTEs	4.0	34,884,000
18	Housing and support services		13,031,000
19	Kids' food basket		525,000
20	Runaway and homeless youth grants		12,533,800
21	Senior university		400,000
22	Weatherization assistance		22,633,700
23	Weatherization assistance IIJA--FTEs	11.0	40,024,000
24	GROSS APPROPRIATION		\$ 178,204,500
25	Appropriated from:		
26	Federal revenues:		
27	Social security act, temporary assistance for		
28	needy families		29,033,100



1	Capped federal revenues		110,696,500
2	Total other federal revenues		14,050,700
3	HR-1 costs fund		315,400
4	State general fund/general purpose	\$	24,108,800
5	Sec. 105. CHILDREN'S SERVICES AGENCY - CHILD		
6	WELFARE		
7	Full-time equated classified positions	3,813.7	
8	Adoption subsidies	\$	228,000,000
9	Adoption support services--FTEs	10.0	38,935,800
10	Attorney general contract		5,191,100
11	Child abuse and neglect - children's justice		
12	act--FTE	1.0	631,500
13	Child care fund		339,928,700
14	Child care fund - indirect cost allotment		3,500,000
15	Child protection		2,050,300
16	Child welfare administration travel		390,000
17	Child welfare licensing--FTEs	53.0	7,826,600
18	Child welfare local office staff - noncaseload		
19	compliance--FTEs	353.0	43,983,900
20	Child welfare medical/psychiatric evaluations		7,118,300
21	Children's protective services - caseload		
22	staff--FTEs	1,461.0	179,594,900
23	Children's protective services supervisors--		
24	FTEs	387.0	51,197,700
25	Children's services administration--FTEs	206.2	29,873,800
26	Children trust Michigan--FTEs	12.0	5,200,900
27	Contractual services, supplies, and materials		9,852,000
28	Court-appointed special advocates		2,250,000



1	Education planners--FTEs	15.0	2,030,300
2	Family preservation and prevention services		
3	administration--FTEs	9.0	1,466,800
4	Family preservation programs--FTEs	34.0	59,859,300
5	Foster care payments		365,706,300
6	Foster care services - caseload staff--FTEs	838.0	103,282,100
7	Foster care services supervisors--FTEs	227.0	32,955,600
8	Guardianship assistance program		12,800,000
9	Interstate compact		179,600
10	Peer coaches--FTEs	45.5	6,738,700
11	Permanency resource managers--FTEs	28.0	3,743,200
12	Prosecuting attorney contracts		8,142,800
13	Second line supervisors and technical staff--		
14	FTEs	126.0	20,990,800
15	Settlement monitor		2,261,700
16	Strong families/safe children		10,075,400
17	Title IV-E compliance and accountability		
18	office--FTEs	4.0	484,800
19	Youth in transition--FTEs	4.0	7,390,400
20	GROSS APPROPRIATION		\$ 1,593,633,300
21	Appropriated from:		
22	Interdepartmental grant revenues:		
23	IDG from department of lifelong education,		
24	advancement, and potential		244,400
25	Federal revenues:		
26	Social security act, temporary assistance for		
27	needy families		291,416,300
28	Capped federal revenues		104,646,500



1	Total other federal revenues		272,617,800
2	Special revenue funds:		
3	Local funds - county chargeback		48,503,000
4	Private - collections		1,500,000
5	HR-1 costs fund		1,999,200
6	Children's trust fund		2,895,300
7	Total other state restricted revenues		3,600,000
8	State general fund/general purpose	\$	866,210,800
9	Sec. 106. CHILDREN'S SERVICES AGENCY - JUVENILE		
10	JUSTICE		
11	Full-time equated classified positions	189.5	
12	Bay Pines Center--FTEs	53.0	\$ 7,845,000
13	Committee on juvenile justice administration--		
14	FTEs	2.5	376,600
15	Committee on juvenile justice grants		3,000,000
16	Community support services--FTEs	2.0	1,856,800
17	County juvenile officers		3,977,600
18	Juvenile justice, administration and		
19	maintenance--FTEs	21.0	5,605,500
20	Michigan youth treatment center--FTEs	111.0	16,567,300
21	GROSS APPROPRIATION	\$	39,228,800
22	Appropriated from:		
23	Federal revenues:		
24	Capped federal revenues		7,754,800
25	Total other federal revenues		268,200
26	Special revenue funds:		
27	Local funds - state share education funds		1,527,500
28	Local funds - county chargeback		10,493,300



1	State general fund/general purpose	\$	19,185,000
2	Sec. 107. PUBLIC ASSISTANCE		
3	Full-time equated classified positions	1.0	
4	Family independence program		82,485,000
5	Family independence program - clothing		
6	allowance		10,000,000
7	Family independence program - child		
8	supplemental payment		23,240,100
9	Food assistance program benefits		3,446,187,200
10	Indigent burial		2,384,700
11	Low-income home energy assistance program		174,951,600
12	Michigan energy assistance program--FTE	1.0	100,000,000
13	Prenatal and infant support program		20,000,000
14	Refugee assistance program		7,954,200
15	State disability assistance payments		4,543,800
16	State supplementation		58,085,000
17	GROSS APPROPRIATION	\$	3,929,831,600
18	Appropriated from:		
19	Federal revenues:		
20	Social security act, temporary assistance for		
21	needy families		107,019,200
22	Capped federal revenues		182,905,800
23	Total other federal revenues		3,441,477,200
24	Special revenue funds:		
25	Child support collections		9,908,100
26	Low-income energy assistance fund		100,000,000
27	Public assistance recoupment revenue		4,811,300
28	Supplemental security income recoveries		1,797,500



1	State general fund/general purpose	\$	81,912,500
2	Sec. 108. LOCAL OFFICE OPERATIONS AND SUPPORT		
3	SERVICES		
4	Full-time equated classified positions	5,959.2	
5	Administrative support workers--FTEs	127.0	\$ 15,415,500
6	Adult services local office staff--FTEs	550.0	71,524,000
7	Contractual services, supplies, and materials		33,567,000
8	Donated funds positions--FTEs	159.0	28,001,600
9	Elder Law of Michigan MiCAFE contract		450,000
10	Electronic benefit transfer (EBT)		7,198,000
11	Employment and training support services		3,869,100
12	Food assistance reinvestment--FTEs	16.0	3,841,100
13	Local office policy and administration--FTEs	122.0	22,319,600
14	Medical/psychiatric evaluations		166,200
15	Public assistance local office staff--FTEs	4,985.2	596,101,300
16	SSI advocacy legal services grant		975,000
17	GROSS APPROPRIATION	\$	783,428,400
18	Appropriated from:		
19	Interdepartmental grant revenues:		
20	IDG from department of corrections		120,200
21	IDG from department of lifelong education,		
22	advancement, and potential		8,303,900
23	Federal revenues:		
24	Social security act, temporary assistance for		
25	needy families		66,586,000
26	Capped federal revenues		54,067,800
27	Total other federal revenues		214,841,600
28	Special revenue funds:		



1	Local funds - donated funds		4,473,100
2	Private funds - donated funds		10,262,500
3	Private revenues		250,000
4	HR-1 costs fund		98,792,600
5	State general fund/general purpose	\$	325,730,700
6	Sec. 109. DISABILITY DETERMINATION SERVICES		
7	Full-time equated classified positions	628.4	
8	Disability determination operations--FTEs	624.3	\$ 126,213,900
9	Retirement disability determination--FTEs	4.1	662,000
10	GROSS APPROPRIATION	\$	126,875,900
11	Appropriated from:		
12	Interdepartmental grant revenues:		
13	IDG from department of technology, management,		
14	and budget - office of retirement services		838,600
15	Federal revenues:		
16	Total other federal revenues		123,235,300
17	State general fund/general purpose	\$	2,802,000
18	Sec. 110. BEHAVIORAL HEALTH PROGRAM		
19	ADMINISTRATION AND SPECIAL PROJECTS		
20	Full-time equated classified positions	98.6	
21	Behavioral health program administration--FTEs	59.0	\$ 50,171,200
22	Community substance use disorder prevention,		
23	education, and treatment--FTEs	6.6	78,186,700
24	Family support subsidy		17,173,100
25	Federal and other special projects		2,535,600
26	Gambling addiction--FTEs	4.0	9,534,000
27	Mental health diversion council		3,850,000
28	Office of recipient rights--FTEs	25.0	3,629,500



1	Opioid response activities--FTEs	4.0	122,165,100
2	Protection and advocacy services support		194,400
3	GROSS APPROPRIATION	\$	287,439,600
4	Appropriated from:		
5	Federal revenues:		
6	Social security act, temporary assistance for		
7	needy families		17,173,100
8	Total other federal revenues		160,072,500
9	Special revenue funds:		
10	Total private revenues		2,704,700
11	Total other state restricted revenues		67,389,500
12	State general fund/general purpose	\$	40,099,800
13	Sec. 111. BEHAVIORAL HEALTH SERVICES		
14	Full-time equated classified positions	16.0	
15	Autism services	\$	560,716,600
16	Behavioral health community supports and		
17	services--FTEs	12.0	45,628,700
18	Certified community behavioral health clinic		
19	demonstration		916,062,700
20	Civil service charges		297,500
21	Community mental health non-Medicaid services		125,578,200
22	Federal mental health block grant--FTEs	4.0	27,491,100
23	Health homes		50,239,800
24	Healthy Michigan plan - behavioral health		518,153,900
25	Medicaid mental health services		3,663,869,500
26	Medicaid substance use disorder services		84,902,600
27	Multicultural integration funding		17,284,900
28	Nursing home PAS/ARR-OBRA		18,813,600

1	State disability assistance program substance		
2	use disorder services		2,018,800
3	GROSS APPROPRIATION	\$	6,031,057,900
4	Appropriated from:		
5	Federal revenues:		
6	Social security act, temporary assistance for		
7	needy families		421,000
8	Capped federal revenues		184,500
9	Total other federal revenues		4,119,475,400
10	Special revenue funds:		
11	Total local revenues		9,943,600
12	Total other state restricted revenues		142,088,900
13	State general fund/general purpose	\$	1,758,944,500
14	Sec. 112. STATE PSYCHIATRIC HOSPITALS AND		
15	FORENSIC MENTAL HEALTH SERVICES		
16	Full-time equated classified positions	2,320.6	
17	Average population	852.0	
18	Caro Regional Mental Health Center -		
19	psychiatric hospital - adult--FTEs	432.7	\$ 71,981,700
20	Average population	145.0	
21	Center for forensic psychiatry--FTEs	624.5	129,037,800
22	Average population	240.0	
23	Developmental disabilities council and		
24	projects--FTEs	10.0	3,244,200
25	Gifts and bequests for patient living and		
26	treatment environment		1,000,000
27	IDEA, federal special education		120,000
28	Kalamazoo Psychiatric Hospital - adult--FTEs	473.2	74,708,100



1	Average population	170.0	
2	Purchase of medical services for residents of		
3	hospitals and centers		445,600
4	Revenue recapture		750,100
5	Southeast Michigan state psychiatric hospital--		
6	FTEs	746.2	136,384,200
7	Average population	297.0	
8	Special maintenance		924,600
9	State hospital administration--FTEs	34.0	5,879,400
10	GROSS APPROPRIATION		\$ 424,475,700
11	Appropriated from:		
12	Federal revenues:		
13	Total other federal revenues		38,993,800
14	Special revenue funds:		
15	Total local revenues		24,110,800
16	Total private revenues		1,000,000
17	Total other state restricted revenues		19,189,200
18	State general fund/general purpose		\$ 341,181,900
19	Sec. 113. HEALTH AND HUMAN SERVICES POLICY AND		
20	INITIATIVES		
21	Full-time equated classified positions	77.3	
22	Certificate of need program administration--		
23	FTEs	11.3	\$ 2,783,000
24	Child advocacy centers		1,407,000
25	Child advocacy centers - supplemental grants		2,000,000
26	Crime victim grants administration services--		
27	FTEs	15.0	3,161,700
28	Crime victim justice assistance grants		78,579,300



1	Crime victim rights services grants		19,869,900
2	Crime victim rights sustaining grants--FTEs	2.0	30,000,000
3	Domestic violence prevention and treatment--		
4	FTEs	15.6	20,507,300
5	Human trafficking intervention services--FTE	1.0	200,000
6	Michigan essential health provider		3,519,600
7	Minority health grants and contracts--FTEs	3.0	1,169,300
8	Nurse education and research program--FTEs	3.0	828,300
9	Policy and planning administration--FTEs	19.9	2,836,900
10	Primary care services--FTEs	3.0	3,813,300
11	Rape prevention and services--FTEs	0.5	7,216,600
12	Rural health services		175,000
13	Rural health transformation program--FTEs	3.0	250,000,000
14	Uniform statewide sexual assault evidence kit		
15	tracking system		369,500
16	GROSS APPROPRIATION	\$	428,436,700
17	Appropriated from:		
18	Interdepartmental grant revenues:		
19	IDG from department of licensing and regulatory		
20	affairs		828,300
21	IDG from department of lifelong education,		
22	advancement, and potential		2,400
23	IDG from department of treasury, Michigan		
24	finance authority		117,700
25	Federal revenues:		
26	Social security act, temporary assistance for		
27	needy families		6,575,900
28	Capped federal revenues		11,912,900



1	Total other federal revenues		336,240,500
2	Special revenue funds:		
3	Total private revenues		855,000
4	Child advocacy centers fund		1,407,000
5	Compulsive gaming prevention fund		1,040,500
6	Crime victims rights fund		18,798,200
7	Sexual assault victims' prevention and		
8	treatment fund		3,000,000
9	Total other state restricted revenues		3,444,300
10	State general fund/general purpose	\$	44,214,000
11	Sec. 114. EPIDEMIOLOGY, EMERGENCY MEDICAL		
12	SERVICES, AND LABORATORY		
13	Full-time equated classified positions	448.9	
14	Bioterrorism preparedness--FTEs	53.0	\$ 31,265,300
15	Childhood lead program--FTEs	4.5	2,358,300
16	Emergency medical services program--FTEs	27.0	7,276,700
17	Epidemiology administration--FTEs	73.5	27,115,500
18	Healthy homes program--FTEs	62.0	51,885,600
19	Laboratory services--FTEs	102.0	32,065,500
20	Newborn screening follow-up and treatment		
21	services--FTEs	10.5	10,370,600
22	PFAS and environmental contamination response--		
23	FTEs	46.0	18,583,900
24	Vital records and health statistics--FTEs	70.4	11,886,400
25	GROSS APPROPRIATION	\$	192,807,800
26	Appropriated from:		
27	Interdepartmental grant revenues:		

1	IDG from department of environment, Great		
2	Lakes, and energy		2,525,000
3	Federal revenues:		
4	Total other federal revenues		79,336,700
5	Special revenue funds:		
6	Total private revenues		1,342,600
7	Total other state restricted revenues		34,295,700
8	State general fund/general purpose	\$	75,307,800
9	Sec. 115. LOCAL HEALTH AND ADMINISTRATIVE		
10	SERVICES		
11	Full-time equated classified positions	197.6	
12	AIDS prevention, testing, and care programs--		
13	FTEs	79.0	\$ 111,041,200
14	Cancer prevention and control program--FTEs	18.0	15,915,500
15	Chronic disease control and health promotion		
16	administration--FTEs	28.4	10,570,700
17	Community health programs--FTEs	5.0	16,273,300
18	Diabetes and kidney program--FTEs	8.0	4,233,100
19	Essential local public health services		81,419,300
20	Local health services--FTEs	4.3	9,610,400
21	Medicaid outreach cost reimbursement to local		
22	health departments		12,500,000
23	Public health administration--FTEs	8.0	2,196,900
24	Sexually transmitted disease control program--		
25	FTEs	20.0	8,655,500
26	Smoking prevention program--FTEs	15.0	6,495,700
27	Violence prevention--FTEs	11.9	17,366,000
28	GROSS APPROPRIATION	\$	296,277,600

1	Appropriated from:		
2	Federal revenues:		
3	Social security act, temporary assistance for		
4	needy families		2,300
5	Total other federal revenues		91,667,800
6	Special revenue funds:		
7	Total local revenues		10,150,000
8	Total private revenues		74,556,600
9	Total other state restricted revenues		15,292,600
10	State general fund/general purpose	\$	104,608,300
11	Sec. 116. FAMILY HEALTH SERVICES		
12	Full-time equated classified positions	135.9	
13	Child and adolescent health care and centers	\$	41,242,700
14	Dental programs--FTEs	3.6	7,028,700
15	Drinking water declaration of emergency		3,971,000
16	Family, maternal, and child health		
17	administration--FTEs	50.0	10,683,200
18	Family planning local agreements		15,810,700
19	Immunization program--FTEs	20.8	19,229,300
20	Local MCH services		7,018,100
21	Pregnancy prevention program		1,297,900
22	Prenatal care outreach and service delivery		
23	support--FTEs	19.5	45,944,400
24	Special projects		6,789,100
25	Sudden and unexpected infant death and		
26	suffocation prevention program		321,300
27	Women, infants, and children program		
28	administration and special projects--FTEs	42.0	19,880,200

1	Women, infants, and children program local		
2	agreements and food costs		251,285,000
3	GROSS APPROPRIATION	\$	430,501,600
4	Appropriated from:		
5	Federal revenues:		
6	Social security act, temporary assistance for		
7	needy families		500,000
8	Total other federal revenues		272,269,600
9	Special revenue funds:		
10	Total local revenues		42,817,700
11	Total private revenues		64,785,700
12	Total other state restricted revenues		3,270,500
13	State general fund/general purpose	\$	46,858,100
14	Sec. 117. CHILDREN'S SPECIAL HEALTH CARE		
15	SERVICES		
16	Full-time equated classified positions	51.8	
17	Bequests for care and services--FTEs	9.8	\$ 2,415,300
18	Children's special health care services		
19	administration--FTEs	42.0	9,377,900
20	Medical care and treatment		443,164,400
21	Outreach and advocacy		6,722,200
22	GROSS APPROPRIATION	\$	461,679,800
23	Appropriated from:		
24	Federal revenues:		
25	Total other federal revenues		252,754,900
26	Special revenue funds:		
27	Total private revenues		1,037,200
28	Total other state restricted revenues		4,582,400

1	State general fund/general purpose	\$	203,305,300
2	Sec. 118. AGING SERVICES		
3	Community services	\$	58,901,900
4	Employment assistance		3,500,000
5	Nutrition services		50,004,200
6	Respite care program		7,268,700
7	Senior volunteer service programs		4,765,300
8	GROSS APPROPRIATION	\$	124,440,100
9	Appropriated from:		
10	Federal revenues:		
11	Total other federal revenues		67,787,400
12	Special revenue funds:		
13	Total private revenues		300,000
14	Michigan merit award trust fund		4,068,700
15	Total other state restricted revenues		2,800,000
16	State general fund/general purpose	\$	49,484,000
17	Sec. 119. HEALTH AND AGING SERVICES		
18	ADMINISTRATION		
19	Full-time equated classified positions	561.2	
20	Aging services administration--FTEs	43.0	\$ 9,819,200
21	Health services administration--FTEs	518.2	146,649,500
22	GROSS APPROPRIATION	\$	156,468,700
23	Appropriated from:		
24	Federal revenues:		
25	Total other federal revenues		100,050,600
26	Special revenue funds:		
27	Total local revenues		187,700
28	Total private revenues		1,721,300

1	Total other state restricted revenues	3,994,400
2	State general fund/general purpose	\$ 50,514,700
3	Sec. 120. HEALTH SERVICES	
4	Adult home help services	\$ 800,184,800
5	Ambulance services	25,335,200
6	Auxiliary medical services	5,126,300
7	Dental clinic program	1,000,000
8	Dental services	336,287,500
9	Federal Medicare pharmaceutical program	410,467,600
10	Federally qualified health centers	157,182,600
11	Health plan services	8,609,060,500
12	Healthy Michigan plan	6,195,577,200
13	Home health services	18,690,200
14	Hospice services	258,260,400
15	Hospital services and therapy	688,595,200
16	Integrated care organizations	676,213,300
17	Long-term care services	2,586,950,000
18	Maternal and child health	37,267,500
19	Medicaid home- and community-based services	
20	waiver	603,442,900
21	Medicare premium payments	1,049,501,000
22	Personal care services	5,035,500
23	Pharmaceutical services	405,935,200
24	Physician services	138,041,600
25	Program of all-inclusive care for the elderly	326,992,100
26	School-based services	196,691,300
27	Special Medicaid reimbursement	370,654,700
28	Transportation	26,057,200



1	GROSS APPROPRIATION	\$ 23,928,549,800
2	Appropriated from:	
3	Federal revenues:	
4	Total other federal revenues	17,109,327,300
5	Special revenue funds:	
6	Total local revenues	34,530,400
7	Total private revenues	28,551,500
8	Michigan merit award trust fund	186,700,000
9	Total other state restricted revenues	3,626,823,100
10	State general fund/general purpose	\$ 2,942,617,500
11	Sec. 121. INFORMATION TECHNOLOGY	
12	Bridges information system	\$ 121,783,300
13	Child support automation	45,186,700
14	Comprehensive child welfare information system	8,761,900
15	Information technology services and projects	173,071,200
16	Michigan Medicaid information system	104,265,300
17	GROSS APPROPRIATION	\$ 453,068,400
18	Appropriated from:	
19	Interdepartmental grant revenues:	
20	IDG from department of lifelong education,	
21	advancement, and potential	609,700
22	Federal revenues:	
23	Social security act, temporary assistance for	
24	needy families	12,295,000
25	Capped federal revenues	17,359,400
26	Total other federal revenues	282,148,900
27	Special revenue funds:	
28	Total local revenues	3,900



1	Total private revenues	5,187,500
2	Total other state restricted revenues	14,573,000
3	State general fund/general purpose	\$ 120,891,000
4	Sec. 122. ONE-TIME APPROPRIATIONS	
5	Full-time equated classified positions	3.0
6	Competitive community health infrastructure	
7	grants	\$ 14,000,000
8	Emergency services local office allocations	7,553,900
9	Health and human services grants	12,000,000
10	Health services administration--FTEs	3.0 4,000,000
11	Housing and community development grants	3,000,000
12	Local office staff travel	8,327,400
13	Michigan agricultural surplus system	12,045,000
14	Michigan clinical consultation and care	5,789,000
15	Nursing facility staffing initiative	10,000,000
16	Opioid response activities	75,500,000
17	Permanent supportive housing	5,000,000
18	Property management	8,718,700
19	State hospital administration	3,600,000
20	State psychiatric DSH disallowance	54,722,900
21	Uterine fibroid study	250,000
22	Water affordability	10,000,000
23	Weatherization assistance	10,000,000
24	GROSS APPROPRIATION	\$ 244,506,900
25	Appropriated from:	
26	IDG from department of lifelong education,	
27	advancement, and potential	92,900
28	Federal revenues:	

1	Social security act, temporary assistance for	
2	needy families	8,035,000
3	Capped federal revenues	2,159,600
4	Total other federal revenues	7,831,400
5	Special revenue funds:	
6	Total private revenues	20,005,200
7	Certificate of need fees	2,822,000
8	Child care home and center licenses fund	2,499,700
9	Fees and collections	10,000,000
10	HR-1 costs fund	39,200
11	Laboratory services fees	500,000
12	Michigan opioid healing and recovery fund	75,500,000
13	Newborn screening fees	10,000,000
14	Total other state restricted revenues	68,800
15	State general fund/general purpose	\$ 104,953,100

PART 2

PROVISIONS CONCERNING APPROPRIATIONS

FOR FISCAL YEAR 2026-2027

GENERAL SECTIONS

Sec. 201. In accordance with section 30 of article IX of the state constitution of 1963, for the current fiscal year, total state spending under part 1 from state sources is \$11,833,610,900.00 and state spending under part 1 from state sources to be paid to local units of government is \$2,397,961,800.00. The following itemized statement identifies appropriations from which spending to local units of government will occur:



1	DEPARTMENT OF HEALTH AND HUMAN SERVICES	
2	CHILD SUPPORT ENFORCEMENT	
3	Child support incentive payments	\$ 9,754,900
4	Legal support contracts	1,400
5	COMMUNITY SERVICES AND OUTREACH	
6	Community services and outreach administration	1,000
7	Homeless programs	10,000
8	Housing and support services	138,500
9	CHILDREN'S SERVICES AGENCY - CHILD WELFARE	
10	Child care fund	208,675,600
11	Child care fund - indirect cost allotment	3,500,000
12	Child welfare licensing	26,500
13	Child welfare medical/psychiatric evaluations	15,100
14	Children trust Michigan	35,100
15	Contractual services, supplies, and materials	50,700
16	Family preservation programs	100
17	Foster care payments	2,117,400
18	Prosecuting attorney contracts	1,235,600
19	Strong families/safe children	35,000
20	Youth-in-transition	700
21	CHILDREN'S SERVICES AGENCY - JUVENILE JUSTICE	
22	Bay Pines Center	40,700
23	Community support services	118,400
24	County juvenile officers	67,900
25	PUBLIC ASSISTANCE	
26	Indigent burial	2,800
27	Michigan energy assistance program	439,300
28	State disability assistance payments	256,300

1	LOCAL OFFICE OPERATIONS AND SUPPORT SERVICES	
2	Contractual services, supplies, and materials	82,800
3	Employment and training support services	5,400
4	DISABILITY DETERMINATION SERVICES	
5	Disability determination operations	3,300
6	BEHAVIORAL HEALTH PROGRAM ADMINISTRATION AND	
7	SPECIAL PROJECTS	
8	Behavioral health program administration	42,600
9	Community substance use disorder prevention,	
10	education, and treatment	8,688,500
11	Gambling addiction	1,065,000
12	Mental health diversion council	113,900
13	Opioid response activities	927,800
14	BEHAVIORAL HEALTH SERVICES	
15	Autism services	189,487,300
16	Behavioral health community supports and	
17	services	407,300
18	Certified community behavioral health clinic	
19	demonstration	183,500,000
20	Community mental health non-Medicaid services	125,578,200
21	Health homes	3,163,900
22	Healthy Michigan plan - behavioral health	48,553,300
23	Medicaid mental health services	1,264,786,600
24	Medicaid substance use disorder services	29,231,400
25	Multicultural integration funding	519,000
26	Nursing home PAS/ARR-OBRA	3,834,900
27	State disability assistance program substance	
28	use disorder services	1,539,000

1	STATE PSYCHIATRIC HOSPITALS AND FORENSIC MENTAL	
2	HEALTH SERVICES	
3	Caro Regional Mental Health Center -	
4	psychiatric hospital - adult	68,500
5	Center for forensic psychiatry	696,600
6	Kalamazoo Psychiatric Hospital - adult	84,200
7	Southeast Michigan state psychiatric hospital	21,600
8	HEALTH AND HUMAN SERVICES POLICY AND	
9	INITIATIVES	
10	Crime victim rights services grants	4,454,800
11	Crime victims rights sustaining grants	7,343,700
12	Domestic violence prevention and treatment	72,200
13	Primary care services	10,300
14	EPIDEMIOLOGY, EMERGENCY MEDICAL SERVICES, AND	
15	LABORATORY	
16	Emergency medical services program	6,700
17	Epidemiology administration	521,000
18	Healthy homes program	1,233,400
19	Laboratory services	94,600
20	PFAS and environmental contamination response	500
21	LOCAL HEALTH AND ADMINISTRATIVE SERVICES	
22	AIDS prevention, testing, and care programs	2,663,100
23	Cancer prevention and control program	15,000
24	Essential local public health services	69,050,700
25	Local health services	1,356,800
26	Public health administration	200
27	Sexually transmitted disease control program	720,400
28	Smoking prevention program	242,900

1	FAMILY HEALTH SERVICES	
2	Dental programs	2,000,000
3	Drinking water declaration of emergency	70,700
4	Family planning local agreements	4,165,000
5	Immunization program	1,000,000
6	Pregnancy prevention program	65,000
7	Prenatal care outreach and service delivery	
8	support	7,464,100
9	CHILDREN'S SPECIAL HEALTH CARE SERVICES	
10	Medical care and treatment	943,000
11	Outreach and advocacy	3,176,000
12	AGING SERVICES	
13	Community services	32,428,100
14	Nutrition services	14,513,000
15	Respite care program	5,664,000
16	Senior volunteer service programs	1,217,000
17	HEALTH AND AGING SERVICES ADMINISTRATION	
18	Aging services administration	123,800
19	Health services administration	303,000
20	HEALTH SERVICES	
21	Adult home help services	86,000
22	Ambulance services	1,216,000
23	Dental services	717,400
24	Federally qualified health centers	800,100
25	Healthy Michigan plan	907,500
26	Hospital services and therapy	613,000
27	Long-term care services	124,067,800



1	Medicaid home- and community-based services	
2	waiver	15,862,900
3	Personal care services	24,000
4	Physician services	1,378,000
5	Transportation	401,000
6	ONE-TIME APPROPRIATIONS	
7	Emergency services local office allocations	2,045,000
8	TOTAL OF PAYMENTS TO LOCAL UNITS OF GOVERNMENT	\$ 2,397,961,800

9 Sec. 202. The appropriations under this part and part 1 are
10 subject to the management and budget act, 1984 PA 431, MCL 18.1101
11 to 18.1594.

12 Sec. 203. As used in this part and part 1:

13 (a) "AIDS" means acquired immunodeficiency syndrome.

14 (b) "CCBHC" means certified community behavioral health
15 clinic.

16 (c) "CMHSP" means a community mental health services program
17 as that term is defined in section 100a of the mental health code,
18 1974 PA 258, MCL 330.1100a.

19 (d) "CMS" means the Centers for Medicare and Medicaid
20 Services.

21 (e) "CPT" means current procedural terminology.

22 (f) "Current fiscal year" means the fiscal year ending
23 September 30, 2027.

24 (g) "Department" means the department of health and human
25 services.

26 (h) "Director" means the director of the department.

27 (i) "EPSDT" means early and periodic screening, diagnosis, and
28 treatment.

29 (j) "Federal poverty level" means the poverty guidelines



revised periodically and published in the Federal Register by the Secretary of the United States Department of Health and Human Services under the Secretary's authority to revise the poverty line under 42 USC 9902.

(k) "FQHC" means federally qualified health center.

(l) "FTE" means full-time equated.

(m) "GME" means graduate medical education.

(n) "Health plan" means, at a minimum, an organization that meets the criteria for delivering the comprehensive package of services under the department's comprehensive health plan.

(o) "HEDIS" means health care effectiveness data and information set.

(p) "HMO" means health maintenance organization.

(q) "IDEA" means the individuals with disabilities education act, 20 USC 1400 to 1482.

(r) "IDG" means interdepartmental grant.

(s) "MCH" means maternal and child health.

(t) "Medicaid" means benefits under the medical assistance program established under title XIX of the social security act, 42 USC 1396 to 1396w-8, and administered by the department under the social welfare act, 1939 PA 280, MCL 400.1 to 400.119b.

(u) "Medicare" means benefits under the federal Medicare program established under title XVIII of the social security act, 42 USC 1395 to 1395mmm.

(v) "MiCAFE" means Michigan's coordinated access to food for the elderly.

(w) "MiChild" means the program described in section 1670 of this part.

(x) "MiSACWIS" means Michigan statewide automated child



1 welfare information system.

2 (y) "PACE" means program of all-inclusive care for the
3 elderly.

4 (z) "PAS/ARR-OBRA" means the preadmission screening and annual
5 resident review required under the omnibus budget reconciliation
6 act of 1987, section 1919(e)(7) of the social security act, 42 USC
7 1396r.

8 (aa) "PATH" means Partnership. Accountability. Training. Hope.

9 (bb) "PFAS" means perfluoroalkyl and polyfluoroalkyl
10 substances.

11 (cc) "PIHP" means an entity designated by the department as a
12 regional entity or a specialty prepaid inpatient health plan for
13 Medicaid mental health services, services to individuals with
14 developmental disabilities, and substance use disorder services.
15 Regional entities are described in section 204b of the mental
16 health code, 1974 PA 258, MCL 330.1204b. Specialty prepaid
17 inpatient health plans are described in section 109f of the social
18 welfare act, 1939 PA 280, MCL 400.109f.

19 (dd) "Previous fiscal year" means the fiscal year ending
20 September 30, 2026.

21 (ee) "Quarterly basis" means February 1, April 1, July 1, and
22 September 30 of the current fiscal year.

23 (ff) "Semiannual basis" means March 1 and September 30 of the
24 current fiscal year.

25 (gg) "Settlement" means the settlement agreement entered in
26 the case of *Dwayne B. v Snyder*, Docket No. 2:06-cv-13548 in the
27 United States District Court for the Eastern District of Michigan.

28 (hh) "SSI" means supplemental security income.

29 (ii) "Standard report recipients" means the senate and house



1 of representatives appropriations subcommittees on the department
2 budget, the senate and house fiscal agencies, the senate and house
3 of representatives policy offices, and the state budget office.

4 (jj) "Temporary assistance for needy families" or "TANF" or
5 "title IV-A" means part A of title IV of the social security act,
6 42 USC 601 to 619.

7 (kk) "Title IV-B" means part B of title IV of the social
8 security act, 42 USC 621 to 629m.

9 (ll) "Title IV-D" means part D of title IV of the social
10 security act, 42 USC 651 to 669b.

11 (mm) "Title IV-E" means part E of title IV of the social
12 security act, 42 USC 670 to 679c.

13 (nn) "Title X" means subchapter VIII of the public health
14 service act, 42 USC 300 to 300a-8, which establishes grants to
15 states for family planning services.

16 Sec. 204. A department or agency shall use the internet to
17 fulfill the reporting requirements of this part, make each report
18 readily accessible to the public, and conspicuously post each
19 required report in a single archivable location on the department's
20 or agency's Michigan.gov website not later than the due date
21 required for each report. In addition to placing all reports
22 required in the current fiscal year on the department's or agency's
23 website, the department or agency shall maintain on its website all
24 reports placed on the website from previous fiscal years posted by
25 fiscal year in the same single archivable location. The department
26 or agency shall also transmit all required reports for the current
27 fiscal year to the standard report recipients and any other
28 required recipients by email.

29 Sec. 205. To the extent permissible under section 261 of the



1 management and budget act, 1984 PA 431, MCL 18.1261, all of the
2 following apply to the expenditure of funds appropriated in part 1:

3 (a) The funds must not be used for the purchase of foreign
4 goods or services, or both, if competitively priced and of
5 comparable quality American goods or services, or both, are
6 available.

7 (b) Preference must be given to goods or services, or both,
8 manufactured or provided by Michigan businesses, if they are
9 competitively priced and of comparable quality.

10 (c) Preference must be given to goods or services, or both,
11 that are manufactured or provided by Michigan businesses owned and
12 operated by veterans, if they are competitively priced and of
13 comparable quality.

14 Sec. 206. To the extent permissible under the management and
15 budget act, 1984 PA 431, MCL 18.1101 to 18.1594, the director of
16 each department or agency receiving appropriations in part 1 shall
17 take all reasonable steps to ensure geographically disadvantaged
18 business enterprises compete for and perform contracts to provide
19 services or supplies, or both. The director shall strongly
20 encourage firms with which the department or agency contracts to
21 subcontract with certified geographically disadvantaged business
22 enterprises for services, supplies, or both. As used in this
23 section, "geographically disadvantaged business enterprises" means
24 that term as defined in Executive Directive No. 2023-1.

25 Sec. 207. Consistent with section 217 of the management and
26 budget act, 1984 PA 431, MCL 18.1217, each department and agency
27 receiving appropriations in part 1 shall prepare a report on out of
28 state travel expenses not later than January 1. The report must
29 list all travel by classified and unclassified employees outside



1 this state in the previous fiscal year that was funded in whole or
2 in part with funds appropriated in the department's or agency's
3 budget. The department or agency shall submit the report to the
4 standard report recipients and to the house and senate
5 appropriations committees. The report must include all of the
6 following information:

7 (a) The dates of each travel occurrence.

8 (b) The total transportation and related expenses of each
9 travel occurrence and the proportions funded with state general
10 fund/general purpose revenues, state restricted revenues, federal
11 revenues, and other revenues.

12 Sec. 208. (1) In addition to any other requirements under this
13 part, if the department is authorized under this part to expend
14 funds in addition to those appropriated in part 1, the department
15 must do all of the following:

16 (a) Not later than November 1, provide a report to the
17 chairpersons of the house and senate appropriations committees, the
18 house and senate fiscal agencies, and the state budget office that
19 details all of the following:

20 (i) The type of funding received during the previous fiscal
21 year that was authorized in part 2 of the article that made
22 appropriations for the department in the previous fiscal year.

23 (ii) When the funding was received.

24 (iii) The amount of funding received.

25 (iv) How much of the funding was spent and for what purpose or
26 purposes.

27 (b) Not later than 60 days after receipt of funds authorized
28 under this part, provide a report to the chairpersons of the house
29 and senate appropriations committees, the house and senate fiscal



1 agencies, and the state budget office that details all of the
2 following:

3 (i) The type of funding received.

4 (ii) When the funding was received.

5 (iii) The amount of funding received.

6 (iv) The anticipated or actual amount to be spent and the
7 specified purpose or purposes.

8 (c) Not later than February 15, provide a report to the
9 chairpersons of the house and senate appropriations committees, the
10 house and senate fiscal agencies, and the state budget office with
11 an estimate of funding authorized by this part that the department
12 anticipates it will receive in the subsequent fiscal year,
13 identifying all of the following:

14 (i) The type or types of funding anticipated.

15 (ii) The amount or amounts of funding anticipated.

16 (iii) The purpose or purposes of the funding.

17 (2) If another reporting requirement under this part would
18 provide substantially similar information on a substantially
19 similar timeframe as would be reported under subsection (1),
20 subsection (1) does not apply.

21 Sec. 209. Not later than December 15, the state budget office
22 shall prepare and submit a report that provides for estimates of
23 the total general fund/general purpose appropriation lapses at the
24 close of the previous fiscal year. The report must summarize the
25 projected year-end general fund/general purpose appropriation
26 lapses by major departmental program or program areas. The state
27 budget office shall submit the report to the standard report
28 recipients and the chairpersons of the senate and house
29 appropriations committees.



1 Sec. 210. (1) In addition to the funds appropriated in part 1,
2 there is appropriated an amount not to exceed \$100,000,000.00 for
3 federal contingency authorization. Amounts appropriated under this
4 subsection are not available for expenditure until they have been
5 transferred to another line item in part 1 under section 393(2) of
6 the management and budget act, 1984 PA 431, MCL 18.1393. Federal
7 contingency authorization must not be made available to increase
8 TANF authorization.

9 (2) In addition to the funds appropriated in part 1, there is
10 appropriated an amount not to exceed \$50,000,000.00 for state
11 restricted contingency authorization. Amounts appropriated under
12 this subsection are not available for expenditure until they have
13 been transferred to another line item in part 1 under section
14 393(2) of the management and budget act, 1984 PA 431, MCL 18.1393.

15 (3) In addition to the funds appropriated in part 1, there is
16 appropriated an amount not to exceed \$30,000,000.00 for local
17 contingency authorization. Amounts appropriated under this
18 subsection are not available for expenditure until they have been
19 transferred to another line item in part 1 under section 393(2) of
20 the management and budget act, 1984 PA 431, MCL 18.1393.

21 (4) In addition to the funds appropriated in part 1, there is
22 appropriated an amount not to exceed \$45,000,000.00 for private
23 contingency authorization. Amounts appropriated under this
24 subsection are not available for expenditure until they have been
25 transferred to another line item in part 1 under section 393(2) of
26 the management and budget act, 1984 PA 431, MCL 18.1393.

27 (5) In addition to the funds appropriated in part 1, there is
28 appropriated an amount not to exceed \$100,000,000.00 for state
29 restricted Medicaid caseload reserve fund contingency



1 authorization. Amounts appropriated under this subsection are not
2 available for expenditure until they have been transferred to
3 another line item in part 1 under section 393(2) of the management
4 and budget act, 1984 PA 431, MCL 18.1393. It is the intent of the
5 legislature that this authorization must only be used to address
6 caseload costs in the traditional Medicaid program and the Healthy
7 Michigan plan.

8 Sec. 211. A department or agency shall cooperate with the
9 department of technology, management, and budget to maintain a
10 searchable website accessible by the public at no cost that
11 includes, but is not limited to, all of the following for each
12 department or agency:

13 (a) Fiscal year-to-date expenditures by category.

14 (b) Fiscal year-to-date expenditures by appropriation unit.

15 (c) Fiscal year-to-date payments to a selected vendor,
16 including the vendor name, payment date, payment amount, and
17 payment description.

18 Sec. 212. Not later than 14 days after the release of the
19 executive budget recommendation, the department shall cooperate
20 with the state budget office to provide an annual report on
21 estimated state restricted fund balances, state restricted fund
22 projected revenues, and state restricted fund expenditures for the
23 previous 2 fiscal years. The report must be submitted to the
24 standard report recipients and to the chairpersons of the senate
25 and house of representatives appropriations committees.

26 Sec. 214. A department or agency that receives an
27 appropriation under this part or part 1 must provide an annual
28 report to the standard report recipients detailing federal policy
29 changes that do, or are expected to do, any of the following:



1 (a) Affect the operations of the department, including
2 reductions in federal revenue. For federal revenue reductions, the
3 annual report must detail the impact on the department or agency
4 and residents of this state, including, but not limited to, all of
5 the following information:

6 (i) The amount of additional state revenue required to replace
7 the lost federal revenue.

8 (ii) The services the department or agency will have to reduce
9 or eliminate due to the lost revenue.

10 (iii) The anticipated fiscal impact on residents of this state.

11 (b) Affect an industry, community, population, or other group
12 regulated or served by, or that otherwise engages with, the
13 department or agency.

14 (c) Create a regulatory gap that could negatively impact the
15 public.

16 (d) Increase the department's or agency's costs.

17 (e) Reduce or eliminate a program that provides services to
18 residents of this state.

19 Sec. 215. If either of the following events occurs, not later
20 than 30 days after the event occurs, the department shall notify
21 the standard report recipients of that fact:

22 (a) A legislative objective of this part or of a bill or
23 amendment to a bill to amend the social welfare act, 1939 PA 280,
24 MCL 400.1 to 400.119b, cannot be implemented because implementation
25 would conflict with or violate federal law.

26 (b) A federal grant for which a notice of an award has been
27 received cannot be used or will not be used.

28 Sec. 216. (1) In addition to funds appropriated in part 1 for
29 all programs and services, there is appropriated, for write-offs of



1 accounts receivable, deferrals, and for prior year obligations in
2 excess of applicable prior year appropriations, an amount equal to
3 total write-offs and prior year obligations, but not to exceed
4 amounts available in prior year revenues.

5 (2) The department's ability to satisfy appropriation fund
6 sources in part 1 is not limited to collections and accruals
7 pertaining to services provided in the current fiscal year and
8 includes reimbursements, refunds, adjustments, and settlements from
9 prior years.

10 Sec. 217. Not later than February 1 of the current fiscal
11 year, the department shall submit, to the standard report
12 recipients, a report on the detailed names and amounts of estimated
13 federal, restricted, private, and local sources of revenue that
14 support the appropriations in each of the line items in part 1 for
15 the previous fiscal year. The report must itemize, rather than
16 aggregate, specific revenue sources deposited into the generic
17 statewide integrated governmental management application (SIGMA)
18 fund numbers 1200, 1274, 4000, and 5000.

19 Sec. 218. As required under part 23 of the public health code,
20 1978 PA 368, MCL 333.2301 to 333.2321, the appropriations in part 1
21 must include the following:

22 (a) Immunizations.

23 (b) Communicable disease control.

24 (c) Sexually transmitted infection control.

25 (d) Tuberculosis control.

26 (e) Prevention of gonorrhea eye infection in newborns.

27 (f) Screening newborn infants for the conditions listed in
28 section 5431 of the public health code, 1978 PA 368, MCL 333.5431,
29 or recommended by the newborn screening quality assurance advisory



1 committee created under section 5430 of the public health code,
2 1978 PA 368, MCL 333.5430.

3 (g) Health and human services annex of the Michigan Emergency
4 Management Plan.

5 (h) Prenatal care.

6 (i) Mental health.

7 Sec. 219. (1) The department may contract with the Michigan
8 Public Health Institute for the design and implementation of
9 projects and for other public health-related activities prescribed
10 in section 2611 of the public health code, 1978 PA 368, MCL
11 333.2611. The department may develop a master agreement with the
12 Michigan Public Health Institute to carry out the activities
13 described in this subsection for up to a 1-year period.

14 (2) On a semiannual basis, the department shall submit, to the
15 standard report recipients, a report that includes all of the
16 following:

17 (a) A detailed description of each funded project.

18 (b) The amount allocated for each project, the appropriation
19 line item from which the allocation is funded, and the source of
20 financing for each project.

21 (c) The expected project duration.

22 (d) A detailed spending plan for each project, including a
23 list of all subgrantees and the amount allocated to each
24 subgrantee.

25 (3) On a semiannual basis, the department shall provide, to
26 the standard report recipients, a copy of all reports, studies, and
27 publications produced by the Michigan Public Health Institute, its
28 subcontractors, or the department with the funds appropriated in
29 the department's budget in the previous fiscal year and allocated



1 to the Michigan Public Health Institute.

2 Sec. 220. The department shall ensure that faith-based
3 organizations are able to apply and compete for services, programs,
4 or contracts that the organizations are qualified and suitable to
5 fulfill. The department shall not disqualify faith-based
6 organizations solely on the basis of the religious nature of the
7 organizations or the guiding principles or statements of faith for
8 the organizations.

9 Sec. 221. In accordance with section 1b of the social welfare
10 act, 1939 PA 280, MCL 400.1b, the department shall treat part 1 and
11 this part as a time-limited addendum to the social welfare act,
12 1939 PA 280, MCL 400.1 to 400.119b.

13 Sec. 222. (1) Not later than 30 days before the implementation
14 date of a major policy change, the department shall report the
15 change to the standard report recipients.

16 (2) The department shall make the department's entire policy
17 and procedures manual available and accessible to the public on the
18 department's website.

19 (3) The department shall attach each policy bulletin issued
20 during the previous calendar year to the report under section 248.

21 Sec. 223. The department may establish and collect fees for
22 publications, videos and related materials, conferences, and
23 workshops. Collected fees are appropriated when received and must
24 be used to offset expenditures for publication printing and
25 mailing, costs of the publications, videos and related materials,
26 conferences, and workshops. The department shall not collect fees
27 under this section that exceed the cost of the expenditures. If
28 collected fees are appropriated under this section in an amount
29 that exceeds the current fiscal year appropriation, not later than



1 30 days after the collected fee appropriation, the department shall
2 notify the standard report recipients of that fact.

3 Sec. 224. The department may retain all of this state's share
4 of food assistance overissuance collections as an offset to general
5 fund/general purpose costs. Retained collections must be applied
6 against federal funds deductions in all appropriation units where
7 department costs related to the investigation and recoupment of
8 food assistance overissuances are incurred. Retained collections in
9 excess of the investigation and recoupment costs must be applied
10 against the federal funds deducted in the departmental
11 administration and support appropriation unit.

12 Sec. 226. If the revenue collected by the department from fees
13 and collections exceeds the amount appropriated in part 1, the
14 revenue may be carried forward with the approval of the state
15 budget director into the subsequent fiscal year. The revenue
16 carried forward under this section must be used as the first source
17 of funds in the subsequent fiscal year.

18 Sec. 227. If the department receives tobacco tax funds and
19 Healthy Michigan fund revenue from part 1, not later than April 1
20 of the current fiscal year, the department shall submit, to the
21 standard report recipients, a report on both of the following
22 activities during the previous fiscal year:

23 (a) Tobacco tax revenue appropriations in the Medicaid
24 program.

25 (b) Information for each project implemented with revenue
26 under this section, including all of the following:

27 (i) The project's name.

28 (ii) The appropriation line item and amount.

29 (iii) The project's target population.



1 (iv) A description of the project.

2 (v) The outcomes or accomplishments of the project.

3 Sec. 228. If the department is authorized under federal law or
4 the law of this state to collect an overpayment owed to the
5 department, beginning 60 days after the initial notification date
6 of the overpayment amount, the department may assess a penalty of
7 1% per month. If an overpayment is caused by department error, a
8 penalty may be assessed 6 months after the initial notification
9 date of the overpayment amount. The department shall not collect
10 penalty interest in an amount that exceeds the amount of the
11 original overpayment. This state's share of any funds collected
12 under this section must be deposited in the general fund of this
13 state.

14 Sec. 229. The state budget director shall take steps to ensure
15 that all state fiscal recovery funds allocated to this state under
16 the American rescue plan act of 2021, Public Law 117-2, are
17 expended by December 31, 2026, as required by law. Any state fiscal
18 recovery funds that would otherwise lapse after September 30, 2026,
19 are automatically reappropriated for the same purpose as originally
20 authorized and available for expenditure through December 31, 2026,
21 and any subsequent financial close out period.

22 Sec. 230. (1) Not later than March 1 and June 1 of the current
23 fiscal year, the department shall submit, to the standard report
24 recipients, a report on the status of the implementation of any
25 noninflationary, noncaseload, programmatic funding increases in the
26 current fiscal year from the previous fiscal year. The report must
27 confirm the implementation of already-implemented funding increases
28 and provide an explanation for any planned implementation of
29 funding increases that have not yet occurred. For any planned



1 implementation of funding increases that have not yet occurred, the
2 report must include an expected implementation date and the reason
3 for delayed implementation.

4 (2) For any programmatic funding increases not reported as
5 implemented or in process under subsection (1), the department
6 shall submit, to the standard report recipients, a status update
7 not later than June 1 of the current fiscal year.

8 Sec. 231. (1) The department shall not expend the funds
9 appropriated in part 1 to enter into any contract with a Medicaid
10 managed care organization of MI Choice Waiver, MI Coordinated
11 Health, or behavioral health unless the Medicaid managed care
12 organization agrees to do all of the following:

13 (a) Continue the direct care wage increase funded at \$3.40 per
14 hour for the services noted in the department's Medicaid provider
15 letter L 21-76 under the Medicaid managed care organization's
16 relevant program.

17 (b) Ensure, to the greatest extent possible, that the full
18 amount of funds appropriated for direct care worker wages, except
19 for costs incurred by the employer, including payroll taxes, is
20 provided to direct care workers through maintained increased wages.

21 (c) Permit a direct care worker to elect, in writing or
22 electronically, to not receive the wage increase provided in this
23 section.

24 (2) Not later than March 1 of the current fiscal year, the
25 department shall submit a report to the standard report recipients
26 that includes the following information by program and provider
27 type for the previous fiscal year:

28 (a) Hours of service that qualified for the direct care worker
29 wage increase.



1 (b) The aggregate increase in wages attributable to the
2 funding appropriated in part 1.

3 (c) A comparison of the projected increase included in the
4 capitation rates and the reported amount expended on the wage
5 increase.

6 Sec. 232. The department shall provide the approved spending
7 plan for each line item receiving an appropriation in the current
8 fiscal year to the senate and house of representatives
9 appropriations subcommittees on the department budget and the
10 senate and house fiscal agencies not later than 60 days after
11 approval by the department or not later than January 15 of the
12 current fiscal year, whichever is earlier. In all places that a
13 line-item appropriation number is listed, a line-item appropriation
14 name must be included. The spending plan must include the following
15 information regarding planned expenditures for each category:
16 allocation in the previous period, change in the allocation, and
17 new allocation. The spending plan must include the following
18 information regarding each revenue source for the line item:
19 category of the fund source indicated by general fund/general
20 purpose, state restricted, local, private, or federal. Figures
21 included in the approved spending plan must not be assumed to
22 constitute the actual final expenditures, as line items may be
23 updated on an as-needed basis to reflect changes in projected
24 expenditures and projected revenue. The department shall supplement
25 the spending plan information by providing a list of all active
26 contracts and grants in the department's contract system. For
27 amounts listed in the other contracts category of each spending
28 plan, the department shall include the name of the line item and
29 the name of the fund source for each contract, grant, and amount



1 for the current fiscal year. For amounts listed in the all other
2 costs category of each spending plan, the department shall provide
3 a list detailing planned expenditures and amounts for the current
4 fiscal year and include the name of the line item and the name of
5 the fund source related to each expenditure and amount.

6 Sec. 233. If the state administrative board, acting under
7 section 3 of 1921 PA 2, MCL 17.3, transfers funds from an amount
8 appropriated under part 1, the legislature may, by a concurrent
9 resolution adopted by a majority of the members elected to and
10 serving in each house, intertransfer funds within part 1 for the
11 particular department, board, commission, officer, or institution.

12 Sec. 234. The department shall receive and retain copies of
13 all reports funded from appropriations in part 1. The department
14 shall follow federal and state law and guidelines for short-term
15 and long-term retention of records. The department may
16 electronically retain copies of reports unless otherwise required
17 by federal and state guidelines.

18 Sec. 235. (1) Funds appropriated in part 1 must not be used to
19 restrict or impede a marginalized community's access to government
20 resources, programs, or facilities.

21 (2) From the funds appropriated in part 1, local governments
22 shall report any action or policy that attempts to restrict or
23 interfere with the duties of the local health officer.

24 Sec. 236. (1) The department shall maximize the utilization of
25 its in-person state workforce. The department shall prioritize
26 occupancy utilization of office space for each division within the
27 department. Employees with job responsibilities that require the
28 employees to serve in their capacities outside of an office shall
29 be monitored each pay period to ensure all work hours reported on



1 the timesheet were actually worked.

2 (2) The department shall comply with requirements set by the
3 office of state employer on in-person work and utilization and
4 occupancy rates of state buildings to ensure in-person work is
5 optimized and occupancy rates are 80 percent or higher, subject to
6 market conditions.

7 (3) The department shall adhere to the rules and regulations
8 of civil service, which state that the standard biweekly work
9 period for a full-time employee in the classified services is the
10 equivalent of 80 hours of work. The department shall establish
11 policies and processes to ensure all employees are working their
12 jobs during agreed-upon business hours.

13 Sec. 238. Not later than 6 months after the state budget
14 office issues work project letters, a department and an agency
15 shall submit an annual report that summarizes all work project
16 accounts. The report must include all of the following:

17 (a) A list of all work project accounts.

18 (b) The status of all work project accounts, including amounts
19 expended, amounts encumbered, and available balances for each
20 account.

21 (c) The amount of funds that lapsed from any previously
22 designated work project accounts, the name and description of the
23 work project account, and the funds that received the lapsed
24 amounts.

25 Sec. 239. For behavioral and physical health services provided
26 through managed care or the fee-for-service program, the department
27 shall require, for the nonfacility component of the reimbursement
28 rate, at least the same reimbursement for that service, if that
29 service is provided through telemedicine, as if the service



1 involved face-to-face contact between the health care professional
2 and the patient.

3 Sec. 240. To the extent possible, the department shall not
4 expend appropriations under part 1 until all existing authorized
5 work project funds available for the same purposes are exhausted.

6 Sec. 241. Not later than March 1 of the current fiscal year,
7 the department shall submit, to the standard report recipients, a
8 report on total actual expenditures in the previous fiscal year for
9 advertising and media outreach, including the purpose, amount, and
10 fund source by program or appropriation line item.

11 Sec. 242. Not later than March 1 of the current fiscal year,
12 the department shall submit a description of programs report to the
13 standard report recipients. For each program, the report must
14 include the appropriation unit; the line item name and number; the
15 appropriation history; the program name; the program overview; a
16 financing summary; and, where applicable, the program's legal
17 basis, effectiveness, and outcomes.

18 Sec. 243. Total authorized appropriations from all sources
19 under part 1 for legacy costs for the fiscal year ending September
20 30, 2027 are estimated at \$174,401,600.00. From this amount, total
21 department appropriations for pension-related legacy costs are
22 estimated at \$174,401,600.00. Total department appropriations for
23 retiree health care legacy costs are estimated at \$0.00.

24 Sec. 244. On a quarterly basis, the department shall submit,
25 to the standard report recipients, a report on any line-item
26 appropriation for which the department estimates total annual
27 expenditures would exceed the funds appropriated for the line-item
28 appropriation by 5% or more. The department shall provide a
29 detailed explanation for any relevant line-item appropriation



1 exceedance and identify the corrective actions undertaken to
2 mitigate line-item appropriation expenditures from exceeding the
3 funds appropriated for the line-item appropriation by a greater
4 amount. This section does not apply for line-item appropriations
5 that are part of the May revenue estimating conference caseload and
6 expenditure estimates.

7 Sec. 245. Not later than April 1, the department shall provide
8 to the standard report recipients a copy of its annual strategic
9 plan prepared in compliance with section 363 of the management and
10 budget act, 1984 PA 431, MCL 18.1363. The plan must include the
11 mission, vision, goals, strategies, and performance measures of the
12 department.

13 Sec. 246. The department shall report on any court settlement
14 that may require further legislative review of state statutory
15 programs or regulations in this state.

16 Sec. 247. Not later than November 15, the department shall
17 disclose on a publicly accessible website private and other third-
18 party funds received by the department in the previous fiscal year.
19 The report must include the amount of funding received, the
20 specific source of funding received, the purpose for which funding
21 was expended, and the amount of any remaining funds. The report
22 must be submitted to the standard report recipients and to the
23 chairpersons of the senate and house appropriations committees.

24 Sec. 248. Not later than April 1, the department shall report
25 on each specific policy change made to implement a public act
26 affecting the department that took effect during the previous
27 calendar year. The report must include a reference to the public
28 act that necessitates the policy change. The department shall
29 submit the report to the standard report recipients, the senate and



1 house appropriations committees, and the joint committee on
2 administrative rules.

3 Sec. 249. (1) The state budget director shall take steps to
4 ensure that all state fiscal recovery funds allocated to this state
5 under the American rescue plan act of 2021, Public Law 117-2, are
6 expended by December 31, 2026, as required by law. The state budget
7 director may reallocate appropriated funds for the purpose of fully
8 utilizing state fiscal recovery funds that are in jeopardy of not
9 meeting the expenditure deadline for reasons that may include, but
10 are not limited to, completed projects coming in under budget or
11 funds that were not fully used by subrecipients. The state budget
12 director shall reallocate any of the funds reallocated under this
13 subsection to the programs or purposes specified in this section.
14 Any funds reallocated are unappropriated and immediately
15 reappropriated for the following purposes:

16 (a) To reclassify general fund/general purpose appropriations
17 for payroll and covered benefits for eligible public health and
18 safety employees at the department of corrections.

19 (b) To reclassify general fund/general purpose appropriations
20 for payroll and covered benefits for eligible public health and
21 safety employees at the department of state police.

22 (2) All applicable guidance, implementation, and reporting
23 provisions under the American rescue plan act of 2021, Public Law
24 117-2 must be followed for state fiscal recovery funds reallocated
25 and reappropriated under subsection (1).

26 (3) The state budget director shall notify the senate and
27 house appropriations committees in writing not later than 1
28 business day after making any reallocations under subsection (1).
29 The notice required under this subsection must include all of the



1 following:

2 (a) The authorized program under which funds were originally
3 appropriated.

4 (b) The amount of the reallocation.

5 (c) The program or the purpose of the funds that were
6 reallocated.

7 (d) The department to which the funds were reallocated under
8 subsection (1).

9 (e) The amount reallocated to each program or purpose.

10 Sec. 250. (1) Within 30 days after the enactment of this act,
11 the house and senate shall provide to the state budget office a
12 jointly agreed upon list of legislatively directed spending items
13 as that term is defined in section 1364 of the management and
14 budget act, 1984 PA 431, MCL 18.1364, funded in part 1 The list
15 must include all information and documents pertaining to the funded
16 items as publicly disclosed in accordance with sections 364 and
17 364a of the management and budget act, 1984 PA 431, MCL 18.1364 and
18 18.1364a.

19 (2) In accordance with section 364(4) of the management and
20 budget act, 1984 PA 431, MCL 18.1364 the department or agency
21 administering the grant shall post a report in a publicly
22 accessible location on its website beginning March 15 of the
23 current fiscal year. The department or agency shall update the
24 report and shall post an updated report not later than June 15 of
25 the current fiscal year and again not later than September 15 of
26 the current fiscal year. The department shall include in the report
27 the most comprehensive information the department has available at
28 the time of posting for grants awarded.

29 Sec. 253. (1) The department shall ensure that each federally



1 recognized tribe is able to apply and compete for services,
2 programs, grants, and contracts.

3 (2) For competitive grant programs described in this part,
4 each federally recognized tribe is eligible to apply for grant
5 funds made available to organizations exempt from federal income
6 tax under section 501(c)(3) of the internal revenue code of 1986,
7 26 USC 501, and to local units of government.

8 Sec. 263. (1) Except as provided in this subsection, before
9 submission of a waiver, state plan amendment, or similar proposal
10 to CMS or another federal agency, the department shall notify the
11 standard report recipients of the planned submission. This
12 subsection does not apply to the submission of a waiver, state plan
13 amendment, or similar proposal that does not propose a material
14 change or is outside of the ordinary course of a waiver, state plan
15 amendment, or similar proposal.

16 (2) On a semiannual basis, the department shall submit, to the
17 standard report recipients, a report that summarizes the status of
18 any new or ongoing discussions with CMS, the United States
19 Department of Health and Human Services, or another federal agency
20 regarding any potential or future waiver applications and the
21 status of any submitted waivers that have not yet received federal
22 approval. If there is not a reportable item at the time that a
23 semiannual report is due, a report is not required.

24 Sec. 264. The department shall not take disciplinary action
25 against an employee of the department for communicating with a
26 member of the senate or house of representatives or a member's
27 staff, unless the communication is prohibited by law and the
28 department is exercising its authority as provided by law.

29 Sec. 270. The department shall advise the legislature of the



1 receipt of a notification from the attorney general's office of a
2 legal action in which expenses had been recovered under section 10b
3 of the medicaid false claims act, 1977 PA 72, MCL 400.610b. If
4 applicable, not later than February 1 of the current fiscal year,
5 the department shall submit, to the standard report recipients, a
6 report that includes, but is not limited to, all of the following:

7 (a) The total amount recovered from the legal action.

8 (b) The program or service for which the money was originally
9 expended.

10 (c) Details on the disposition of the funds recovered, such as
11 the appropriation or revenue account in which the money was
12 deposited.

13 (d) A description of the facts involved in the legal action.

14 Sec. 274. On the day that is 1 week after the day that the
15 governor submits the executive budget proposal for the ensuing
16 fiscal year to the legislature, the department, in collaboration
17 with the state budget office, shall submit, to the standard report
18 recipients, a report on spending and revenue projections for each
19 of the capped federal funds listed in this subsection. The report
20 must contain actual spending and revenue in the previous fiscal
21 year, spending and revenue projections for the current fiscal year
22 as enacted, and spending and revenue projections in the executive
23 budget proposal for the immediately ensuing fiscal year for each
24 individual line item for the department budget. The report must
25 also include federal funds transferred to other departments. The
26 capped federal funds include, but are not limited to, all of the
27 following:

28 (a) TANF.

29 (b) Title XX social services block grant.



1 (c) Title IV-B subpart I child welfare services block grant.

2 (d) Title IV-B subpart II promoting safe and stable families
3 funds.

4 (e) Low-income home energy assistance program.

5 Sec. 275. (1) On a quarterly basis, the department, with the
6 approval of the state budget director, is authorized to realign
7 sources between other federal, TANF, and capped federal financing
8 authorizations to maximize federal revenues. The realignment of
9 financing must not produce any of the following:

10 (a) A gross increase or decrease in the department's total
11 individual line item authorizations.

12 (b) A net increase or decrease in total federal revenues.

13 (c) A net increase in TANF authorization.

14 (2) On a quarterly basis, the department shall submit, to the
15 standard report recipients, a report on the realignment of federal
16 fund sources transacted to date in the current fiscal year under
17 subsection (1), including the dates, line items, and amounts of the
18 transactions. If, at the time a quarterly report is due, a
19 transaction was not made under subsection (1), a report is not
20 required.

21 (3) Not later than 30 days after the date on which year-end
22 book closing is completed, the department shall submit, to the
23 standard report recipients, a report on the realignment of federal
24 fund sources that took place as part of the year-end closing
25 process for the previous fiscal year.

26 Sec. 290. Any public advertisement for public assistance must
27 inform the public of the welfare fraud hotline operated by the
28 department.

29 Sec. 291. (1) The department shall not release funds to



1 agencies providing physical and behavioral health services that are
2 used to increase the hourly wage rate for direct care workers until
3 the contractors, subcontractors, or providers receiving funding to
4 support the direct care wage increases have provided documentation
5 that demonstrates the distribution of payments to eligible staff of
6 the wage increases provided under sections 231, 1644, and 1645 of
7 article 6 of 2025 PA 22, as required in the contract between each
8 individual PIHP and the department.

9 (2) The department shall require each contractor,
10 subcontractor, or provider described in subsection (1) that receive
11 funds to provide a wage increase for direct care workers to comply
12 with performance-related metrics to maintain their eligibility for
13 funding. The performance-related metrics must include, but are not
14 limited to, all of the following:

15 (a) Efforts to improve data collection to quantify the number
16 of direct care workers, including by type, employer, compensation
17 and benefits, job classification, region of the state, and other
18 relevant information.

19 (b) Accounting of the total hours worked under which a wage
20 increase was applied under sections 231, 1644, and 1645 of article
21 6 of 2025 PA 22 by county in which the worker was employed.

22 (3) The department shall require each contractor,
23 subcontractor, or provider described in subsection (1) to ensure
24 that the funds appropriated in this section are only used to
25 provide the direct care worker wage increase, except for costs
26 incurred by the employer, including payroll taxes.

27 (4) The department shall require an annual report from the
28 contractors, subcontractors, and providers described in subsection
29 (2). Not later than February 1 of the current fiscal year, the



1 department must submit the annual reports to the standard report
2 recipients. The report required under this subsection must be
3 submitted to the department not later than 60 days after the end of
4 the contract period and must include the following information:

5 (a) The amount of funding used to maintain the direct care
6 worker wage increase.

7 (b) A comparison of the amount of funding appropriated and the
8 amount of funding expended for the direct care worker wage
9 increase.

10 (c) An explanation for any difference in amount appropriated
11 and amount expended for the direct care worker wage increase.

12 (d) Confirmation that any unexpended or unencumbered funds
13 appropriated for the direct care worker wage increase were returned
14 to the department.

15 Sec. 295. Not later than April 1 of the current fiscal year,
16 the department shall submit, to the standard report recipients, a
17 report on funds appropriated for the healthy moms, healthy babies
18 initiative. The report must include the budgeted amount, year-to-
19 date expenditures, remaining balance of appropriations, and the
20 percent of budget spent for each appropriation related to the
21 initiative. The report must also include information on how the
22 funds have assisted with meeting the goals and outcomes of the
23 initiative.

24 Sec. 297. On a quarterly basis, the department or agency
25 receiving appropriations in part 1 shall report on the number of
26 full-time equated positions in pay status by civil service
27 classification, including a comparison by line item of the number
28 of full-time equated positions authorized from funds appropriated
29 in part 1 to the actual number of full-time equated positions



1 employed by the department at the end of the reporting period. The
2 report must be submitted to the senate and house appropriations
3 committees and to the standard report recipients.
4

5 **DEPARTMENTAL ADMINISTRATION AND SUPPORT**

6 Sec. 301. From the funds appropriated in part 1 for child
7 welfare institute, the department shall train private child placing
8 agency staff in the pre-service training requirements for child
9 welfare caseworkers and supervisors. The department shall allocate
10 \$1,000,000.00 to provide private child placing agency staff an
11 opportunity to complete the training in a virtual format at the
12 staff's private child placing agency facility. If a private child
13 placing agency prefers a hybrid training format that includes
14 virtual and in-person instruction, the department may utilize the
15 \$1,000,000.00 allocated under this section to make that training
16 format available to the private child placing agency staff.

17 Sec. 302. From the funds appropriated in part 1 for property
18 management, \$44,613,600.00 is appropriated for facilities or
19 materials, equipment, other contracts, and all other costs. From
20 the funds appropriated in this section, the department shall employ
21 market analyses, office space utilization data, contract
22 negotiations or renegotiations, and any other appropriate
23 mechanisms to assess and determine the minimum required square
24 footage of leased or owned office space essential for departmental
25 operations. The number and geographic placement of local county
26 offices active as of September 30, 2026, must be preserved,
27 regardless of any alterations in physical addresses or
28 modifications to leased or owned office space resulting from
29 contract negotiations or property management efficiency efforts by



1 the department.

2
3 **CHILD SUPPORT ENFORCEMENT**

4 Sec. 401. (1) The appropriations in part 1 assume a total
5 federal child support incentive payment of \$26,500,000.00.

6 (2) From the federal money received for child support
7 incentive payments, \$12,000,000.00 must be retained by this state
8 and expended for child support program expenses.

9 (3) From the federal money received for child support
10 incentive payments, \$14,500,000.00 must be paid to counties based
11 on each county's performance level for each of the performance
12 measures under 45 CFR 305.2.

13 (4) If the child support incentive payment to this state from
14 the federal government is greater than \$26,500,000.00, then 100% of
15 the amount in excess must be retained by this state and is
16 appropriated until the total retained by this state reaches
17 \$15,397,400.00.

18 (5) If the child support incentive payment to this state from
19 the federal government is greater than the amount needed to satisfy
20 subsections (1), (2), (3), and (4), the additional funds are
21 subject to appropriation by the legislature.

22 (6) If the child support incentive payment to this state from
23 the federal government is less than \$26,500,000.00, then the state
24 share and the county share must each be reduced by 50% of the
25 shortfall.

26 Sec. 409. (1) If statewide retained child support collections
27 exceed \$38,300,000.00, 75% of the amount in excess of
28 \$38,300,000.00 is appropriated to legal support contracts. The
29 excess appropriation may be distributed to eligible counties to



1 supplement, but not supplant, county title IV-D funding.

2 (2) Each county whose retained child support collections in
3 the current fiscal year exceed its fiscal year 2004-2005 retained
4 child support collections, excluding tax offset and financial
5 institution data match collections in both the current fiscal year
6 and fiscal year 2004-2005, shall receive its proportional share of
7 the 75% excess appropriation.

8 Sec. 410. (1) If title IV-D-related child support collections
9 are escheated, the state budget director is authorized to adjust
10 the sources of financing for the funds appropriated in part 1 for
11 legal support contracts to reduce federal authorization by 66% of
12 the escheated amount and increase general fund/general purpose
13 authorization by the same amount. The adjustment is required to
14 offset the loss of federal revenue due to the escheated amount
15 being counted as title IV-D program income in accordance with 45
16 CFR 304.50.

17 (2) Not later than 30 days after an adjustment under
18 subsection (1), the department shall notify the standard report
19 recipients of the adjustment.
20

21 **COMMUNITY SERVICES AND OUTREACH**

22 Sec. 453. (1) From the funds appropriated in part 1 for
23 homeless programs, the department shall allocate funds to the
24 emergency shelter program to support efforts of shelter providers
25 to move homeless individuals and households into permanent housing
26 as quickly as possible. The funds must be equal to or exceed the
27 amount that a provider would receive if the provider is paid a
28 \$19.00 per diem rate. Expected outcomes are increased shelter
29 discharges to stable housing destinations, decreased recidivism



1 rates for shelter clients, and a reduction in the average length of
2 stay in emergency shelters.

3 (2) Not later than March 1 of the current fiscal year, the
4 department shall submit, to the standard report recipients, a
5 report on the total amount expended for the emergency shelter
6 program in the prior 2 fiscal years, the total number of shelter
7 nights provided, and the average length of stay in an emergency
8 shelter.

9 Sec. 454. The department shall allocate the full amount of
10 funds appropriated in part 1 for homeless programs to provide
11 services for homeless individuals and families, including, but not
12 limited to, third-party contracts for emergency shelter services.

13 Sec. 455. As a condition of receipt of federal TANF revenue,
14 after admitting a family to a homeless shelter, the homeless
15 shelter and human services agencies shall collaborate with the
16 department to obtain necessary TANF eligibility information on the
17 family as soon as possible. From the funds appropriated in part 1
18 for homeless programs, the department is authorized to make
19 allocations of TANF revenue only to the homeless shelters and human
20 services agencies that report necessary data to the department to
21 meet TANF eligibility reporting requirements. Homeless shelters or
22 human services agencies that do not report necessary data to the
23 department to meet TANF eligibility reporting requirements shall
24 not receive reimbursements that exceed the per diem amount the
25 homeless shelters or human service agencies received in fiscal year
26 2000. The use of TANF revenue under this section is not an ongoing
27 commitment of funding.

28 Sec. 456. From the funds appropriated in part 1 for homeless
29 programs, the department shall allocate \$10,000.00 to reimburse



1 public service agencies that provide documentation of paying birth
2 certificate fees on behalf of category 1 homeless clients at county
3 clerk's offices. Each public service agency must be reimbursed for
4 the cost of the birth certificate fees quarterly until the
5 allocation is fully spent.

6 Sec. 457. From the funds appropriated in part 1 for homeless
7 programs, the department shall allocate \$8,500,000.00 of TANF
8 revenue to support family shelters or families who are homeless and
9 at risk of being homeless. Funds appropriated under this section
10 must be used as follows:

11 (a) \$3,000,000.00 for emergency hotels for families
12 experiencing homelessness.

13 (b) \$3,500,000.00 for assistance and supports to families
14 engaged with child welfare. This may include, but is not limited
15 to, eviction diversion, first month's rent and deposit, and utility
16 arrears.

17 (c) \$2,000,000.00 for creating additional spaces at family
18 homeless shelters that have been in operation for at least 24
19 months.

20 Sec. 458. From the funds appropriated in part 1 for homeless
21 programs, the department shall require any entities receiving
22 direct or indirect state funds to report data to a Homeless
23 Management Information System that satisfies the baseline data
24 collection requirements.

25 Sec. 459. From the funds appropriated in part 1 for homeless
26 programs, the department shall allocate \$2,000,000.00 of TANF
27 revenue to acquire and develop for individuals and families
28 noncongregate shelter that utilizes options under a Housing First
29 model and prioritizes providing stable and permanent housing



1 without preconditions or requirements, such as sobriety or
2 participation in treatment programs. Eligible uses for this funding
3 may include, but are not limited to, hotels, motels, dormitories,
4 recuperative care facilities, and other facilities that offer
5 noncongregate shelter.

6 Sec. 460. From the funds appropriated in part 1 for kids' food
7 basket, the department shall allocate \$525,000.00 to fund a project
8 with a nonprofit, community-based organization organized under the
9 laws of this state that is exempt from federal income tax under
10 section 501(c)(3) of the internal revenue code of 1986, 26 USC 501,
11 and is located in a city with a population between 185,000 and
12 200,000 and in a county with a population between 600,000 and
13 700,000, according to the most recent federal decennial census. The
14 nonprofit organization recipient must have an existing network of
15 food delivery to low-income children in not less than 3 counties in
16 this state. The nonprofit organization shall use the funds to
17 expand its services to additional schools and communities. The
18 funding may be used to cover employee costs, food and supplies,
19 equipment, and other operational costs identified by the
20 organization to support its mission and goals.

21 Sec. 462. From the funds appropriated in part 1 for senior
22 university, the department shall allocate \$400,000.00 to a
23 community action alliance located in a city with a population over
24 500,000 according to the most recent federal decennial census to
25 improve connectivity and computer skills to seniors.

26 Sec. 463. From the funds appropriated in part 1 for runaway
27 and homeless youth grants and domestic violence prevention and
28 treatment, the department is authorized to make allocations of TANF
29 revenue only to agencies that report necessary data to the



1 department to meet TANF eligibility reporting requirements.

2 Sec. 464. (1) From the funds appropriated in part 1 for diaper
3 assistance grant, the department shall allocate grants to diaper
4 assistance programs, maternity homes, local county offices, and
5 other nonprofit agencies that distribute diapers free of charge and
6 were established as of January 1, 2020. The funds must be used only
7 to purchase diapering supplies and to cover related administrative
8 costs. Not more than 10% of the funds appropriated in part 1 are
9 expendable for administrative purposes.

10 (2) Not later than March 1 of the current fiscal year, the
11 department shall submit, to the standard report recipients, a
12 report on the distribution of diapering supplies that includes, but
13 is not limited to, the names and locations of the entities
14 described in subsection (1) that distribute diaper supplies and the
15 total amount of diapering supplies distributed to each entity.

16 (3) Funds appropriated for diaper assistance grant are
17 considered work project funds, do not lapse at the end of the
18 fiscal year, and are available for expenditures for projects under
19 this section until the projects have been completed. The following
20 is in compliance with section 451a of the management and budget
21 act, 1984 PA 431, MCL 18.1451a:

22 (a) The purpose of the work project is to provide funding for
23 grants for eligible entities to distribute diapers free of charge.

24 (b) The work project will be accomplished through partnerships
25 with diaper assistance programs, maternity homes, and other
26 nonprofit agencies.

27 (c) The total estimated cost of the work project is
28 \$6,404,000.00.

29 (d) The tentative completion date for the work project is



1 September 30, 2030.

2 Sec. 465. (1) From the funds appropriated in part 1 for
3 community services and outreach administration, \$2,950,000.00 must
4 be distributed as provided in subsection (2). Michigan 2-1-1 must
5 continue to seek funding from local United Way organizations and
6 other nonprofit organizations and foundations.

7 (2) Funds distributed under subsection (1) must be distributed
8 to Michigan 2-1-1, a nonprofit corporation organized under the laws
9 of this state that is exempt from federal income tax under section
10 501(c)(3) of the internal revenue code of 1986, 26 USC 501, and
11 whose mission is to coordinate and support a statewide 2-1-1
12 system. Michigan 2-1-1 shall use the funds only to fulfill the
13 Michigan 2-1-1 business plan adopted by Michigan 2-1-1 in January
14 2005.

15 (3) Michigan 2-1-1 shall refer any received calls that report
16 fraud, waste, or abuse of state-administered public assistance to
17 the department.

18 (4) Michigan 2-1-1 shall submit, to the department, the senate
19 and house of representatives standing committees with primary
20 jurisdiction over matters relating to human services and
21 telecommunications on 2-1-1 system performance, and the standard
22 report recipients, a report that includes, but is not limited to,
23 call volume by health and human service needs and unmet needs
24 identified through caller data and number and the percentage of
25 callers referred to public or private provider types.

26 Sec. 466. Not later than March 1 of the current fiscal year,
27 the department shall submit to the standard report recipients a
28 report on the runaway homeless youth program that includes, but is
29 not limited to, all of the following:



1 (a) A list of counties served and the amount of funding
2 allocated to each county.

3 (b) The amount of funding being allocated to previously
4 underserved communities and how capacity has been expanded or is
5 planned to be expanded in those communities.

6 (c) Identified barriers that have hindered providers from
7 expanding capacity.

8
9 **CHILDREN'S SERVICES AGENCY - CHILD WELFARE**

10 Sec. 501. (1) A goal is established that not more than 25% of
11 all children in foster care at any given time during the current
12 fiscal year, unless contrary to the best interest of the child,
13 will have been in foster care for 24 months or more.

14 (2) Not later than March 1 of the current fiscal year, the
15 department shall submit, to the standard report recipients, a
16 report describing the steps that will be taken to achieve the goal
17 under subsection (1). The report must also include the following:

18 (a) An explanation of the most significant barriers that
19 prevent long-term foster children from permanent placements.

20 (b) The number of children currently in foster care for longer
21 than 24 months and the percentage of those children who meet any of
22 the following requirements:

23 (i) Had paid Medicaid behavioral health claims or encounters
24 within the last year.

25 (ii) Were living in a relative placement, a child caring
26 institution, or a licensed foster home.

27 (iii) Were within the ages of 0-5, 6-11, or 12-17 years of age.

28 (c) The total number of children in foster care as of October
29 1 of the current fiscal year.



1 Sec. 502. From the funds appropriated in part 1 for foster
2 care, the department shall reimburse Indian tribal governments for
3 50% of the foster care expenditures for children who are under the
4 jurisdiction of Indian tribal courts and are not otherwise eligible
5 for federal foster care cost sharing. However, the department may
6 reimburse up to 100% of the foster care expenditures for an Indian
7 tribal government that enters into a state-tribal Title IV-E
8 agreement allowed under this state's Title IV-E state plan.

9 Sec. 505. Not later than March 1 of the current fiscal year,
10 the department shall submit, to the standard report recipients, a
11 report on youth referred or committed to the department for care or
12 supervision in the previous fiscal year that outlines the number of
13 youth served by the department in the juvenile justice system by
14 the type of setting for each youth.

15 Sec. 506. From the funds appropriated in part 1 for attorney
16 general contract, not later than March 1 of the current fiscal
17 year, the department shall submit, to the standard report
18 recipients, a report on the juvenile justice system in any county
19 in which funds appropriated in part 1 are expended. The report must
20 include, but not be limited to, all of the following:

21 (a) The number of youth referred or committed to the
22 department for care or supervision in the previous fiscal year and
23 in the first quarter of the current fiscal year.

24 (b) The number of youth referred or committed to the care or
25 supervision of the county in which funds appropriated in part 1
26 were expended for the previous fiscal year and the first quarter of
27 the current fiscal year.

28 (c) The type of setting for each youth referred or committed
29 for care or supervision, any applicable performance outcomes, and



1 identified financial costs or savings.

2 (d) The required and actual staff-to-youth ratios.

3 Sec. 507. The department's ability to satisfy appropriation
4 deductions in part 1 for foster care private collections is not
5 limited to collections and accruals pertaining to services provided
6 only in the current fiscal year and may include revenues collected
7 during the current fiscal year for services provided in prior
8 fiscal years.

9 Sec. 508. (1) In addition to the amount appropriated in part 1
10 for children trust Michigan, money granted or money received as
11 gifts or donations to the children's trust fund created in 1982 PA
12 249, MCL 21.171 to 21.172, is appropriated for expenditure.

13 (2) For the funds described in subsection (1), the department
14 shall ensure that administrative delays are avoided and local grant
15 recipients and direct service providers receive money in an
16 expeditious manner. The department and the state board as that term
17 is defined in section 2 of the child abuse and neglect prevention
18 act, 1982 PA 250, MCL 722.601, shall make the children's trust fund
19 contract funds available to grantees not later than 31 days after
20 the start date of the funded project.

21 Sec. 509. (1) From the funds appropriated in part 1 for
22 adoption support services, the department shall maintain a rate
23 structure that pays for cases based on the average length of time
24 it takes to reach adoption finalization by case characteristics for
25 licensed child placing agencies contracted with the department that
26 provide adoption services for youth in foster care.

27 (2) For cases accepted before the implementation of the new
28 rate structure described in subsection (1), the department shall
29 maintain the increase of contracted rates paid to private child



1 placing agencies, including the \$23.00 per diem for all foster
2 youth from the date of the case acceptance to the date of adoption
3 petition acceptance or 150 days, whichever occurs sooner, for
4 licensed child placing agencies contracted with the department to
5 provide adoption services for foster youth. The per diem rate must
6 be separate from the outcome-based reimbursement system and must
7 not be deducted from the total reimbursement an agency receives for
8 the applicable placement or finalization rate of an adoption.

9 Sec. 510. The department shall submit reports on a monthly
10 basis to the standard report recipients on all of the following:

11 (a) The number of children awaiting placement in a residential
12 setting by child caring institution.

13 (b) The reasons for the denial of placements that were
14 referred within the month, including, but not limited to, facility
15 bed shortages, placement process delays, facility hold, or other
16 reasons.

17 (c) The number of incentive payments that were awarded by the
18 department by child caring institution.

19 (d) The number of incentive payments that were denied by the
20 department by child caring institution.

21 (e) Of the denials identified in subdivision (d), the
22 department shall provide the rationale for denial of incentive
23 payments including, but not limited to, refusal of placement, lack
24 of staffing, or other reasons.

25 Sec. 511. The department shall submit, to the standard report
26 recipients and the senate and house of representatives standing
27 committees that cover subject matters dealing with families and
28 human services, reports on a semiannual basis that include the
29 number and percentage of children who received timely physical and



1 mental health examinations after entry into foster care. The goal
2 of the program is for not less than 85% of children to have an
3 initial medical and mental health examination that is not later
4 than 30 days after entry into foster care.

5 Sec. 512. (1) From the funds appropriated in part 1 for foster
6 care payments, the department shall allocate \$500,000.00 of TANF
7 revenue to provide luggage to a child who is being removed from the
8 child's home or changing placement and is a TANF eligible
9 individual. The luggage provided under this section is considered
10 to belong to the child and may not be confiscated by the department
11 or the child's foster parent. The department is not required to
12 provide new luggage under this section to a child who is changing
13 placement and has had luggage previously provided by the
14 department.

15 (2) The department may partner with local charities to
16 establish and maintain the supply of luggage to be used to
17 transport a child's personal belongings. Additionally, the
18 department may accept donations of luggage to fulfill the
19 requirements of this section.

20 (3) As used in this section, "luggage" means any of the
21 following:

22 (a) A suitcase of any size.

23 (b) A duffel bag that holds at least 30 liters.

24 Sec. 513. (1) The department shall not expend funds
25 appropriated in part 1 to pay for the department's direct placement
26 of a child in an out-of-state facility unless all of the following
27 conditions are met:

28 (a) An appropriate placement is not available in this state,
29 as determined by the department's interstate compact office.



1 (b) An out-of-state placement exists that is nearer to the
2 child's home than the closest appropriate in-state placement, as
3 determined by the department's interstate compact office.

4 (c) The out-of-state facility meets all of the licensing
5 standards for a comparable facility in this state.

6 (d) The out-of-state facility meets all of the applicable
7 licensing standards of the state in which it is located.

8 (e) The department has visited the site of the out-of-state
9 facility; has reviewed the facility records, licensing records, and
10 reports; and believes that the facility is an appropriate placement
11 for the child.

12 (2) The department shall not expend money for a child placed
13 in an out-of-state facility without approval of the executive
14 director of the children's services agency.

15 (3) Not later than March 1 of the current fiscal year, the
16 department shall submit, to the state court administrative office
17 and the standard report recipients, a report on the number of
18 Michigan children residing in out-of-state facilities in the
19 previous fiscal year, the total cost and average per diem cost of
20 the out-of-state placements to this state, a list of each out-of-
21 state placement arranged by the Michigan county of residence for
22 each child, and a list of out-of-state facilities that were visited
23 by the department before the child's placement.

24 Sec. 514. (1) From the funds appropriated in part 1 for foster
25 care payments, the department shall maintain a statewide respite
26 care services network available to licensed foster parents and
27 unlicensed relative caregivers that care for children in foster
28 care.

29 (2) Not later than March 1 of the current fiscal year, the



1 department shall submit, to the standard report recipients, a
2 report on the total number of licensed foster parents and
3 unlicensed relative caregivers that were provided respite services,
4 the average amount of respite time per month, and the total amount
5 of funding spent on respite services during the previous fiscal
6 year.

7 Sec. 515. If a children's protective services caseworker
8 requests approval for another children's protective services
9 caseworker or other department employee to accompany the caseworker
10 on a home visit because the caseworker believes that it would be
11 unsafe to conduct the home visit alone, the department shall not
12 deny the request.

13 Sec. 516. (1) From funds appropriated in part 1 for child care
14 fund, the administrative or indirect cost payment equal to 10% of a
15 county's total monthly gross expenditures must be distributed to
16 the county on a monthly basis, and a county is not required to
17 submit documentation to the department for any of the expenditures
18 that are covered under the 10% payment as described in section
19 117a(4)(b)(ii) and (iv) of the social welfare act, 1939 PA 280, MCL
20 400.117a.

21 (2) From the funds appropriated in part 1 for child care fund
22 - indirect cost allotment, the department shall allocate
23 \$3,500,000.00 to counties and tribal governments that receive
24 reimbursements in part 1 from child care fund.

25 (3) The amount described in subsection (2) must be distributed
26 to each county or tribal government in the same proportion as
27 indirect cost allotments are provided to counties in the same
28 manner described in section 117a of the social welfare act, 1939 PA
29 280, MCL 400.117a.



1 Sec. 517. For a child placed in a family foster care home
2 located out of this state, the department may ask a state or
3 private child placing agency contracted by the receiving state to
4 carry out required visits and any additional visits that the
5 department finds necessary.

6 Sec. 518. Not later than March 1 of the current fiscal year,
7 the department shall submit, to the standard report recipients, a
8 report on the cumulative child care fund expenditures of in-home
9 juvenile justice care that are eligible for the 75% state and 25%
10 local split required under section 117a(4)(i) of the social welfare
11 act, 1939 PA 280, MCL 400.117a. Eligible expenditures include
12 community-based juvenile supervision, services, and related
13 practices, and per diem rates for the use of respite and shelter
14 for less than 30 days. The report must also include the
15 expenditures by county, the type of service provided, and the
16 number of youth receiving care.

17 Sec. 520. Not later than February 15 of the current fiscal
18 year, the department shall submit, to the standard report
19 recipients, a report on the number of days of care and expenditures
20 by funding source for the previous fiscal year for out-of-home
21 placements by specific placement programs for child abuse or child
22 neglect and juvenile justice, including, but not limited to, paid
23 relative placement, department direct family foster care, private-
24 agency-supervised foster care, private child caring institutions,
25 county-supervised facilities, and independent living. The report
26 must also include the number of days of care for department-
27 operated residential juvenile justice facilities.

28 Sec. 522. (1) From the funds appropriated in part 1 for youth
29 in transition, the department shall allocate \$750,000.00 for



1 scholarships through the fostering futures scholarship program in
2 the Michigan education trust to youth who were in foster care
3 because of child abuse or child neglect and are attending a college
4 or a career technical educational institution located in this
5 state. One hundred percent of the funds appropriated must be used
6 to fund scholarships for the youth described in this section.

7 (2) Not later than June 1 of the current fiscal year, the
8 department shall submit, to the standard report recipients, a
9 report that includes the number of youth who applied for
10 scholarships under this section, the number of youth who received
11 scholarships under this section and the amount of each scholarship,
12 and the total amount of funds spent or encumbered in the current
13 fiscal year.

14 Sec. 523. Not later than February 15 of the current fiscal
15 year, the department shall submit, to the standard report
16 recipients, a report on the MI Family Together program. The report
17 must include both of the following:

18 (a) Utilization and outcome data based on families served.

19 (b) For each program, information on any innovations or
20 expansions that may increase child safety and reduce risk.

21 Sec. 524. As a condition of receiving funds appropriated in
22 part 1 for strong families/safe children, not later than October 1
23 of the current fiscal year, counties shall submit the service
24 spending plan to the department for approval. Not later than 30
25 calendar days after receipt of a properly completed service
26 spending plan, the department shall approve the service spending
27 plan.

28 Sec. 525. The department shall maintain the same on-site
29 evaluation processes for privately operated child welfare and



1 juvenile justice residential facilities as is used to evaluate
2 state-operated facilities. Penalties for noncompliance must be the
3 same for privately operated child welfare and juvenile justice
4 residential facilities and state-operated facilities.

5 Sec. 526. From the funds appropriated in part 1 for court-
6 appointed special advocates, the department shall allocate
7 \$2,250,000.00 to fund a project with a nonprofit, community-based
8 organization organized under the laws of this state that is exempt
9 from federal income tax under section 501(c)(3) of the internal
10 revenue code of 1986, 26 USC 501, located in a charter township
11 with a population between 18,000 and 19,000 that is located in a
12 county with a population between 600,000 and 700,000, according to
13 the most recent federal decennial census. The nonprofit
14 organization recipient must have an existing network of affiliate
15 programs operating in not less than 25 counties in this state. The
16 recipient nonprofit organization shall use the funds to recruit,
17 screen, train, and supervise volunteers who provide advocacy
18 services on behalf of abused and neglected children.

19 Sec. 528. From the funds appropriated in part 1 for child care
20 fund, the department shall allocate \$3,730,300.00 to support the
21 annual basic grant to counties with a population of less than
22 75,000, according to the most recent federal decennial census, and
23 as described in section 117e of the social welfare act, 1939 PA
24 280, MCL 400.117e, and to eligible tribal entities. The basic grant
25 must be \$56,520.00 to eligible counties and tribal entities.

26 Sec. 529. From the funds appropriated in part 1 for family
27 preservation programs, the department shall maintain the total
28 combined funding levels of the MI Family Together program at an
29 amount not less than the amount provided as of September 30, 2025.



1 Sec. 530. (1) All master contracts relating to foster care and
2 adoption services as funded by the appropriations in section 105 of
3 part 1 must be performance-based contracts that employ a client-
4 centered and results-oriented process that is based on measurable
5 performance indicators and desired outcomes and includes an annual
6 assessment of the quality of services provided.

7 (2) Not later than February 1 of the current fiscal year, the
8 department shall submit, to the standard report recipients, a
9 report detailing measurable performance indicators, desired
10 outcomes, and an assessment of the quality of services provided by
11 the department during the previous fiscal year.

12 Sec. 532. Beginning on October 1 of the current fiscal year,
13 the department shall hold semiannual meetings with state and
14 private residential providers to receive feedback and discuss
15 potential improvements to the residential system.

16 Sec. 534. Not later than March 1 of the current fiscal year,
17 the department shall submit, to the standard report recipients, a
18 report on the adoption subsidies expenditures from the previous
19 fiscal year. The report must include, but is not limited to, the
20 range of non-\$0.00 annual adoption support subsidy amounts, for
21 both Title IV-E eligible cases and state-funded cases, paid to
22 adoptive families; the number of Title IV-E and state-funded cases;
23 the number of cases in which an adoption support subsidy request by
24 an adoptive parent was denied by the department; and the number of
25 adoptive parents who requested a renegotiation of their adoption
26 support subsidy contract.

27 Sec. 537. Not later than March 1 of the current fiscal year,
28 the department shall submit, to the standard report recipients, a
29 report on the following information for cases of child abuse or



1 child neglect from the previous fiscal year:

2 (a) The total number of relative care placements.

3 (b) The total number of relative care placements into
4 unlicensed relative homes.

5 (c) The total number of relative care placements into licensed
6 relative homes.

7 (d) The total number of unlicensed relative providers with a
8 relative placement that were denied a foster home license due to
9 not meeting the standards established for foster care licensing in
10 this state.

11 (e) From a sample of cases, a list of the reasons documented
12 by the department for denial of relative foster home licensure.

13 (f) For licensed or approved relative caregivers with
14 placements, the status of Title IV-E claims for foster care
15 maintenance payments and foster care administrative payments.

16 Sec. 540. If a physician or psychiatrist who is providing
17 services to a state or court ward placed in a residential facility
18 submits a formal request to the department to change the
19 psychotropic medication for a ward, the department shall, if the
20 ward is a state ward, make a determination on the proposed change
21 not later than 7 business days after the request or, if the ward is
22 a temporary court ward, seek parental consent not later than 7
23 business days after the request. If the determination or parental
24 consent is not provided by the seventh business day, the department
25 shall petition the court for the determination or consent on the
26 eighth business day.

27 Sec. 546. (1) From the funds appropriated in part 1 for foster
28 care payments and from child care fund, the department shall pay an
29 administrative rate before incentive payments of not less than



1 \$60.20 to providers of general foster care, independent living, and
2 trial reunification services.

3 (2) From the funds appropriated in part 1, the department
4 shall pay providers of independent living plus services per diem
5 statewide rates for staff-supported housing at a rate of \$252.30
6 and host-home housing at a rate of \$119.95. The independent living
7 plus program provides staff-supported housing and services for
8 foster youth 16 years of age to 19 years of age who, because of
9 their individual needs and assessments, are not initially
10 appropriate for general independent living foster care.

11 (3) If required by the federal government to meet Title IV-E
12 requirements, on a quarterly basis, providers of foster care
13 services shall submit a report on expenditures to the department to
14 identify actual costs of providing foster care services.

15 Sec. 547. (1) From the funds appropriated in part 1 for the
16 guardianship assistance program, the department shall pay a minimum
17 rate that is not less than the approved age-appropriate payment
18 rates for youth placed in family foster care.

19 (2) The department shall submit, to the standard report
20 recipients, a report that includes quarterly data on the number of
21 children enrolled in the guardianship assistance and foster care -
22 children with serious emotional disturbance waiver programs.

23 Sec. 550. (1) The department shall not offset against
24 reimbursements to counties or seek reimbursement from counties for
25 charges that were received by the department more than 12 months
26 before the department seeks to offset against reimbursement. A
27 county shall not request reimbursement, and reimbursements must not
28 be paid, for a charge that is more than 12 months after the date of
29 service or original status determination when initially submitted



1 by the county.

2 (2) Not later than 12 months after a date of service, a
3 service provider shall submit a request for payment. A request for
4 payment submitted later than 12 months after the date of service
5 requires the provider to submit an exception request to the county
6 or the department for approval or denial.

7 (3) A county is not subject to any offset, chargeback, or
8 reimbursement liability for a prior expenditure resulting from an
9 error in a foster care fund source determination.

10 Sec. 551. Not later than 30 days after a county requests a
11 clarification through the department's child care fund management
12 unit email address, the department shall respond to the request.

13 Sec. 552. Sixty days after a county's child care fund review
14 is completed, including the receipt of all requested documentation
15 from the county, the department shall provide the results of the
16 review to the county. In the review, the department shall not
17 evaluate the relevancy, quality, effectiveness, efficiency, or
18 impact of the services provided to youth by the county's child care
19 fund programs. The department shall not release the results of a
20 county's child care fund review to a third party without the
21 permission of the county.

22 Sec. 554. From the funds appropriated in part 1 for foster
23 care payments, the department shall allocate \$50,000.00 to a
24 nonprofit organization organized under the laws of this state that
25 is exempt from federal income tax under section 501(c)(3) of the
26 internal revenue code of 1986, 26 USC 501; operates on a 100%
27 volunteer basis with a board of directors consisting of not more
28 than 15 members; is a dedicated community of individuals that give
29 their time, talent, and resources to provide the best quality



1 shopping environment that they can to local children in need; and
2 provides clothing, shoes, toys, linens, nursery furniture,
3 strollers, car seats, school supplies, hygiene products, and safety
4 equipment to local foster children and their families free of
5 charge.

6 Sec. 557. If a vehicle that is owned by the state is available
7 and not scheduled for use by other state workers, the department
8 may consider a children's protective services caseworker or a
9 foster care caseworker driving the vehicle to a foster home visit
10 or driving the vehicle to the caseworker's own home as an allowable
11 use of the vehicle if the driving would be helpful to the
12 caseworker in conducting the caseworker's work.

13 Sec. 559. (1) From the funds appropriated in part 1 for
14 adoption support services, not later than December 1 of the current
15 fiscal year, the department shall allocate \$500,000.00 to a grant
16 recipient to operate and expand its adoptive parent mentor program
17 to provide a listening ear, knowledgeable guidance, and community
18 connections to adoptive parents and children who were adopted in
19 this state or another state.

20 (2) Not later than March 1 of the current fiscal year, the
21 grant recipient shall submit, to the standard report recipients, a
22 report on the program described in subsection (1), including, but
23 not limited to, the number of cases served and the number of cases
24 in which the program prevented an out-of-home placement.

25 Sec. 562. If a foster parent transports a foster child to
26 parent-child visitation, the department shall reimburse the foster
27 parent for the foster parent's time and travel. As part of the
28 foster care parent contract, the department shall provide written
29 confirmation to foster parents that states that the foster parents



1 have the right to request reimbursement for all parent-child
2 visitations. Not later than 60 days after receiving a request from
3 a foster parent for eligible reimbursement, the department shall
4 provide the reimbursement. If the foster parent is unable to
5 transport a foster child to parent-child visitation, the private
6 child placing agency may provide transportation and shall receive
7 reimbursement from the department not later than 60 days after the
8 private child placing agency submits a request for eligible
9 reimbursement.

10 Sec. 564. (1) The department shall maintain a clear policy for
11 parent-child visitations. All of the following individuals shall
12 meet an 85% success rate, after accounting for factors outside of
13 caseworker control:

14 (a) Caseworkers and supervisors of local county offices.

15 (b) Caseworkers and supervisors of child placing agencies.

16 (2) In accordance with the court-ordered number of required
17 meetings between caseworkers and a parent, the caseworkers shall
18 achieve a success rate of 85%, after accounting for factors outside
19 of caseworker control.

20 (3) Not later than March 1 of the current fiscal year, the
21 department shall submit, to the standard report recipients, a
22 report on the following:

23 (a) The percentage of success rates for parent-child
24 visitations and court-ordered required meetings under subsections
25 (1) and (2) for the previous fiscal year.

26 (b) The barriers to achieve the success rates described in
27 subsections (1) and (2) and how this information is tracked.

28 Sec. 568. (1) The department shall ensure each youth
29 transitioning out of foster care is given assistance with obtaining



1 a driver license or state identification card and is issued a copy
2 of the youth's Social Security number, as required by department
3 policy. Assistance must be provided to each youth who is eligible
4 to obtain a driver license or state identification card and, based
5 on the youth's citizenship and legal residency status, a Social
6 Security card.

7 (2) Not later than April 1 of the current fiscal year, the
8 department shall submit, to the standard report recipients, a
9 report on the number of youth who obtained a driver license or
10 state identification card, the number of youth who obtained a
11 Social Security card, and the number of youth who were eligible but
12 did not receive a driver license, state identification card, or
13 Social Security card and an explanation as to why the youth did not
14 receive the documents.

15 Sec. 569. The department shall reimburse each private child
16 placing agency that completes an adoption at the rate on the date
17 when the petition for adoption and the required support
18 documentation were accepted by the court and not the rate on the
19 date when the court's order placing for adoption was entered.

20 Sec. 574. (1) From the funds appropriated in part 1 for foster
21 care payments, \$1,375,000.00 is allocated to support family
22 incentive grants to private and community-based foster care service
23 providers and relative caregivers for assistance with home
24 improvements to alleviate safety concerns or obtain items needed to
25 ensure compliance with licensing rule requirements and to
26 accommodate children in foster care.

27 (2) Not later than March 1 of the current fiscal year, the
28 department shall submit, to the standard report recipients, a
29 report on the total amount expended in the previous year for grants



1 to private and community-based foster care service providers for
2 home improvements or physical exams described in subsection (1) and
3 the number of grants issued.

4 Sec. 575. From the funds appropriated in part 1 for children's
5 services administration, the department shall allocate \$200,000.00
6 to provide support and coordinated services to the kinship
7 caregiver advisory council. The responsibilities of the council may
8 include all of the following:

9 (a) Establishing a public awareness campaign to educate the
10 public about kinship caregivers and this state's efforts to better
11 serve kinship caregivers.

12 (b) Consulting and coordinating with the kinship caregiver
13 navigator program to collect aggregate data on individuals being
14 served by the kinship caregiver navigator program, including
15 information on what services the individuals need.

16 (c) Consulting and collaborating with the provider of the
17 kinship caregiver navigator program on the design and
18 administration of the program.

19 (d) Establishing, maintaining, and updating a list of local
20 support groups and programs that provide services to kinship
21 families and, in order to obtain a better understanding of the
22 issues facing kinship families, devising a plan of action for
23 engaging with the groups and programs on the list.

24 (e) Developing methods to promote and improve collaboration
25 between state, county, and local governments and agencies and
26 private stakeholders for all of the following reasons:

27 (i) To obtain a broad understanding of the characteristics and
28 prevalence of kinship caregiving.

29 (ii) To improve service delivery.



1 (iii) To include the methods in the council's recommendations.

2 Sec. 578. (1) From the funds appropriated in part 1 for foster
3 care payments, the department shall allocate Title IV-E passthrough
4 funds for educational programs to strengthen this state's child
5 welfare workforce. The department shall enter into contractual
6 arrangements with 1 or more state universities to provide bachelor
7 of social work and master of social work educational training,
8 including field placements and stipends for tuition and educational
9 expenses. In exchange, students completing eligible educational
10 programs are contractually obligated to work for Michigan child
11 welfare agencies for a minimum of 4 months for every semester they
12 receive the stipend. The matching funds for the Title IV-E funds
13 must be provided by the participating state universities from the
14 expenses incurred for training child welfare students who
15 participate in the program.

16 (2) Not later than March 1 of the current fiscal year, the
17 department shall submit, to the standard report recipients, a
18 report on the status of pilot programs under subsection (1) that
19 includes, but is not limited to, the total number of applicants,
20 the total number of program participants, a list of state
21 universities that participated in the pilot programs, and the total
22 amount of matching funds that each state university contributed to
23 the programs.

24 Sec. 581. From the funds appropriated in part 1 for foster
25 care payments, the department shall allocate at least \$50,000.00
26 for caseworkers to provide immediate assistance with urgent needs,
27 including, but not limited to, food, clothing, and other basic
28 necessities, for children, including children who are victims of
29 human trafficking, on the children's removal from the children's



1 homes or other dangerous environments.

2 Sec. 583. Not later than March 1 of the current fiscal year,
3 the department shall submit, to the standard report recipients and
4 the senate and house of representatives standing committees that
5 cover subject matters dealing with families and human services, a
6 report that includes all of the following:

7 (a) The number and percentage of foster parents that closed
8 their license in the previous fiscal year, the reasons the foster
9 parents left, and how the figures compare to the figures for prior
10 fiscal years.

11 (b) The number and percentage of foster parents successfully
12 retained in the previous fiscal year and how the figures compare to
13 the figures for prior fiscal years.

14 (c) The number and percentage of licensed foster homes that
15 closed their license because they adopted their foster child based
16 on survey data from foster parents closing the foster parents'
17 licenses.

18 Sec. 585. Each month, the department shall make available at
19 least 1 pre-service training class in which new caseworkers for
20 private foster care and adoption agencies can enroll.

21 Sec. 588. (1) Concurrently with public release, the department
22 shall transmit, without revision, all reports from the court-
23 appointed settlement monitor, including, but not limited to, the
24 needs assessment and period outcome reporting, to the standard
25 report recipients.

26 (2) Not later than October 1 of the current fiscal year, the
27 department shall submit, to the standard report recipients, a
28 detailed plan that addresses the status and progress toward exiting
29 the settlement by September 30 of the current fiscal year. The



1 report must include an update on the department's child welfare
2 initiative.

3 Sec. 589. (1) From the funds appropriated in part 1 for child
4 care fund, the department shall pay 100% of the administrative rate
5 for all new cases referred to providers of foster care services.

6 (2) On a quarterly basis, the department shall submit a
7 report, to the standard report recipients, on the monthly number of
8 all foster care cases administered by the department and all foster
9 care cases administered by private providers.

10 Sec. 592. On a quarterly basis, the department shall submit,
11 to the chairs of the senate and house of representatives standing
12 oversight committees and the standard report recipients, a report
13 that includes data from children's protective services staff for
14 each of the following for the most recent quarter before the
15 applicable report is submitted:

16 (a) The percent of investigations commenced in 24 hours
17 immediately after receiving a report.

18 (b) The percent of central registry reviews performed for
19 required individuals.

20 (c) The percent of face-to-face contacts made within the
21 established timeframe required by the department.

22 (d) In appropriate cases, the percent of sibling placement
23 evaluations completed when 1 or more children remain in the home
24 after a child has been removed.

25 (e) The percent of supervisory reviews performed in a timely
26 manner.

27 (f) The results of a department survey of children's
28 protective services investigators on the number of investigators
29 who are concerned for their own personal safety.



1 (g) The percent of investigators using the mobile application
2 or another tool to document compliance.

3 Sec. 593. The department shall conduct an annual review in
4 each county to determine if the county has adopted and implemented
5 standard child abuse and child neglect investigation and interview
6 protocols under section 8(6) of the child protection law, 1975 PA
7 238, MCL 722.628.

8 Sec. 594. From the funds appropriated in part 1 for foster
9 care payments, the department shall support regional resource teams
10 to provide for the recruitment, retention, and training of foster
11 and adoptive parents and shall expand the Michigan youth
12 opportunities initiative to all counties of this state. The purpose
13 of the funding is to increase the number of annual inquiries from
14 prospective foster parents, increase the number of nonrelative
15 foster homes that achieve licensure each year, increase the annual
16 retention rate of nonrelative foster homes, reduce the number of
17 older foster youth placed outside of family settings, and provide
18 older youth with enhanced support in transitioning to adulthood.

19 Sec. 598. Partial child care fund reimbursements to counties
20 for undisputed charges must not be made later than 45 business days
21 after receipt of the required forms and documentation. Not later
22 than 15 business days after receiving a request from a county for
23 reimbursement of a disputed charge, the department shall commence
24 activity to investigate and resolve the disputed reimbursement
25 charge. The activity to investigate and resolve a disputed
26 reimbursement charge may include, but is not limited to, the use of
27 a formal appeals process under applicable law and the department
28 chargeback policy. Not later than 45 business days after a properly
29 corrected submission by the county, the department shall reimburse



1 the county for the corrected charge or charges.

2
3 **PUBLIC ASSISTANCE**

4 Sec. 601. After a client agrees to the release of the client's
5 name and address to the local housing authority, the department
6 shall request from the local housing authority information
7 regarding whether the housing unit for which vendoring has been
8 requested meets applicable local housing codes. Vendoring must be
9 terminated if the local housing authority indicates in writing that
10 the unit does not meet local housing codes and until the local
11 housing authority indicates in writing that the local housing codes
12 have been met.

13 Sec. 602. The department shall conduct a full evaluation of an
14 individual's assistance needs if the individual has applied for
15 disability more than 1 time in a 1-year period.

16 Sec. 603. For any change in the income of a recipient of the
17 food assistance program, the family independence program, or state
18 disability assistance that results in a benefit decrease, the
19 department shall notify the recipient of the amount of the decrease
20 not later than 15 work days before the first day of the month in
21 which the decrease takes effect.

22 Sec. 604. (1) From the funds appropriated in part 1 for state
23 disability assistance payments, the department shall operate a
24 state disability assistance program. Except as provided in
25 subsection (3), to be eligible for the program, an individual must
26 be a needy citizen of the United States or alien exempted from the
27 SSI citizenship requirement who is not less than 18 years of age,
28 or an emancipated minor, and meets 1 or more of the following
29 requirements:



1 (a) Is a recipient of SSI, Social Security, or medical
2 assistance due to disability or being 65 years of age or older.

3 (b) Is an individual with a physical or mental impairment that
4 meets federal SSI disability standards, except that the minimum
5 duration of the disability must be 90 days. Substance use disorder
6 alone is not a basis for eligibility.

7 (c) Is a resident of an adult foster care facility, a home for
8 the aged, a county infirmary, or a substance use disorder treatment
9 center.

10 (d) Is an individual receiving 30-day postresidential
11 substance use disorder treatment.

12 (e) Is an individual diagnosed as having AIDS.

13 (f) Is an individual receiving special education services
14 through a local intermediate school district.

15 (g) Is a caretaker of a disabled individual who meets the
16 requirements specified in subdivision (a), (b), (e), or (f).

17 (2) An applicant for or recipient of state disability
18 assistance is considered needy if the applicant or recipient does
19 both of the following:

20 (a) Meets the same asset test as is applied for the family
21 independence program.

22 (b) Has a monthly budgetable income that is less than the
23 payment standards.

24 (3) Except for an individual described in subsection (1)(c) or
25 (d), an individual is not disabled under this section if the
26 individual's drug addiction or alcoholism is a contributing factor
27 material to the determination of disability.

28 (4) As used in this section:

29 (a) "Material to the determination of disability" means that,



1 if the individual stopped using drugs or alcohol, the individual's
2 remaining physical or mental limitations would not be disabling. If
3 the individual's remaining physical or mental limitations would be
4 disabling, then the drug addiction or alcoholism is not material to
5 the determination of disability and the individual may receive
6 state disability assistance, but the individual must actively
7 participate in a substance abuse treatment program, and the
8 assistance must be paid to a third party or through vendor
9 payments.

10 (b) "Substance abuse treatment" includes receipt of inpatient
11 or outpatient services or participation in Alcoholics Anonymous or
12 a similar program.

13 Sec. 605. The level of reimbursement provided to state
14 disability assistance recipients in licensed adult foster care
15 facilities must be the same as the prevailing SSI rate under the
16 personal care category.

17 Sec. 606. County department offices shall require each
18 recipient of family independence program and state disability
19 assistance who has applied with the Social Security Administration
20 for SSI to sign a contract to repay any assistance rendered through
21 the family independence program or state disability assistance
22 program on receipt of retroactive SSI benefits.

23 Sec. 607. (1) The department's ability to satisfy
24 appropriation deductions in part 1 for state disability
25 assistance/supplemental security income recoveries and public
26 assistance recoupment revenues is not limited to recoveries and
27 accruals pertaining to state disability assistance, or family
28 independence program grant payments provided only in the current
29 fiscal year and may include revenues collected during the current



1 year that are prior-year-related and not a part of the department's
2 accrued entries.

3 (2) The department may use SSI recoveries to satisfy the
4 deduct in any line in which the revenues are appropriated,
5 regardless of the source from which the revenue is recovered.

6 Sec. 608. An adult foster care facility that provides
7 domiciliary care or personal care to a resident receiving SSI or a
8 home for the aged serving a resident receiving SSI shall not
9 require a resident described in this section to reimburse the home
10 for the aged or adult foster care facility for care at a rate in
11 excess of a rate that is authorized by the legislature. To the
12 extent permitted by federal law, an adult foster care facility and
13 home for the aged that serves a resident receiving SSI is not
14 prohibited from accepting a third-party payment in addition to SSI
15 if the payment is not for food, clothing, or shelter, or would
16 result in a reduction in the resident's SSI payment.

17 Sec. 609. The department shall not reduce the state
18 supplementation level under the SSI program for the personal
19 care/adult foster care and home for the aged categories during the
20 current fiscal year. Not later than 30 days before a proposed
21 reduction in the state supplementation level, the department shall
22 notify the legislature of the proposed reduction.

23 Sec. 610. (1) The department shall grant an exemption from the
24 good-cause criteria for the state emergency relief program if an
25 emergency results from an unexpected expense related to maintaining
26 or securing employment.

27 (2) In determining housing affordability eligibility for state
28 emergency relief, a group is considered to have sufficient income
29 to meet ongoing housing expenses if the group's total housing



obligation does not exceed 75% of the group's total net income.

(3) The department shall not make a state emergency relief payment to an individual who has been found guilty of fraud in obtaining public assistance.

(4) The department shall not make a state emergency relief payment to an individual who is an out-of-state or nonlegal resident.

(5) The department shall distribute a state emergency relief payment for rent assistance directly to a landlord and shall not add the payment to a Michigan bridge card.

Sec. 611. The state supplementation level under the SSI program for the living independently category or living in the household of another category must not exceed the minimum state supplementation level as required under federal law.

Sec. 613. (1) From the funds appropriated in part 1 for indigent burial, the department shall provide a reimbursement for the final disposition of an indigent individual. A reimbursement under this section must comply with all of the following:

(a) The maximum allowable reimbursement for the final disposition is \$960.00.

(b) The adult burial with services allowance is \$875.00.

(c) The adult burial without services allowance is \$610.00.

(d) The infant burial allowance is \$240.00.

(e) The adult cremation with services allowance is \$640.00.

(f) The adult cremation without services allowance is \$390.00.

(g) The maximum allowable reimbursement if an irrevocable funeral agreement exists is \$260.00.

(2) The department shall reimburse up to \$80.00 for a cremation permit fee and for mileage at the standard rate for an



1 eligible cremation. A reimbursement under this subsection must take
2 into consideration whether an indigent individual's religious
3 preference prohibits cremation.

4 (3) An application for burial services must be made no later
5 than 20 business days after the burial, cremation, or donation
6 takes place. A friend or relative of the indigent individual may
7 supplement the burial payment in any amount up to \$6,000.00 for
8 additional services. A funeral director, with written authorization
9 provided by a relative of the indigent individual, is deemed an
10 authorized representative for burial benefits.

11 (4) By January 31 of the current fiscal year, the department
12 shall submit a report to the standard report recipients on burial
13 service payments issued from the state emergency relief program
14 during the previous fiscal year. The report must include the number
15 of applicants denied and the number of payments by the following
16 burial service categories:

- 17 (a) Fetus or infant less than 1 month of age.
- 18 (b) Burial with memorial service.
- 19 (c) Burial without memorial service.
- 20 (d) Cremation with memorial service.
- 21 (e) Cremation without memorial service.
- 22 (f) Transportation of a donated or unclaimed body being
23 cremated.
- 24 (g) Cremation permit fee for an unclaimed body.
- 25 (h) Disposition of an unclaimed body.
- 26 (i) Payment if an irrevocable funeral agreement exists.
- 27 (j) An unclaimed body received by a university.

28 Sec. 614. By January 15 of the current fiscal year, the
29 department shall submit a report to the standard report recipients



1 on the number and percentage of state disability assistance
2 recipients who were determined to be eligible for federal SSI
3 benefits in the previous fiscal year.

4 Sec. 615. Except as required by federal law, the department
5 shall not use funds appropriated in part 1 to provide public
6 assistance to an individual who is not a United States citizen,
7 permanent resident alien, or refugee. This section does not
8 prohibit the department from entering into a contract with a food
9 bank, emergency shelter provider, or another human service agency
10 that may, as a normal part of doing business, provide food or
11 emergency shelter.

12 Sec. 616. The department shall require a retailer that
13 participates in the electronic benefits transfer program to charge
14 no more than a \$2.50 fee for cash back as a condition of
15 participation.

16 Sec. 619. The department shall not deny a title IV-A
17 assistance and food assistance benefit under 21 USC 862a to an
18 individual who has been convicted of a felony for the possession,
19 use, or distribution of a controlled substance, if both of the
20 following are met:

21 (a) The act that resulted in the conviction occurred after
22 August 22, 1996.

23 (b) The individual is not in violation of the individual's
24 probation or parole requirements.

25 Sec. 620. (1) The department shall determine a Medicaid
26 applicant's Medicaid eligibility not later than 90 days after the
27 Medicaid applicant completes a Medicaid application if the Medicaid
28 applicant's disability is an eligibility factor. For other Medicaid
29 applicants, including an applicant who is a patient of a nursing



1 home, the department shall determine the applicant's Medicaid
2 eligibility within 45 days after receiving the Medicaid applicant's
3 application.

4 (2) On a quarterly basis, the department shall submit a report
5 to the standard report recipients on the number of recipients who
6 were ineligible for Medicaid after Medicaid eligibility
7 redeterminations. The report must include, in a monthly data
8 format, the number of recipients who had their eligibility examined
9 directly, through an ex parte eligibility process or through a
10 passive eligibility process. The report must also include a copy of
11 each baseline and monthly report that the department provides to
12 CMS for unwinding data reporting and the number of recipients who
13 did not respond to the department through eligibility outreach or
14 data requests.

15 Sec. 625. From the funds appropriated in part 1 for SSI
16 advocacy legal services grant, the department shall allocate
17 \$975,000.00 as a grant to the Michigan State Bar Foundation. The
18 purpose of the grant is to assist current or potential recipients
19 of state disability assistance who have applied for or wish to
20 apply for SSI or other federal disability benefits. The Michigan
21 State Bar Foundation shall provide a list of newly eligible SSI
22 recipients to the department to verify that services are provided
23 to department referrals.

24 Sec. 645. The department shall consider an individual or
25 family to be homeless for purposes of eligibility for state
26 emergency relief, if the individual or family is living temporarily
27 with another in order to escape domestic violence. The department
28 shall define and verify domestic violence in the same manner as the
29 department defines and verifies that term in the department's



1 policies on good cause for not cooperating with child support and
2 paternity requirements.

3 Sec. 653. From the funds appropriated in part 1 for food
4 assistance program benefits, an individual who is the victim of
5 domestic violence or human trafficking and who does not qualify for
6 any other exemption may be exempt from the 3-month in 36-month
7 limit on receiving food assistance under 7 USC 2015. The department
8 may extend the exemption for an additional 3 months if an
9 individual described in this section demonstrates to the department
10 a continuing need.

11 Sec. 654. The department shall notify a recipient of food
12 assistance program benefits that the recipient's benefits can be
13 spent with the recipient's Michigan bridge card at many farmers
14 markets in this state. The department shall also provide a
15 recipient with information about the double up food bucks program
16 that is administered by the Fair Food Network. The information
17 about the double up food bucks program must include, but is not
18 limited to, information that if the recipient spends \$20.00 at a
19 participating farmers market through the program, the recipient may
20 receive an additional \$20.00 to buy Michigan produce.

21 Sec. 655. Not later than 14 days after the spending plan for
22 low-income home energy assistance program is approved by the state
23 budget office, the department shall provide the spending plan,
24 including itemized projected expenditures and itemized expenditures
25 for the previous fiscal year, to the standard report recipients.

26 Sec. 669. From the funds appropriated in part 1 for family
27 independence program - clothing allowance, the department shall
28 allocate \$10,000,000.00 for the annual clothing allowance. The
29 department shall grant the allowance to eligible children in a



1 family independence program group.

2 Sec. 672. (1) By February 15 of the current fiscal year, the
3 department's office of inspector general shall submit a report to
4 the standard report recipients on the department's efforts to
5 reduce the inappropriate use of Michigan bridge cards and food
6 assistance program trafficking. The department shall provide
7 information on the number of recipients of services who used their
8 Michigan bridge card inappropriately and the current status of each
9 case, the number of recipients whose benefits were permanently and
10 temporarily revoked as a result of inappropriately using their
11 Michigan bridge cards, and the number of retailers that were fined
12 or removed from the electronic benefit transfer program for
13 permitting the inappropriate use of Michigan bridge cards. The
14 report must also include the number of Michigan bridge card
15 trafficking instances and overall welfare fraud referrals, that
16 includes, but is not limited to, information on the number of
17 investigations completed, fraud and intentional program violation
18 dollar amounts identified, the number of referrals to prosecutors,
19 the number of administrative hearing referrals and waivers, and the
20 number of program disqualifications imposed. The report must
21 distinguish between savings and cost avoidance. As used in this
22 subsection:

23 (a) "Cost avoidance" includes expenditures avoided due to
24 front-end eligibility investigations and other preemptive actions
25 undertaken in the prevention of fraud.

26 (b) "Savings" includes receivables established from instances
27 of fraud committed.

28 (2) If a fourth Michigan bridge card has been issued to a
29 household in a 12-month period, the department shall notify the



1 household that the household has reached the number of issued cards
2 threshold. At a household's fifth and each subsequent card
3 replacement request, a card will not be issued until a recipient
4 from the household has spoken directly to the local office district
5 manager or county director. The district manager or county director
6 may issue a new Michigan bridge card based on the district
7 manager's or county director's assessment of the recipient's
8 situation and the recipient's explanation.

9 (3) As used in this section:

10 (a) "Food assistance trafficking" means the buying and selling
11 of food assistance benefits for cash or items not authorized under
12 7 USC 2036b.

13 (b) "Inappropriate use" means not used to meet a family's
14 ongoing basic needs, including, but not limited to, food, clothing,
15 shelter, utilities, household goods, personal care items, and
16 general incidentals.

17 Sec. 677. (1) The department shall establish a state goal for
18 the percentage of family independence program cases involved in
19 employment activities. The percentage established must not be less
20 than 50%. The goal for long-term employment must be 15% of cases
21 for 6 months or more.

22 (2) The department shall submit an annual report, providing
23 quarterly data, to the standard report recipients on the number of
24 cases referred to PATH, the current percentage of family
25 independence program cases involved in PATH employment activities,
26 an estimate of the current percentage of family independence
27 program cases that meet federal work participation requirements on
28 the whole, and an estimate of the current percentage of the family
29 independence program cases that meet federal work participation



1 requirements for those cases referred to PATH.

2 (3) The department shall submit a report to the standard
3 report recipients. The report must include quarterly data on all of
4 the following:

5 (a) The number and percentage of nonexempt family independence
6 program recipients who are employed.

7 (b) The average and range of wages of employed family
8 independence program recipients.

9 (c) The number and percentage of employed family independence
10 program recipients who remain employed for 6 months or more.

11 Sec. 678. (1) From the funds appropriated in part 1 for family
12 independence program - child supplemental payment, the department
13 shall allocate \$16,240,100.00 of TANF revenue to provide a
14 supplemental payment for the current fiscal year for each child
15 under 6 years of age within a family receiving cash assistance. Not
16 later than November 30 of the current fiscal year, the department
17 shall distribute an equal payment based on the funds available in
18 part 1 and the total number of children under 6 years of age who
19 are within a family receiving cash assistance.

20 (2) From the funds appropriated in part 1 for family
21 independence program - child supplemental payment, the department
22 shall allocate \$7,000,000.00 of TANF revenue to provide a
23 supplemental payment for the current fiscal year for each child 6
24 years of age or older but under 14 years of age within a family
25 receiving cash assistance. Not later than November 30 of the
26 current fiscal year, the department shall distribute an equal
27 payment based on the funds available in part 1 and the total number
28 of children who are 6 years of age or older but under 14 years of
29 age within a family receiving cash assistance.



1 (3) By February 1 of the current fiscal year, the department
2 shall submit a report to the standard report recipients on the
3 amount of funding distributed under this section and shall include
4 the number of family independence program cases, the number of
5 family independence program eligible children by age group, and the
6 amount of funding distributed by age category.

7 Sec. 686. (1) The department shall confirm that an individual
8 who presents a personal identification issued by another state and
9 is seeking assistance through the family independence program, food
10 assistance program, state disability assistance program or medical
11 assistance program is not receiving benefits from another state.

12 (2) The department shall confirm the address provided by an
13 individual who is seeking family independence program benefits or
14 state disability assistance benefits.

15 (3) The department shall prohibit an individual who has
16 property assets assessed at a value higher than \$200,000.00 from
17 receiving assistance through a department-administered program,
18 unless prohibiting assistance would violate a federal law or
19 guideline.

20 (4) The department shall make a reasonable attempt to obtain
21 an up-to-date telephone number for an individual seeking medical
22 assistance benefits during the eligibility determination or
23 redetermination process for the individual.

24 Sec. 687. (1) On a quarterly basis, the department shall
25 compile and make available a report on its website that contains
26 all of the following information about the family independence
27 program, state disability assistance, the food assistance program,
28 indigent burial, Medicaid, and state emergency relief:

29 (a) The number of applications received.



1 (b) The number of applications approved.

2 (c) The number of applications denied.

3 (d) The number of applications pending and neither approved
4 nor denied.

5 (e) The number of cases opened.

6 (f) The number of cases closed.

7 (g) The number of cases at the beginning of the quarter and
8 the number of cases at the end of the quarter.

9 (2) The department shall compile and make the information
10 provided under subsection (1) available for this state as a whole
11 and for each county and shall report the information separately for
12 each program listed in subsection (1).

13 (3) On a quarterly basis, the department shall compile and
14 make available a report on its website of the following family
15 independence program information:

16 (a) The number of new applicants who successfully met the
17 requirements of the 10-day assessment period for PATH.

18 (b) The number of new applicants who did not meet the
19 requirements of the 10-day assessment period for PATH.

20 (c) The number of cases sanctioned because of a school truancy
21 policy.

22 (d) The number of cases closed because of the lifetime limits.

23 (e) The number of first-, second-, and third-time sanctions.

24 (f) The number of children 0 to 5 years of age who are living
25 in a family independence program-sanctioned household.

26 Sec. 689. (1) From the funds appropriated in part 1 for
27 prenatal and infant support program, the department shall allocate
28 \$20,000,000.00 of TANF revenue for programs that are intended to
29 improve the economic stability of households with very young



1 children.

2 (2) In allocating the funds referenced in subsection (1), the
3 department shall give preference to programs that demonstrate the
4 following:

5 (a) Effectiveness in improving the economic stability of
6 households with pregnant women at a minimum of 20 weeks gestation,
7 and with young children.

8 (b) Partnerships with local health care providers and
9 nonprofit human service agencies that provide for improved maternal
10 and infant health outcomes.

11 (c) Compliance with TANF requirements established by the
12 Administration for Children and Families within the United States
13 Department of Health and Human Services.

14 (3) By September 30 of the current fiscal year, the
15 department, through agreements with contracted implementing
16 agencies, shall report to the standard report recipients
17 information for the previous fiscal year on the aggregated
18 demographic data of all program recipients regardless of underlying
19 funding source. The report must include, but not be limited to,
20 aggregated recipient data from contracted implementing agencies
21 with each contracted implementing agency providing the age, race,
22 ethnicity, Hispanic or Latino origin, federal poverty level,
23 funding source, and zip codes of all program recipients.

24
25 **CHILDREN'S SERVICES AGENCY - JUVENILE JUSTICE**

26 Sec. 701. Unless required by a change to federal law or the
27 law of this state or at the request of a provider, the department
28 shall not alter the terms of a signed contract with a private
29 residential facility that serves children who are under state or



1 court supervision without receiving written consent from a
2 representative of the private residential facility.

3 Sec. 702. (1) Not later than December 10 of the current fiscal
4 year, the department shall submit a report to the standard report
5 recipients on all of the following for the prior fiscal year:

6 (a) The average daily population by month of youth residing at
7 state-run juvenile justice facilities.

8 (b) The total number of beds at each facility, separated by
9 those that are staffed and unstaffed, for each month.

10 (c) The average daily number of available beds by month.

11 (d) The average number of staff vacancies by month.

12 (2) The information in subsection (1) must be itemized by each
13 facility.

14 Sec. 706. A county is subject to a 50% chargeback for the use
15 of an alternative regional detention service, if the detention
16 service does not fall under the basic grant provision of section
17 117e of the social welfare act, 1939 PA 280, MCL 400.117e, or if a
18 county operates the detention service program primarily with
19 professional rather than volunteer staff.

20 Sec. 707. To be reimbursed for child care fund expenditures, a
21 county shall submit to the department the report required under
22 section 117a(11) of the social welfare act, 1939 PA 280, MCL
23 400.117a, to enable the department to document a potential
24 federally claimable expenditure.

25 Sec. 708. (1) As a condition of receiving funds appropriated
26 in part 1 for the child care fund line item, by October 15 of the
27 current fiscal year, a county shall have an approved service
28 spending plan for the current fiscal year. Not later than August 15
29 of the current fiscal year, a county shall submit the county's



1 service spending plan for the following fiscal year to the
2 department for approval. The department shall approve a county's
3 service spending plan not later than 30 calendar days after the
4 department receives a properly completed service spending plan from
5 the county that complies with the requirements of the social
6 welfare act, 1939 PA 280, MCL 400.1 to 400.119b. The department
7 shall notify and submit revisions to a service spending plan to a
8 county whose service spending plan is not approved after initial
9 submission. The department shall not request any additional
10 revisions to a county's service spending plan outside of the
11 requested revision notification submitted to the county by the
12 department. The department shall notify a county that its service
13 spending plan is approved not later than 30 days after the
14 department considers the county's revisions to the county's service
15 spending plan.

16 (2) A county shall submit an amendment to its county service
17 spending plan for the current fiscal year to the department not
18 later than August 30 of the current fiscal year. A county shall
19 submit payable estimates for the current fiscal year to the
20 department not later than September 15 of the current fiscal year.

21 (3) Not later than February 15 of the current fiscal year, the
22 department shall submit a report to the standard report recipients
23 on the number of counties that fail to submit a service spending
24 plan by August 15 of the previous fiscal year and the number of
25 service spending plans not approved by October 15. The report must
26 include the number of county service spending plans that were not
27 initially approved by the department and the number of service
28 spending plans that were not approved by the department after being
29 resubmitted by the county after revisions were requested by the



1 department under subsection (1).

2 Sec. 709. The department's master contract for juvenile
3 justice residential foster care services must prohibit a contractor
4 from denying a referral for placing a youth, or terminating a
5 youth's placement, if the youth's assessed treatment needs are in
6 alignment with the facility's residential program type, as
7 identified by a court or the department. The master contract must
8 also require that a youth placed in a juvenile justice residential
9 foster care facility has regularly scheduled treatment sessions
10 with a licensed clinician, regularly scheduled reviews with a
11 licensed psychiatrist for medication management, and has access to,
12 or regular sessions with a licensed psychologist or a psychiatrist,
13 as clinically indicated.

14
15 **LOCAL OFFICE OPERATIONS AND SUPPORT SERVICES**

16 Sec. 801. The department shall submit a quarterly report that
17 contains monthly data to the standard report recipients on the most
18 recent food assistance program error rate derived from the active
19 cases, reported to the United States Department of Agriculture Food
20 and Nutrition Service for the supplemental nutrition assistance
21 program.

22 Sec. 807. From the funds appropriated in part 1 for Elder Law
23 of Michigan MiCAFE contract, the department shall allocate not less
24 than \$450,000.00 to the Elder Law of Michigan MiCAFE to assist this
25 state's elderly population in participating in the food assistance
26 program. Of the \$450,000.00 allocated under this section, the
27 department shall use \$225,000.00 of general fund/general purpose
28 revenue as state matching funds to receive not less than
29 \$225,000.00 in funding from the United States Department of



1 Agriculture to provide outreach program activities as part of a
2 statewide food assistance hotline. The outreach program activities
3 may include eligibility screening and information services.

4 Sec. 825. (1) From the funds appropriated in part 1, the
5 department shall provide an individual with not more than \$2,000.00
6 for vehicle repairs, including a repair done in the previous 12
7 months. The \$2,000.00 limit described in this section includes the
8 combined total of payments made by the department and the work
9 participation program.

10 (2) By February 1 of the current fiscal year, the department
11 shall submit a report to the standard report recipients that
12 details the total amount of funding distributed and the total
13 number of payments made for vehicle repairs.

14 Sec. 826. (1) From the funds appropriated in part 1 for local
15 office policy and administration, not less than \$300,000.00 is
16 allocated for the department to contract with the Prosecuting
17 Attorneys Association of Michigan to provide the support and
18 services necessary to increase the capability of this state's
19 prosecutors, adult protective service system, and criminal justice
20 system to effectively identify, investigate, and prosecute elder
21 abuse and financial exploitation.

22 (2) Not later than March 1 of the current fiscal year, the
23 Prosecuting Attorneys Association of Michigan shall submit a report
24 to the department on the efficacy of the contract. The department
25 shall submit the report to the standard report recipients not later
26 than 30 days after the department receives the report from the
27 Prosecuting Attorneys Association of Michigan.

28 Sec. 850. (1) The department shall maintain each out-stationed
29 eligibility specialist in a community-based organization, community



1 mental health agency, nursing home, adult placement and independent
2 living setting, FQHC, and hospital, unless the community-based
3 organization, community mental health agency, nursing home, adult
4 placement and independent living setting, FQHC, or hospital
5 requests to discontinue the positions at its facility.

6 (2) From the funds appropriated in part 1 for donated funds
7 positions, the department shall enter into a contract with any
8 agency that is able and eligible under federal law to provide the
9 required matching funds for federal funding, as determined by
10 federal law.

11 (3) A contract for a donated funds position for assistance
12 payments must include, but not be limited to, performance metrics
13 on both of the following topics:

14 (a) Meeting a standard of promptness for processing an
15 application for Medicaid and other public assistance programs under
16 the law of this state.

17 (b) Meeting required standards for error rates in determining
18 programmatic eligibility, as determined by the department.

19 (4) The department shall fill an additional donated funds
20 position only after a new contract has been signed with an agency.
21 The position must be abolished when the contract expires or is
22 terminated.

23 (5) The department shall classify as a limited-term FTE a new
24 employee who is hired to fill a donated funds position contract or
25 is hired to fill a vacancy from an employee who transferred to a
26 donated funds position.

27 (6) By March 1 of the current fiscal year, the department
28 shall submit a report to the standard report recipients detailing
29 information on the donated funds positions. The report must



1 include, but is not limited to, the total number of occupied
2 positions, the total private contribution of the positions, and the
3 total cost to this state for a nonsalary expenditure for the
4 donated funds position employees.

5
6 **DISABILITY DETERMINATION SERVICES**

7 Sec. 890. From the funds appropriated in part 1 for disability
8 determination services, the department shall maintain the unit
9 rates in effect on September 30, 2019 for medical consultants
10 performing disability determination services, including physicians,
11 psychologists, and speech-language pathologists.

12
13 **BEHAVIORAL HEALTH PROGRAM ADMINISTRATION AND SPECIAL PROJECTS**

14 Sec. 901. The department shall use the funds appropriated in
15 part 1 to support a system of comprehensive community mental health
16 services under the full authority and responsibility of local
17 CMHSPs or PIHPs in accordance with the mental health code, 1974 PA
18 258, MCL 330.1001 to 330.2106, the Medicaid provider manual,
19 federal Medicaid waivers, and all other applicable federal law and
20 the law of this state.

21 Sec. 902. (1) From the funds appropriated in part 1, the
22 department shall make a final authorization to a CMHSP or PIHP on
23 the execution of a contract between the department and the CMHSP or
24 PIHP. The contract must contain an approved plan and budget and any
25 policy and procedure governing the obligations and responsibilities
26 of each party to the contract. Each contract with a CMHSP or PIHP
27 that the department is authorized to enter into under this
28 subsection must include a provision that the contract is not valid
29 unless the total dollar obligation for all of the contracts between



1 the department and the CMHSPs or PIHPs entered into under this
2 subsection for the current fiscal year does not exceed the amount
3 of money appropriated in part 1 for the contracts authorized under
4 this subsection.

5 (2) The department shall immediately submit a report to the
6 standard report recipients if either of the following occurs:

7 (a) The department enters into a new contract with a CMHSP or
8 PIHP that would affect a rate or expenditure.

9 (b) The department amends a contract that the department has
10 entered into with a CMHSP or PIHP that would affect a rate or
11 expenditure.

12 (3) The report required by subsection (2) must include
13 information about any changes to the contract and the change's
14 effects on rates and expenditures.

15 Sec. 904. (1) Not later than September 30 of the current
16 fiscal year, the department shall provide a report on the CMHSPs,
17 PIHPs, and designated regional entities for substance use disorder
18 prevention and treatment to the standard report recipients that
19 includes the information required by this section.

20 (2) The report required under subsection (1) must contain,
21 unless otherwise noted, information for each CMHSP and PIHP and a
22 statewide summary, as follows:

23 (a) A statewide summary of the demographic description of
24 service recipients that, minimally, includes reimbursement
25 eligibility, client population group, age, ethnicity, housing
26 arrangements, and diagnosis.

27 (b) Per capita expenditures in total and by client population
28 group.

29 (c) A statewide summary of Medicaid-funded cost information



1 for the 3 diagnosis groups of adults with a mental illness,
2 children with a serious emotional disturbance, and individuals with
3 an intellectual or developmental disability. The statewide summary
4 must, minimally, include expenditures by service category for each
5 of the 3 diagnosis groups described in this subdivision and cases,
6 units, and cost of each specific service code index or health care
7 common procedure coding system code for each of the 3 diagnosis
8 groups.

9 (d) Financial information on non-Medicaid mental health
10 services by general fund cost reporting category.

11 (e) Information about access to each CMHSP, PIHP, and
12 designated regional entity for substance use disorder prevention
13 and treatment, that includes, but is not limited to, all of the
14 following:

15 (i) The number of individuals receiving requested services.

16 (ii) The number of individuals who requested services but did
17 not receive services.

18 (f) The number of second opinions requested under the mental
19 health code, 1974 PA 258, MCL 330.1001 to 330.2106, and the
20 determination of any appeals.

21 (g) Lapses and carryforwards during the previous fiscal year
22 for each CMHSP, PIHP, and designated regional entity for substance
23 use disorder prevention and treatment.

24 (h) Performance indicator information required to be submitted
25 to the department in the contracts with each PIHP.

26 (i) Administrative expenditures of each CMHSP and PIHP that
27 include a breakout of the salary, benefits, and pension of each
28 executive-level staff, which includes, but is not limited to, the
29 director, chief executive, and chief operating officer.



1 (3) The report required under subsection (1) must contain the
2 following information from the previous fiscal year on substance
3 use disorder prevention, education, and treatment programs:

4 (a) A statewide summary of the demographic description of
5 service recipients that, minimally, must include reimbursement
6 eligibility, primary substance of abuse, age, ethnicity, housing
7 arrangements, and sex at birth.

8 (b) The expenditures stratified by department-designated
9 regional entities for substance use disorder prevention and
10 treatment, by fund source, by subcontractor, by population served,
11 and by service type.

12 (c) The expenditures per state client, with data on the
13 distribution of expenditures reported using a histogram approach.

14 (d) The number of services provided by subcontractor and by
15 service type. Additionally, data on length of stay, referral
16 source, and participation in other state programs.

17 (e) The collections from other first- or third-party payers,
18 private donations, or other state or local programs, by department-
19 designated regional entities for substance use disorder prevention
20 and treatment, by subcontractor, by population served, and by
21 service type.

22 (f) Information about access to CMHSPs, PIHPs, and designated
23 regional entities for substance use disorder prevention and
24 treatment that includes, but is not limited to, the following:

25 (i) The number of individuals receiving requested services.

26 (ii) The number of individuals who requested services but did
27 not receive services.

28 (4) The department shall include the data reporting
29 requirements described in subsections (2) and (3) in the



1 department's annual contract with each CMHSP, PIHP, and designated
2 regional entity for substance use disorder prevention and
3 treatment.

4 (5) The department shall take all reasonable actions to ensure
5 that the data required are complete and consistent among all
6 CMHSPs, PIHPs, and designated regional entities for substance use
7 disorder prevention and treatment.

8 Sec. 907. (1) The department shall expend the amount
9 appropriated in part 1 for community substance use disorder
10 prevention, education, and treatment to coordinate care and
11 services provided to individuals with severe and persistent mental
12 illness and substance use disorder diagnoses.

13 (2) Each managing entity shall continue current efforts to
14 collaborate on the delivery of services to clients with mental
15 illness and substance use disorder diagnoses, with the goal of
16 providing services in an administratively efficient manner.

17 Sec. 909. From the funds appropriated in part 1 for health
18 homes, the department shall use available revenue from the
19 marihuana regulatory fund established in section 604 of the medical
20 marihuana facilities licensing act, 2016 PA 281, MCL 333.27604, to
21 improve physical health, expand access to substance use disorder
22 prevention and treatment services, and strengthen the existing
23 prevention, treatment, and recovery systems.

24 Sec. 910. The department shall ensure that substance use
25 disorder treatment is provided to applicants and recipients of
26 public assistance through the department who are required to obtain
27 substance use disorder treatment as a condition of eligibility for
28 public assistance.

29 Sec. 911. (1) The department shall ensure that a contract with



1 a CMHSP or PIHP requires the CMHSP or PIHP to implement programs to
2 encourage the diversion of individuals with a serious mental
3 illness, serious emotional disturbance, or developmental disability
4 from possible jail incarceration, when appropriate.

5 (2) Each CMHSP or PIHP shall have jail diversion services and
6 shall work toward establishing working relationships with
7 representative staff of local law enforcement agencies, including
8 county prosecutors' offices, county sheriffs' offices, county
9 jails, municipal police agencies, municipal detention facilities,
10 and the courts. Written interagency agreements describing what
11 services each participating agency is prepared to commit to the
12 local jail diversion effort and the procedures to be used by local
13 law enforcement agencies to access mental health jail diversion
14 services are strongly encouraged.

15 Sec. 912. The department shall contract directly with the
16 Salvation Army Harbor Light program, at an amount not less than the
17 amount provided during the fiscal year ending September 30, 2020,
18 to provide non-Medicaid substance use disorder services if the
19 local coordinating agency or the department confirms the Salvation
20 Army Harbor Light program meets the standard of care established by
21 the department. The standard of care must include, but is not
22 limited to, using a medication assisted treatment option.

23 Sec. 915. From the funds appropriated in part 1 for community
24 substance use disorder prevention, education, and treatment and
25 opioid response activities, the department shall, to the extent
26 possible, provide grants, pursuant to federal law, to local public
27 entities that provide substance use disorder services and to 1
28 private entity that has a statewide contract to provide community-
29 based substance use disorder services.



1 Sec. 916. From the funds appropriated in part 1 for behavioral
2 health program administration, the department shall allocate
3 \$100,000.00 as a grant to a nonprofit mental health clinic located
4 in a county with a population between 290,000 and 300,000 according
5 to the most recent federal decennial census that provides
6 counseling services, accepts clients regardless of their ability to
7 pay for services through sliding scale copayments and volunteer
8 services, and uses fundraising to support their clinic.

9 Sec. 917. (1) From the funds appropriated in part 1 for opioid
10 response activities, the department shall allocate \$55,000,000.00
11 from the Michigan opioid healing and recovery fund created under
12 section 3 of the Michigan trust fund act, 2000 PA 489, MCL 12.253,
13 to programs and services to address the opioid crisis in a manner
14 consistent with the opioid judgement, settlement, or compromise of
15 claims pertaining to violations, or alleged violations, of law
16 related to the manufacture, marketing, distribution, dispensing, or
17 sale of opioids. The funds must be allocated as follows:

18 (a) \$8,500,000.00 for the following youth and young adult
19 prevention programs:

20 (i) \$3,000,000.00 for school-based prevention programs.

21 (ii) \$5,500,000.00 for out-of-school youth programming that
22 prevents substance use.

23 (b) \$6,000,000.00 to support families impacted by the opioid
24 epidemic.

25 (c) \$10,000,000.00 for Naloxone distribution.

26 (d) \$5,000,000.00 for mobile treatment programs.

27 (e) \$5,500,000.00 for peer recovery support services programs.

28 (f) \$5,000,000.00 for competitive grants to substance use
29 disorder short-term recovery housing organizations.



1 (g) \$10,000,000.00 for competitive grants to sober living
2 organizations.

3 (h) \$5,000,000.00 to invest in tribal communities.

4 (2) If any allocations remain after the completion of the
5 projects listed in subsection (1)(a) to (h), the department may
6 expend remaining funds for additional opioid response activities
7 that are consistent with the purposes outlined in this section.

8 (3) On a semiannual basis, the department shall submit to the
9 standard report recipients a report on all of the following:

10 (a) Total revenues deposited into and expenditures and
11 encumbrances from the Michigan opioid healing and recovery fund
12 since the creation of the fund.

13 (b) Revenues deposited into and expenditures and encumbrances
14 from the Michigan opioid healing and recovery fund during the
15 previous 6 months.

16 (c) Estimated revenues to be deposited into and the spending
17 plan for the Michigan opioid healing and recovery fund for the next
18 12 months.

19 Sec. 918. On a quarterly basis, providing monthly data, the
20 department shall submit a report to the standard report recipients
21 on the amount of funding paid to PIHPs to support the Medicaid
22 managed mental health care program. The report must include
23 information on the total paid to each PIHP, per capita rate paid
24 for each eligibility group for each PIHP, the number of cases in
25 each eligibility group for each PIHP, and a year-to-date summary of
26 eligibles and expenditures for the Medicaid managed mental health
27 care program.

28 Sec. 920. As part of the Medicaid rate-setting process for
29 behavioral health services, the department shall work with PIHP



1 network providers and actuaries to include, as part of the Medicaid
2 rate, state and federal wage and compensation increases that
3 directly impact staff who provide Medicaid-funded community living
4 supports, personal care services, respite services, skill-building
5 services, and other supports and services that the department
6 determines are similar.

7 Sec. 924. From the funds appropriated in part 1, for the
8 purposes of actuarially sound rate certification and approval for
9 Medicaid behavioral health managed care programs, the department
10 shall maintain a minimum fee schedule of not less than \$66.00 per
11 hour for behavioral technicians providing autism services that are
12 reimbursed under CPT code 97153.

13 Sec. 926. (1) From the funds appropriated in part 1 for
14 community substance use disorder prevention, education, and
15 treatment, \$1,000,000.00 is allocated for a specialized substance
16 use disorder detoxification project administered by a 9-1-1 service
17 district in conjunction with a substance use and case management
18 provider. The project must be located at a hospital within a 9-1-1
19 service district with at least 600,000 residents and 15 member
20 communities and that is located within a county with a population
21 of at least 1,500,000 according to the most recent federal
22 decennial census.

23 (2) The substance use and case management provider receiving
24 funds under this section shall collect and submit to the department
25 data on the outcomes of the project throughout the duration of the
26 project and the department shall submit a report on the project's
27 outcomes to the standard report recipients.

28 Sec. 928. (1) Each PIHP shall provide, from the PIHP's
29 internal resources, local funds to be used as a part of the state



1 match required under the Medicaid program in order to increase
2 capitation rates for PIHPs. The local funds must not include either
3 of the following:

4 (a) State funds received by a CMHSP for services provided to
5 non-Medicaid recipients.

6 (b) The state matching portion of the Medicaid capitation
7 payments made to a PIHP.

8 (2) Not later than April 1 of the current fiscal year, the
9 department shall report to the standard report recipients on the
10 lapse by PIHP from the previous fiscal year and the projected lapse
11 by PIHP in the current fiscal year.

12 Sec. 935. A county required under the mental health code, 1974
13 PA 258, MCL 330.1001 to 330.2106, to provide matching funds to a
14 CMHSP for mental health services rendered to residents in the
15 county's jurisdiction shall pay the matching funds in equal
16 installments on not less than a quarterly basis throughout the
17 fiscal year, with the first payment being made by October 1 of the
18 current fiscal year.

19 Sec. 940. (1) In accordance with section 236 of the mental
20 health code, 1974 PA 258, MCL 330.1236, the department shall review
21 expenditures for each CMHSP to identify any CMHSP with a projected
22 allocation surplus and to identify any CMHSP with a projected
23 allocation shortfall. The department shall encourage the board of a
24 CMHSP with a projected allocation surplus to concur with the
25 department's recommendation to reallocate the projected surplus to
26 a CMHSP with a projected allocation shortfall.

27 (2) A CMHSP that has its projected surplus reallocated during
28 the current fiscal year as described in subsection (1) is not
29 eligible for an additional funding reallocation during the



1 remainder of the current fiscal year, unless the CMHSP is
2 responding to a public health emergency as determined by the
3 department.

4 (3) A CMHSP shall report to the department on a proposed
5 reallocation described in this section at least 30 days before the
6 reallocation takes effect.

7 (4) The department shall notify the chairs of the
8 appropriation subcommittees on the department budget when a request
9 is made and when the department grants approval for a reallocation
10 described in subsection (1). Not later than February 1 of the
11 current fiscal year, the department shall submit a report on the
12 amount of funding reallocated in the previous fiscal year to the
13 standard report recipients.

14 Sec. 942. A CMHSP shall provide at least 30 days' notice
15 before reducing, terminating, or suspending a service provided by
16 the CMHSP to a CMHSP client, unless the service is authorized by a
17 physician and the service no longer meets established criteria for
18 medical necessity.

19 Sec. 960. (1) From the funds appropriated in part 1 for autism
20 services, the department shall continue to cover all Medicaid
21 autism services to Medicaid enrollees eligible for the services
22 that were covered on January 1, 2019.

23 (2) To restrain cost increases in the autism services line
24 item, the department shall do all of the following:

25 (a) Require consultation with the client's evaluation
26 diagnostician and PIHP to approve the client's ongoing therapy for
27 3 years, unless the client's evaluation diagnostician recommended
28 an evaluation before the 3 years or if a clinician on the treatment
29 team recommended an evaluation for the client before the third



1 year.

2 (b) Limit the authority to perform a diagnostic evaluation for
3 Medicaid autism services to qualified licensed practitioners as
4 determined by the department.

5 (c) Allow and expand the utilization of telemedicine and
6 telepsychiatry to increase access to diagnostic evaluation
7 services.

8 (d) Coordinate with the department of insurance and financial
9 services on oversight for compliance with the Paul Wellstone and
10 Pete Domenici mental health parity and addiction equity act of
11 2008, Public Law 110-343, as it relates to autism spectrum disorder
12 services, to ensure appropriate cost sharing between public and
13 private payers.

14 (e) Require that Medicaid eligibility be confirmed through
15 prior evaluations conducted by qualified licensed practitioners as
16 determined by the department.

17 (f) Maintain regular statewide provider trainings on autism
18 spectrum disorder standard clinical best practice guidelines for
19 treatment and diagnostic services.

20 (3) By March 1 of the current fiscal year, the department
21 shall submit a report to the standard report recipients on total
22 autism services spending broken down by PIHP and CMHSP for the
23 previous fiscal year and current fiscal year and total
24 administrative costs broken down by PIHP, CMHSP, and the type of
25 administrative cost for the previous fiscal year and current fiscal
26 year.

27 Sec. 962. For special projects involving high-need children or
28 adults, including the not guilty by reason of insanity population,
29 the department may contract directly with providers of services to



1 the children and adults described in this section.

2 Sec. 965. From the funds appropriated in part 1, the
3 department and each PIHP shall maintain the comparison rate and any
4 associated reimbursement rate of the bundled rate H0020 for the
5 administration and services of methadone at not less than \$19.00.

6 Sec. 972. From the funds appropriated in part 1 for behavioral
7 health program administration, the department shall allocate not
8 less than \$9,386,400.00 of general fund/general purpose revenue and
9 any associated federal match or federal grant funding, including,
10 but not limited to, associated federal 988 grant funding for the
11 mental health telephone access line known as the Michigan crisis
12 and access line (MiCAL), to provide for both of the following in
13 accordance with section 165 of the mental health code, 1974 PA 258,
14 MCL 330.1165:

15 (a) Primary coverage in a region where a regional national
16 suicide prevention lifeline center does not provide coverage.

17 (b) Statewide secondary coverage.

18 Sec. 974. The department and a PIHP shall allow an individual
19 with an intellectual or developmental disability who receives
20 supports and services from a CMHSP to instead receive supports and
21 services from another provider if the individual is eligible and
22 qualified to receive supports and services from another provider.
23 Other providers may include, but are not limited to, MIChoice and
24 PACE.

25 Sec. 978. From the funds appropriated in part 1 for community
26 substance use disorder prevention, education, and treatment and
27 recovery community organizations, the department shall allocate
28 \$1,200,000.00 as grants for recovery community organizations in
29 accordance with section 273b of the mental health code, 1974 PA



1 258, MCL 330.1273b. A grant must be used to offer or expand
2 recovery support center services or recovery community center
3 services to individuals seeking long-term recovery from substance
4 use disorders.

5 Sec. 994. (1) Not later than June 1 of the current fiscal
6 year, the department shall seek, if necessary, federal approval
7 through either a waiver request or state plan amendment to allow a
8 CMHSP, PIHP, or subcontracting provider agency that is reviewed and
9 accredited by a national accrediting entity for behavioral health
10 care services to be considered in compliance with state program
11 review and audit requirements that are addressed and reviewed by
12 that national accrediting entity.

13 (2) Not later than September 30 of the current fiscal year,
14 the department shall report to the standard report recipients all
15 of the following:

16 (a) The status of the federal approval process required in
17 subsection (1).

18 (b) A list of each CMHSP, PIHP, and subcontracting provider
19 agency that is considered to be in compliance with state program
20 review and audit requirements under subsection (1).

21 (c) For each CMHSP, PIHP, or subcontracting provider agency
22 described in subdivision (b), both of the following:

23 (i) The state program review and audit requirements that the
24 CMHSP, PIHP, or subcontracting provider agency is considered to be
25 in compliance with.

26 (ii) The national accrediting entity that reviewed and
27 accredited the CMHSP, PIHP, or subcontracting provider agency.

28 (3) The department shall continue to comply with the laws of
29 this state and federal law and shall not initiate an action that



negatively impacts beneficiary safety. Any cost savings attributed to this action must be reinvested back into services.

(4) As used in this section, "national accrediting entity" means the Joint Commission, formerly known as the Joint Commission on Accreditation of Healthcare Organizations; the Commission on Accreditation of Rehabilitation Facilities; the Council on Accreditation; the URAC, formerly known as the Utilization Review Accreditation Commission; the National Committee for Quality Assurance; or another appropriate entity, as approved by the department.

Sec. 995. (1) From the funds appropriated in part 1 for mental health diversion council, the department shall allocate \$3,850,000.00 to continue to implement the jail diversion programs that are intended to address the recommendations of the mental health diversion council.

(2) Not later than March 1 of the current fiscal year, the department shall submit a report to the standard report recipients on the planned allocation of the funds appropriated for the mental health diversion council.

(3) As used in this section, "mental health diversion council" means the council as that term is defined in section 207e of the mental health code, 1974 PA 258, MCL 330.1207e.

Sec. 996. From the funds appropriated in part 1 for family support subsidy, the department shall make monthly payments of \$300.36 to a parent or legal guardian of a child approved for the family support subsidy by a CMHSP.

Sec. 997. The department shall use population data from the most recent federal data from the United States Census Bureau in determining the distribution of substance use disorder block grant



1 funds.

2 Sec. 998. If the department decides to use census data to
3 distribute state general funds to CMHSPs, the department shall use
4 the most recent federal data from the United States Census Bureau.
5

6 **BEHAVIORAL HEALTH SERVICES**

7 Sec. 1001. Not later than May 15 of the current fiscal year,
8 each CMHSP shall submit a report to the department that identifies
9 populations being served by the CMHSP broken down by program
10 eligibility category. The report must also include the percentage
11 of the operational budget that is related to program eligibility
12 enrollment. Not later than June 30 of the current fiscal year, the
13 department shall submit the reports described in this section to
14 the standard report recipients.

15 Sec. 1002. The funds appropriated in part 1 must not be used
16 by the department to expand the certified community behavioral
17 health clinic demonstration.

18 Sec. 1003. The department shall notify the Community Mental
19 Health Association of Michigan when developing a policy or
20 procedure that will impact a PIHP or CMHSP.

21 Sec. 1004. The department shall submit a report to the
22 standard report recipients on any rebased formula changes to either
23 Medicaid behavioral health services or non-Medicaid mental health
24 services 90 days before the department implements the formula
25 change. The notification must include a table showing the changes
26 in funding allocation by PIHP for Medicaid behavioral health
27 services or by CMHSP for non-Medicaid mental health services.

28 Sec. 1005. (1) From the funds appropriated in part 1 for
29 health homes, the department shall maintain the number of



1 behavioral health homes and maintain the number of substance use
2 disorder health homes, in place by PIHP region as of September 30
3 of the previous fiscal year. The department may expand the number
4 of behavioral health homes and the number of substance use disorder
5 health homes in a PIHP region added after October 1 of the current
6 fiscal year.

7 (2) On a semiannual basis, the department shall submit a
8 report to the standard report recipients on the number of
9 individuals being served and expenditures incurred by each PIHP
10 region by site.

11 Sec. 1006. (1) From the funds appropriated in part 1 for
12 certified community behavioral health clinics, not later than May 1
13 of the current fiscal year the department shall submit to the
14 standard report recipients an outcomes report for CCBHCs during the
15 previous fiscal year that includes both statewide and CCBHC site-
16 specific information on all of the following:

17 (a) The total number of distinct individuals served by the
18 CCBHCs.

19 (b) The percentage of individuals served by the CCBHCs that
20 were Medicaid recipients.

21 (c) The percentage of individuals served by the CCBHCs that
22 were not Medicaid recipients.

23 (d) The total number of CCBHC daily visits.

24 (e) Total number of CCBHC services provided, broken down by
25 the 9 core CCBHC services.

26 (f) Total expenditures from base and supplemental payments.

27 (g) Staffing and staff vacancy levels of the CCBHCs.

28 (h) The amount of prospective payment system rates for each
29 CCBHC over the entire demonstration period allocated across the 9



1 service types.

2 (i) The total expenditures by CCBHC in the previous fiscal
3 year.

4 (j) The total cost factors and implications in interpreting
5 how CCBHCs deliver care over the course of the demonstration
6 period.

7 (k) The comparison of costs for a random sample of enrollees
8 between care provided by a CCBHC provider and a Medicaid provider
9 that is not a CCBHC. The sample must include participants known to
10 have received services at CCBHC providers and Medicaid providers
11 that are not CCBHCs.

12 (2) From the funds appropriated in part 1 for certified
13 community behavioral health clinics, the department shall submit
14 the CCBHC cost efficiency evaluation to the standard report
15 recipients not later than 7 business days after the department's
16 receipt of the final information required from the relevant
17 contractors.

18 Sec. 1008. (1) A PIHP and CMHSP shall do all of the following:

19 (a) Work to reduce administration costs by ensuring that PIHP
20 and CMHSP responsible functions are efficient in allowing optimal
21 transition of dollars to the direct services considered most
22 effective in assisting individuals served. Any consolidation of
23 administrative functions must demonstrate, by independent analysis,
24 a reduction in dollars spent on administration resulting in greater
25 dollars spent on direct services. Savings resulting from increased
26 efficiencies must not be applied to PIHP and CMHSP net assets,
27 internal service fund increases, building costs, increases in the
28 number of PIHP and CMHSP personnel, or other areas not directly
29 related to the delivery of improved services.



1 (b) Take an active role in managing mental health care by
2 ensuring consistent and high-quality service delivery throughout
3 its network and promote a conflict-free care management
4 environment.

5 (c) Ensure that direct service rate variances are related to
6 the level of need or other quantifiable measures to ensure that the
7 most money possible reaches direct services.

8 (d) Whenever possible, promote fair and adequate direct care
9 reimbursement, including, but not limited to, fair wages for direct
10 service workers.

11 (2) Not later than June 30 of the current fiscal year, the
12 department shall submit a report to the standard report recipients
13 on any actual reduction of administrative costs over the prior 2
14 fiscal years.

15 Sec. 1010. (1) The department shall use the funds appropriated
16 in part 1 for behavioral health community supports and services to
17 reduce waiting lists at state-operated hospitals and centers
18 through cost-effective community-based and residential services,
19 including, but not limited to, assertive community treatment,
20 forensic assertive community treatment, crisis stabilization units
21 in accordance with chapter 9A of the mental health code, 1974 PA
22 258, MCL 330.1971 to 330.1979, and psychiatric residential
23 treatment facilities in accordance with section 137a of the mental
24 health code, 1974 PA 258, MCL 330.1137a.

25 (2) From the funds appropriated in part 1 for behavioral
26 health community supports and services, the department shall
27 allocate \$30,450,000.00 to reimburse private providers for
28 intensive psychiatric treatments and services that are provided
29 outside of state-operated hospitals and centers and for support



1 efforts related to overseeing community-based programs placement.

2 (3) If a private provider has an existing wait list for
3 intensive psychiatric treatments and services, a reimbursement to
4 the private provider under this section must not be conditioned on
5 the private provider giving wait-list priority to individuals
6 placed with funds appropriated in this section.

7 (4) Not later than March 1 of the current fiscal year, the
8 department shall submit a report to the standard report recipients
9 on all of the following:

10 (a) The types of community supports and services purchased.

11 (b) The quantity, measured by days or other relevant unit of
12 service, of each community support and service purchased.

13 (c) The quantifiable impact of the purchase of community
14 supports and services, including the number of individuals served,
15 the number of successful discharges, and the number of re-
16 escalations to either the discharging entity or a state psychiatric
17 hospital.

18 Sec. 1014. (1) From the funds appropriated in part 1 to
19 agencies providing physical and behavioral health services to
20 multicultural populations, the department shall award grants in
21 accordance with the requirements of subsections (2) and (3). This
22 state is not liable for any spending above the contract amount. The
23 department shall not release funds until reporting requirements
24 under section 1014 of article 6 of 2024 PA 121 are satisfied.

25 (2) The department shall require each contractor described in
26 subsection (1) that receives greater than \$1,000,000.00 in state
27 grant funding to comply with performance-related metrics to
28 maintain their eligibility for funding. The performance-related
29 metrics shall include, but not be limited to, all of the following:



1 (a) Each contractor or subcontractor shall have accreditations
2 that attest to their competency and effectiveness as behavioral
3 health and social service agencies.

4 (b) Each contractor or subcontractor shall have a mission that
5 is consistent with the purpose of the multicultural agency.

6 (c) Each contractor shall validate that any subcontractors
7 utilized within these appropriations share the same mission as the
8 lead agency receiving funding.

9 (d) Each contractor or subcontractor shall demonstrate cost-
10 effectiveness.

11 (e) Each contractor or subcontractor shall ensure their
12 ability to leverage private dollars to strengthen and maximize
13 service provision.

14 (f) Each contractor or subcontractor shall provide timely and
15 accurate reports regarding the number of clients served, units of
16 service provision, and ability to meet their stated goals.

17 (3) The department shall require each contractor described in
18 subsection (1) to ensure that the funds appropriated in this
19 section are only used on proven or established programs.

20 (4) The department shall require an annual report from the
21 contractors described in subsection (2). The annual report, due 60
22 days following the end of the contract period, must include
23 specific information on services and programs provided, the client
24 base to which the services and programs were provided, information
25 on any wraparound services provided, and the expenditures for those
26 services. Not later than February 1 of the current fiscal year, the
27 department must submit the annual reports to the standard report
28 recipients.

29 Sec. 1034. (1) PIHPs must verify, on a quarterly basis, to the



1 department and to the standard report recipients that every
2 provider within the PIHP's provider network receives not less than
3 the applicable reimbursement rates or fees required in sections 924
4 and 231 of this part. The verification under this subsection must
5 provide actual claims and utilization data.

6 (2) The department shall seek CMS approval to exclude PIHPs
7 that are not compliant with subsection (1) from all performance
8 incentives available to PIHPs.

9 (3) The department shall audit the claims and utilization data
10 provided in this section. If the department audit determines that a
11 PIHP reimburses any provider within that PIHP's provider network at
12 a rate less than the applicable reimbursement rates or fees
13 required in sections 924 and 231 of this part, the department shall
14 notify that PIHP that it is not eligible for performance incentives
15 funded in part 1. Not later than 10 days after a notification to a
16 PIHP under this subsection, the department shall notify the
17 standard report recipients that the PIHP is not eligible for
18 performance incentives funded in part 1.

19
20 **STATE PSYCHIATRIC HOSPITALS AND FORENSIC MENTAL HEALTH SERVICES**

21 Sec. 1051. The department shall continue a revenue recapture
22 project to generate additional revenues from third parties related
23 to cases that have been closed or are inactive. A portion of
24 revenues collected through the project's efforts may be used for
25 departmental costs and contractual fees associated with retroactive
26 collections under the project and to improve ongoing departmental
27 reimbursement management functions.

28 Sec. 1052. The department shall use gifts and bequests
29 received for patient living and treatment environments for



1 additional private funds to provide specific enhancements for
2 individuals residing at state-operated facilities. The department
3 shall use the gifts and bequests consistent with the stipulation of
4 the donor. The department shall use gift and bequest donations
5 within 3 years unless otherwise stipulated by the donor.

6 Sec. 1055. (1) The department shall not implement a closure or
7 consolidation of a state hospital, center, or agency, until each
8 CMHSP or PIHP affected by the closure or consolidation has programs
9 and services in place for the individuals currently in the
10 hospital, center, or agency that is to be closed or consolidated,
11 and has a plan for providing services to the individuals who would
12 have been admitted to the hospital, center, or agency.

13 (2) A closure or consolidation is dependent on adequate
14 department-approved CMHSP and PIHP plans that include a discharge
15 and aftercare plan for each individual currently in a facility
16 described in subsection (1). A discharge and aftercare plan must
17 address an individual's housing needs. A homeless shelter or
18 similar temporary shelter arrangement is inadequate to meet an
19 individual's housing needs.

20 (3) Four months after a closure is certified under section
21 19(6) of the state employees' retirement act, 1943 PA 240, MCL
22 38.19, the department shall provide a closure plan to the standard
23 report recipients.

24 (4) On the closure of a hospital, center, or agency and after
25 transitional costs have been paid, the remaining balances of funds
26 appropriated for the hospital, center, or agency must be
27 transferred to CMHSPs or PIHPs responsible for providing services
28 for individuals previously served by the hospital, center, or
29 agency.



1 Sec. 1056. The department may collect revenue for patient
2 reimbursement from first- and third-party payers, including
3 Medicaid and local county CMHSP payers, to cover the cost of
4 patient placement in state hospitals and centers. The department
5 may adjust financing sources for patient reimbursement based on
6 actual revenues earned. If the revenue collected exceeds current
7 year expenditures, the revenue may be carried forward with approval
8 of the state budget director. The department shall use the revenue
9 carried forward as a first source of funds in the subsequent year.

10 Sec. 1058. Effective October 1 of the current fiscal year, the
11 department, in consultation with the department of technology,
12 management, and budget, may maintain a bid process to identify 1 or
13 more private contractors to provide food and custodial services for
14 the administrative areas at a state hospital identified by the
15 department as capable of generating savings through the outsourcing
16 of food and custodial services.

17 Sec. 1059. (1) The department shall identify specific outcomes
18 and performance measures for state-operated hospitals and centers.
19 Unless specified, the outcomes and performance measures must be
20 calculated on an average monthly basis from the previous calendar
21 year, as follows:

22 (a) The average wait time from the time of the receipt of a
23 court order for the treatment of an individual who is determined
24 incompetent to stand trial until the individual's admission to the
25 center for forensic psychiatry or other state-operated psychiatric
26 hospital.

27 (b) The average number of individuals determined not guilty by
28 reason of insanity by an order of the court who, on the first day
29 of each month, are waiting to receive admission into the center for



1 forensic psychiatry or other state-operated psychiatric hospital.

2 (c) The average number of adults who, on the first day of each
3 month, are waiting to receive admission into another state-operated
4 hospital or center through the civil admissions process.

5 (d) The average number of children who, on the first day of
6 each month, are waiting to receive admission into another state-
7 operated hospital or center through the civil admissions process.

8 (e) The average wait time for an adult who is awaiting
9 admission into another state-operated hospital or center through
10 the civil admissions process.

11 (f) The average wait time for a child who is awaiting
12 admission into another state-operated hospital or center through
13 the civil admissions process.

14 (g) The number of individuals determined not guilty by reason
15 of insanity or incompetent to stand trial by an order of the court
16 who have been determined to be ready for discharge to the
17 community, and the average wait time between being determined to be
18 ready for discharge to the community and actual community
19 placement.

20 (h) The number of adults admitted through the civil admission
21 process that have been determined to be ready for discharge to the
22 community, and the average wait time between being determined to be
23 ready for discharge to the community and actual community
24 placement.

25 (i) The number of children admitted through the civil
26 admission process that have been determined to be ready for
27 discharge to the community, and the average wait time between being
28 determined to be ready for discharge to the community and actual
29 community placement.



1 (j) The most recent 12-month total number of individuals
2 determined not guilty by reason of insanity by an order of the
3 court ordering the individual to be admitted into the center for
4 forensic psychiatry or other state-operated psychiatric hospital.

5 (k) The most recent 12-month total number of adults requested
6 to be admitted to a state-operated hospital or center through the
7 civil admissions process.

8 (l) The most recent 12-month total number of children requested
9 to be admitted to a state-operated hospital or center through the
10 civil admissions process.

11 (m) The number of individuals determined not guilty by reason
12 of insanity by an order of the court who were removed from the
13 admissions waiting list and the reason for the removal from the
14 admissions waiting list.

15 (n) The number of adults awaiting admission through the civil
16 admission process removed from the admission waiting list and the
17 reason for the removal from the admission waiting list.

18 (o) The number of children awaiting admission through the
19 civil admission process removed from the admission waiting list and
20 the reason for the removal from the admission waiting list.

21 (p) The number of individuals determined not guilty by reason
22 of insanity by an order of the court and not admitted into the
23 center for forensic psychiatry or other state-operated hospital or
24 center, and the rationale for the individual not being admitted.

25 (q) The number of adults not admitted into the other state-
26 operated hospitals or centers through the civil admissions process
27 and the rationale for the individual not being admitted.

28 (r) The number of children not admitted into a state-operated
29 hospital or center through the civil admission process and the



1 rationale for the individual not being admitted.

2 (2) Not later than April 1 of the current fiscal year, the
3 department shall submit a report to the standard report recipients
4 of this part on the outcomes and performance measures required
5 under subsection (1).

6 Sec. 1060. Not later than March 1 of the current fiscal year,
7 the department shall submit a report on mandatory overtime, staff
8 turnover, and staff retention at the state psychiatric hospitals
9 and centers to the standard report recipients. The report must
10 include, but is not limited to, the following:

11 (a) The number of direct care and clinical staff positions
12 that are currently vacant by hospital, and how that number compares
13 to the number of vacancies during the previous fiscal year.

14 (b) A breakdown of voluntary and mandatory overtime hours
15 worked by position and by hospital, and how that breakdown compares
16 to the breakdown of voluntary and mandatory overtime hours during
17 the previous fiscal year.

18 (c) The ranges of wages paid by position and by hospital, and
19 how the ranges of wages paid compare to wages paid during the
20 previous fiscal year.

21 Sec. 1063. (1) From the funds appropriated in part 1 for
22 southeast Michigan state psychiatric hospital, the department shall
23 maintain a psychiatric transitional unit and children's transition
24 support team. The unit and support team described in this
25 subsection shall augment the continuum of behavioral health
26 services for high-need youth and provide additional continuity of
27 care and transition into supportive community-based services.

28 (2) The outcome and performance measures for the unit and
29 support team described in subsection (1) include, but are not



1 limited to, the following:

2 (a) The rate of rehospitalization for youth served through the
3 unit or support team at 30 and 180 days.

4 (b) The measured change in the Michigan Child and Adolescent
5 Needs and Strengths tool for youth and families served through the
6 unit or support team.

7
8 **HEALTH AND HUMAN SERVICES POLICY AND INITIATIVES**

9 Sec. 1140. From the funds appropriated in part 1 for primary
10 care services, \$400,000.00 is allocated to free health clinics
11 operating in this state. The department shall distribute the funds
12 equally to each free health clinic. As used in this section, "free
13 health clinic" means a nonprofit organization that uses a volunteer
14 health professional to provide care to an uninsured individual.

15 Sec. 1143. From the funds appropriated in part 1 for primary
16 care services, the department shall allocate no less than
17 \$675,000.00 for island primary health care access and services
18 including island clinics, in the following amounts:

19 (a) Beaver Island, \$250,000.00.

20 (b) Mackinac Island, \$250,000.00.

21 (c) Drummond Island, \$150,000.00.

22 (d) Bois Blanc Island, \$25,000.00.

23 Sec. 1145. The department shall take steps necessary to work
24 with the Indian Health Service, tribal health program facilities,
25 or Urban Indian Health Program facilities, that provide services
26 under a contract with a Medicaid managed care entity to ensure that
27 the facilities described in this section receive the maximum amount
28 allowable under federal law for Medicaid services.

29 Sec. 1146. From the funds appropriated in part 1 for domestic



1 violence prevention and treatment, the department shall allocate
2 \$1,000,000.00 to support programs that serve survivors of domestic
3 violence, sexual violence, and human trafficking. The funds
4 appropriated in this section must be allocated in the following
5 manner:

6 (a) \$500,000.00 must be used to provide technical assistance,
7 training, and support to victim service organizations and must be
8 allocated to a nonprofit organization organized under the laws of
9 this state that is exempt from federal income tax under section
10 501(c)(3) of the internal revenue code of 1986, 26 USC 501. To be
11 eligible for funding under this subsection, the nonprofit
12 organization must be a statewide tribal domestic violence and
13 sexual assault coalition serving the tribes located in this state
14 recognized as the Tribal Coalition in this state by the Office on
15 Violence Against Women within the United States Department of
16 Justice.

17 (b) \$500,000.00 must be allocated to a nonprofit organization
18 organized under the laws of this state that is exempt from federal
19 income tax under section 501(c)(3) of the internal revenue code of
20 1986, 26 USC 501, recognized as the Domestic and Sexual Violence
21 Coalition in this state by the Office on Violence Against Women
22 within the United States Department of Justice.

23 Sec. 1153. From the funds appropriated in part 1 for crime
24 victim rights sustaining grants, the department shall allocate
25 \$102,600.00 of state general fund/general purpose revenue for a
26 sexual assault nurse examiners program at a hospital that is
27 located in a city with a population between 21,600 and 21,700 in a
28 county with a population between 64,300 and 64,400, according to
29 the most recent federal decennial census. The funds allocated under



1 this section must be used to support staff compensation and
2 training, victim needs, and community awareness, education, and
3 prevention programs.

4 Sec. 1155. (1) From the funds appropriated in part 1 for the
5 uniform statewide sexual assault evidence kit tracking system, in
6 accordance with the final report of the Michigan sexual assault
7 evidence kit tracking and reporting commission, the department
8 shall allocate \$369,500.00 for administering a uniform statewide
9 sexual assault evidence kit tracking system. The system must
10 include all of the following:

11 (a) A uniform statewide system to track the submission and
12 status of sexual assault evidence kits.

13 (b) A uniform statewide system to audit untested kits that
14 were collected on or before March 1, 2015 and were released by
15 victims to law enforcement.

16 (c) Secure electronic access for victims.

17 (d) The ability to accommodate concurrent data entry with kit
18 collection through mechanisms that include, but are not limited to,
19 web entry through computers or smartphones, and through scanning
20 devices.

21 (2) The sexual assault evidence tracking fund established in
22 section 1451 of 2017 PA 158 shall continue to be maintained in the
23 department of treasury. Money in the sexual assault evidence
24 tracking fund at the close of a fiscal year remains in the sexual
25 assault evidence tracking fund, does not revert to the general
26 fund, and is appropriated as provided by law for the development
27 and implementation of a uniform statewide sexual assault evidence
28 kit tracking system as described in subsection (1).

29 Sec. 1157. (1) From the funds appropriated in part 1 for child



1 advocacy centers - supplemental grants, the department shall
2 allocate \$2,000,000.00 to provide additional funding to child
3 advocacy centers to support the general operations of child
4 advocacy centers. The department shall allocate the additional
5 funding to each center according to the formula under this section.
6 The department shall set a formula in consultation with children's
7 advocacy centers of Michigan (CAC-MI) to allocate the additional
8 funding. The formula must include base funding for each program and
9 factors, such as the number of children in the service area, square
10 miles of the service area, and prior service levels. The purpose of
11 the additional funding is to increase the amount of services
12 provided to children and their families who are victims of abuse
13 over the amount provided in the previous fiscal year.

14 (2) Not later than March 1 of the current fiscal year, the
15 department shall submit to the standard report recipients a report
16 on the distribution of child advocacy center - supplemental grants
17 funding from the previous fiscal year. The report must include the
18 amount allocated to each specific child advocacy center or other
19 community-based child protection entity, including, but not limited
20 to, child abuse councils.

21 Sec. 1158. From the funds appropriated in part 1 for crime
22 victim rights sustaining grants, the department shall allocate
23 \$29,897,400.00 to supplement the loss of federal victims of crime
24 act and state crime victim rights funding. The department must
25 distribute the funds consistent with the regular allocation formula
26 for crime victim justice grants and crime victim rights services
27 grants.

28 Sec. 1160. Not later than March 1 of the current fiscal year,
29 the department shall submit to the standard report recipients a



1 report on the distribution of crime victim rights justice
2 assistance grants, crime victim rights services grants, and crime
3 victim rights sustaining grants from the previous fiscal year. The
4 report must include the amount allocated to nonprofit agencies for
5 crime victim services listed by agency and the amount of funding
6 that the department has used for administrative purposes.
7

8 **EPIDEMIOLOGY, EMERGENCY MEDICAL SERVICES, AND LABORATORY**

9 Sec. 1180. From the funds appropriated in part 1 for
10 epidemiology administration and for childhood lead program, the
11 department shall maintain a public health drinking water program
12 and maintain enhanced efforts to monitor child blood lead levels.
13 The public health drinking water program shall ensure that
14 appropriate investigations of potential health hazards occur for
15 all community and noncommunity drinking water supplies where
16 chemical exceedances of action levels, health advisory levels, or
17 maximum contaminant limits are identified. The goals of the
18 childhood lead program must include improving the identification of
19 children affected by lead exposure, improving the timeliness of
20 case follow-up, and attaining nurse care management for children
21 with lead exposure, and to achieve a long-term reduction in the
22 percentage of children in this state with elevated blood lead
23 levels.

24 Sec. 1181. From the funds appropriated in part 1 for
25 epidemiology administration, the department shall maintain a vapor
26 intrusion response program. The vapor intrusion response program
27 shall assess risks to public health at vapor intrusion sites and
28 respond to vapor intrusion risks if appropriate. The goals of the
29 vapor intrusion response program must include reducing the number



1 of individuals who are exposed to toxic substances through vapor
2 intrusion and improving health outcomes for individuals who are
3 identified as having been exposed to vapor intrusion.

4 Sec. 1182. Not later than April 1 of the current fiscal year,
5 the department shall submit a report to the standard report
6 recipients on the expenditures and activities undertaken by the
7 lead abatement program during the previous fiscal year using the
8 funds previously appropriated for the healthy homes program. The
9 report must include, but is not limited to, a funding allocation
10 schedule, the expenditures by category of expenditure and by
11 subcontractor, a description of program elements, the number of
12 housing units abated of lead-based paint hazards by zip code, and a
13 description of program accomplishments and progress.

14
15 **LOCAL HEALTH AND ADMINISTRATIVE SERVICES**

16 Sec. 1220. The amount appropriated in part 1 for
17 implementation of the 1993 additions of or amendments to sections
18 9161, 16221, 16226, 17015, and 17515 of the public health code,
19 1978 PA 368, MCL 333.9161, 333.16221, 333.16226, 333.17015, and
20 333.17515, must be used to reimburse local health departments for
21 costs incurred to implement section 17015(18) of the public health
22 code, 1978 PA 368, MCL 333.17015.

23 Sec. 1221. If a county that participates in a district health
24 department or has an associated arrangement with another local
25 health department takes action to stop participating in that
26 arrangement after October 1 of the current fiscal year, the
27 department may assess a penalty from the local health department's
28 operational accounts in an amount equal to no more than 6.25% of
29 the local health department's essential local public health



1 services funding. The department shall assess a penalty only if a
2 county requests the dissolution of the health department.

3 Sec. 1222. (1) The department shall prospectively allocate
4 funds appropriated in part 1 for essential local public health
5 services to local health departments to support immunizations,
6 infectious disease control, sexually transmitted disease control
7 and prevention, hearing screening, vision services, food
8 protection, public water supply, private groundwater supply, and
9 on-site sewage management. The department shall consult with the
10 department of agriculture and rural development before allocating
11 funds for food protection under this section. The department shall
12 consult with the department of environment, Great Lakes, and energy
13 before allocating funds for public water supply, private
14 groundwater supply, and on-site sewage management under this
15 section.

16 (2) The department shall not distribute funds under subsection
17 (1) to a county unless the county maintains local spending in the
18 current fiscal year in an amount that is equal to or exceeds the
19 amount the county expended in fiscal year 1992-1993 for the
20 services described in subsection (1).

21 (3) Not later than February 1 of the current fiscal year, the
22 department shall submit a report to the standard report recipients
23 on the planned allocation of the funds appropriated for essential
24 local public health services.

25 (4) The department shall continue to implement the
26 distribution formula for allocating essential local public health
27 services funding to local health departments as specified in
28 section 1234 of article X of 2018 PA 207.

29 (5) From the funds appropriated in part 1 for essential local



1 public health services, each local public health department is
2 allocated not less than the amount allocated to that local public
3 health department during the previous fiscal year.

4 Sec. 1227. The department shall establish criteria for all
5 funds allocated for health and wellness initiatives. The criteria
6 must include a requirement that a program receiving funding is
7 evidence-based and supported by research, includes interventions
8 that have been shown to demonstrate outcomes that lower cost and
9 improve quality, and is designed for statewide impact. The
10 department shall give preference to a program that uses the funding
11 as match for additional resources, including, but not limited to,
12 federal sources.

13 Sec. 1231. (1) From the funds appropriated for local health
14 services, up to \$4,750,000.00 is allocated for grants to local
15 health departments to support PFAS response and emerging public
16 health threat activities. The department shall allocate a portion
17 of the funding in a collaborative fashion with local health
18 departments in jurisdictions experiencing PFAS contamination. The
19 department shall allocate the remainder of the funding to address
20 infectious and vector-borne disease threats, and other
21 environmental contamination issues, including, but not limited to,
22 vapor intrusion, drinking water contamination, and lead exposure.
23 The department shall allocate the funding to address issues
24 including, but not limited to, staffing, planning and response, and
25 creating and disseminating materials related to PFAS contamination
26 issues and other emerging public health issues and threats.

27 (2) Not later than March 1 of the current fiscal year, the
28 department shall submit a report to the standard report recipients
29 on actual expenditures in the previous fiscal year and planned



1 spending in the current fiscal year of the funds described in
2 subsection (1). The report must include recipient entities, the
3 amount of allocation, the general category of allocation, and
4 detailed uses.

5 Sec. 1232. The department may work to ensure that the United
6 States Department of Defense reimburses the state for costs
7 associated with PFAS and environmental contamination response at
8 military training sites and support facilities.

9 Sec. 1233. The department shall not expend general fund and
10 state restricted fund appropriations in part 1 for PFAS and
11 environmental contamination response if federal funding or private
12 grant funding is available for the same expenditures.

13 Sec. 1239. The department shall participate in and give
14 necessary assistance to the Michigan PFAS action response team
15 (MPART) pursuant to Executive Order No. 2019-03. The department
16 shall collaborate with MPART and other departments to carry out
17 appropriate activities, actions, and recommendations as coordinated
18 by MPART. Efforts must be continuous to ensure that the
19 department's activities are not duplicative with activities of
20 another department or agency.

21 Sec. 1240. From the funds appropriated in part 1 for chronic
22 disease control and health promotion administration, \$70,000.00 is
23 allocated to support a rare disease advisory council and the
24 responsibilities of the rare disease advisory council, which may
25 include all of the following:

26 (a) Developing a list of rare diseases.

27 (b) Posting the list of rare diseases on the department's
28 website.

29 (c) Updating the list of rare diseases.



1 (d) Annually investigating and reporting to the legislature on
2 1 rare disease on the list, and including legislative
3 recommendations in the report.

4 Sec. 1241. (1) From the funds appropriated in part 1 for
5 community health programs, the department shall support preventive
6 health supports and services in regions with high health care
7 access and outcome disparities. The department shall use the funds
8 appropriated pursuant to this section to provide for all of the
9 following:

10 (a) Financial support for the operation of community-based
11 health clinics. A community-based health clinic shall provide
12 preventive health supports and services, be established in
13 communities with high social vulnerability and health disparities,
14 and be operated in cooperation with trusted community partners with
15 demonstrated experience in serving as an access point for
16 preventive health supports and services.

17 (b) Financial support for the operation of healthy community
18 zones. The healthy community zones must utilize long-term
19 strategies to address access to healthy food, affordable housing,
20 and safety networks.

21 (c) Financial support for the operation of mobile health units
22 to provide preventive health supports and services for individuals
23 residing in areas with high disparities in health care outcomes and
24 access.

25 (2) Not later than March 1 of the current fiscal year, the
26 department shall submit to the standard report recipients a report
27 on the outcome of the community health programs described in
28 subsection (1). The report must include, but is not limited to, all
29 of the following:



1 (a) The list of communities served.

2 (b) The types of health services offered by grant recipients.

3 (c) A spending report from the grant recipients.

4
5 **FAMILY HEALTH SERVICES**

6 Sec. 1301. (1) Not later than April 1 of the current fiscal
7 year, the department shall submit to the standard report recipients
8 a report on planned allocations from the amounts appropriated in
9 part 1 for local MCH services, prenatal care outreach and service
10 delivery support, family planning local agreements, and pregnancy
11 prevention programs. Using applicable federal definitions, the
12 report must include information on all of the following:

13 (a) The funding allocations.

14 (b) The actual number of women, children, and adolescents
15 served and the amounts expended for each group for the previous
16 fiscal year.

17 (c) A breakdown of the expenditure of the funds between urban
18 and rural communities.

19 (2) The department shall ensure that the distribution of funds
20 through the programs described in subsection (1) takes into account
21 the needs of rural communities.

22 (3) As used in this section, "rural community" means any of
23 the following:

24 (a) A county, city, village, or township with a population of
25 30,000 or less.

26 (b) A county, city, village, or township described in
27 subdivision (a), if it is located within a metropolitan statistical
28 area.

29 Sec. 1302. From the funds appropriated in part 1 for special



1 projects, the department shall allocate \$500,000.00 of TANF revenue
2 to purchase child restraint systems for newborn children who are
3 TANF eligible. The child restraint systems must meet the standards
4 of all applicable federal law and the laws of this state, be
5 purchased in volume by this state, and be distributed through
6 maternal infant health program providers.

7 Sec. 1306. (1) From the funds appropriated in part 1 for the
8 drinking water declaration of emergency, the department shall
9 allocate funds to address needs in a city in which a declaration of
10 emergency was issued because of drinking water contamination. The
11 funds allocated under this section may be used to support any of
12 the following activities:

13 (a) Nutrition assistance, nutritional and community education,
14 food bank resources, and food inspections.

15 (b) Epidemiological analysis and case management of
16 individuals at risk of elevated blood lead levels.

17 (c) Support for child and adolescent health centers, and the
18 children's health care access program.

19 (d) Nursing services, breastfeeding education, evidence-based
20 home visiting programs, intensive services, and outreach for
21 children exposed to lead coordinated through local community mental
22 health organizations.

23 (e) Department local office operations costs.

24 (f) Lead poisoning surveillance, investigations, treatment,
25 and abatement.

26 (g) Nutritional incentives provided to local residents through
27 the double up food bucks expansion program.

28 (h) Genesee County health department food inspectors to
29 perform water testing at local food service establishments.



- 1 (i) Transportation related to health care delivery.
- 2 (j) Senior initiatives.
- 3 (k) Lead abatement contractor workforce development.
- 4 (l) Any other activity that the department considers

5 appropriate.

6 (2) From the funds appropriated in part 1 for the drinking
7 water declaration of emergency, the department shall allocate
8 \$500,000.00 for rides to wellness through the Flint mass
9 transportation authority.

10 Sec. 1308. From the funds appropriated in part 1 for prenatal
11 care outreach and service delivery support, the department shall
12 allocate not less than \$500,000.00 for evidence-based programs to
13 reduce infant mortality. The funds must be used for enhanced
14 support and education to nursing teams or other teams of health
15 professionals that the department considers qualified, client
16 recruitment in areas designated as underserved for obstetrical and
17 gynecological services and in other high-need communities,
18 strategic planning to expand and sustain programs, and marketing
19 and communications of programs to raise awareness, engage
20 stakeholders, and recruit nurses.

21 Sec. 1311. From the funds appropriated in part 1 for prenatal
22 care outreach and service delivery support, the department shall
23 allocate not less than \$2,750,000.00 of state general fund/general
24 purpose revenue for a rural home visit program. The department
25 shall give equal consideration to all eligible evidence-based
26 providers in all regions in contracting for rural home visitation
27 services.

28 Sec. 1313. (1) From the funds appropriated in part 1, the
29 department shall continue developing an outreach program on fetal



1 alcohol syndrome services, targeting health promotion, prevention,
2 and intervention.

3 (2) The department shall explore federal grant funding to
4 address prevention services for fetal alcohol syndrome and to
5 reduce alcohol consumption among pregnant women.

6 Sec. 1314. From the funds appropriated in part 1, the
7 department shall enhance the department's education and outreach
8 efforts that encourage women of childbearing age to seek the
9 confirmation of a pregnancy at the earliest indication of a
10 possible pregnancy and to initiate continuous and routine prenatal
11 care on the confirmation of a pregnancy. The department shall
12 ensure that the department's programs, policies, and practices
13 promote prenatal and obstetrical care by doing all of the
14 following:

15 (a) Supporting access to care.

16 (b) Reducing and eliminating barriers to care.

17 (c) Supporting recommendations for best practices.

18 (d) Encouraging optimal prenatal habits, including, but not
19 limited to, prenatal medical visits, use of prenatal vitamins, and
20 the cessation of tobacco use, alcohol use, or drug use.

21 (e) Tracking birth outcomes to study improvements in
22 prevalence of neonatal substance exposure, fetal alcohol syndrome,
23 and other preventable neonatal disease.

24 (f) Tracking maternal increase in healthy behaviors following
25 childbirth.

26 Sec. 1315. From the funds appropriated in part 1 for dental
27 programs, \$200,000.00 is allocated to the Michigan Dental
28 Association for the administration of a volunteer dental program
29 that provides dental services to the uninsured.



1 Sec. 1316. The department shall use revenue from permit fees
2 for mobile dental facilities that the department receives under
3 section 21605 of the public health code, 1978 PA 368, MCL
4 333.21605, to offset the costs of processing and issuing permits
5 for mobile dental facilities.

6 Sec. 1325. From the funds appropriated in part 1 for prenatal
7 care outreach and service delivery support, the department shall
8 allocate \$5,000,000.00 to support grants to local collaboratives to
9 enhance the ability of local collaboratives to coordinate and
10 improve maternal and infant health outcomes. To receive a grant
11 under this section, a local collaborative must be a part of a
12 perinatal quality collaborative.

13 Sec. 1341. The department shall use income eligibility and
14 verification guidelines established by the Food and Nutrition
15 Service agency of the United States Department of Agriculture to
16 determine eligibility of individuals for the special supplemental
17 nutrition program for women, infants, and children (WIC) as stated
18 in current WIC policy.

19 Sec. 1343. (1) From the funds appropriated in part 1 for
20 dental programs, the department shall allocate \$4,260,000.00 of
21 state and local funds, plus any private contributions received to
22 support the program, to establish and maintain the dental oral
23 assessment program described in section 9316 of the public health
24 code, 1978 PA 368, MCL 333.9316.

25 (2) Not later than December 31 of the current fiscal year, the
26 department shall submit a report to the standard report recipients
27 that provides a summary of the dental reports the department
28 receives from principals and administrators under section 9316 of
29 the public health code, 1978 PA 368, MCL 333.9316.



1 Sec. 1348. From the funds appropriated in part 1 for prenatal
2 care outreach and service delivery support, the department shall
3 allocate \$5,000,000.00 to support prenatal health care providers
4 operating in this state to provide services for existing group-
5 based prenatal care programs that include 1 or more health care
6 professionals leading small groups of expectant mothers in the same
7 phase of pregnancy in discussions and for other health services
8 that promote the well-being and health of mothers and babies.

9 Sec. 1349. Subject to federal approval, from the funds
10 appropriated in part 1 for immunization program, the department
11 shall allocate all of the following funds to support a statewide
12 media campaign for improving this state's immunization rates:

13 (a) \$740,000.00 of general fund/general purpose revenue.

14 (b) Any available work project funds.

15 (c) Any available federal match through a contract
16 administered by the department with oversight from the behavioral
17 and physical health and aging services administration and the
18 public health administration.

19
20 **CHILDREN'S SPECIAL HEALTH CARE SERVICES**

21 Sec. 1360. From the funds appropriated in part 1, the
22 department may do 1 or more of the following:

23 (a) Provide special formulas for eligible individuals with
24 specified metabolic and allergic disorders.

25 (b) Provide medical care and treatment to eligible individuals
26 with cystic fibrosis who are 26 years of age or older.

27 (c) Provide medical care and treatment to eligible individuals
28 with hereditary coagulation defects, commonly known as hemophilia,
29 who are 26 years of age or older.



1 (d) Provide human growth hormone to eligible individuals.

2 (e) Provide mental health care to eligible individuals for
3 mental health needs that result from, or are a symptom of, the
4 individual's qualifying medical condition.

5 (f) Provide medical care and treatment to eligible individuals
6 with sickle cell disease who are 26 years of age or older.

7 Sec. 1361. From the funds appropriated in part 1 for medical
8 care and treatment, the department may spend the funds to continue
9 developing and expanding telemedicine capacity to allow families
10 with children in the children's special health care services
11 program to access specialty providers more readily and in a more
12 timely manner. The department may spend funds to support chronic
13 complex care management of children enrolled in the children's
14 special health care services program to minimize hospitalizations
15 and reduce costs to the program while improving outcomes and
16 quality of life. As used in this section, "children's special
17 health care services program" or "program" means the program
18 established under section 5815 of the public health code, 1978 PA
19 368, MCL 333.5815.

20
21 **AGING SERVICES**

22 Sec. 1402. The department may encourage the Food Bank Council
23 of Michigan to collaborate directly with each area agency on aging
24 and any other organization that provides senior nutrition services
25 to secure the food access of older adults.

26 Sec. 1403. (1) From the funds appropriated in part 1, the
27 department may implement a palliative care advisory task force. The
28 palliative care advisory task force shall do all of the following:

29 (a) Provide the legislature with a recommended definition for



1 palliative care in this state.

2 (b) Conduct research on palliative care.

3 (c) Make recommendations that will expand the provision of
4 palliative care.

5 (d) Identify palliative care services that are offered and
6 measures for reimbursement of the services.

7 (e) Develop key program metrics for palliative care services
8 and make recommendations to the department and the legislature.

9 (f) Collaborate with individuals who are able to improve and
10 expand high-quality palliative care services.

11 (g) Develop engagement strategies to educate the public on
12 access to palliative care and to improve an individual's ability to
13 make informed decisions on preferred care.

14 (h) Identify the capacity of palliative care providers to
15 provide palliative care services.

16 (2) If the department implements the task force described in
17 subsection (1), then not later than January 1 of the current fiscal
18 year, the palliative care advisory task force shall submit to the
19 standard report recipients a report that identifies the palliative
20 care services available in this state and any palliative care
21 services that are not offered in this state but would provide a
22 benefit.

23 Sec. 1404. From the funds appropriated in part 1 for community
24 services, the department shall allocate \$658,000.00 to area
25 agencies on aging for home and community-based services.

26 Sec. 1417. Not later than March 31 of the current fiscal year,
27 the department shall submit to the standard report recipients a
28 report that contains all of the following information:

29 (a) The total allocation of state resources made to each area



1 agency on aging by individual program and administration.

2 (b) Detailed expenditures by each area agency on aging by
3 individual program and administration, including both state-funded
4 resources and locally funded resources.

5 Sec. 1421. From the funds appropriated in part 1 for community
6 services, \$1,100,000.00 is allocated for locally determined needs
7 that are provided by area agencies on aging.

8
9 **HEALTH AND AGING SERVICES ADMINISTRATION**

10 Sec. 1505. Not later than March 1 of the current fiscal year,
11 the department shall submit a report to the standard report
12 recipients on the actual reimbursement savings and cost offsets
13 that have resulted from the funds appropriated in part 1 for the
14 office of inspector general and third-party liability efforts in
15 the previous fiscal year.

16 Sec. 1507. From the funds appropriated in part 1 for office of
17 inspector general, the inspector general shall audit and recoup
18 inappropriate or fraudulent payments from Medicaid managed care
19 organizations to health care providers. Unless authorized by
20 federal law or a law of this state, the department shall not fine,
21 temporarily halt operations of, disenroll as a Medicaid provider,
22 or terminate a managed care organization or health care provider
23 from providing services due to the discovery of an inappropriate
24 payment found during the course of an audit.

25 Sec. 1512. From the funds appropriated in part 1, the
26 department shall maintain the Medicaid encounter quality initiative
27 report to separate nonclinical administrative costs from actual
28 claims and encounter costs.

29 Sec. 1518. The department shall coordinate with the department



1 of licensing and regulatory affairs to ensure that, on the issuance
2 of an order suspending the license of an adult foster care
3 facility, home for the aged, or nursing home, the department of
4 licensing and regulatory affairs provides a notice to the
5 department, to the house and senate appropriations subcommittees on
6 the department budget, to the house and senate appropriations
7 subcommittees on the department of licensing and regulatory affairs
8 budget, and to the members of the house of representatives and
9 senate that represent the legislative districts of the county in
10 which the adult foster care facility, home for the aged, or nursing
11 home is located.

12 13 **HEALTH SERVICES**

14 Sec. 1605. The protected income level for Medicaid coverage
15 determined under section 106(1)(b)(iii) of the social welfare act,
16 1939 PA 280, MCL 400.106, is 100% of the related public assistance
17 standard.

18 Sec. 1606. For the purpose of guardian and conservator
19 charges, the department may deduct up to \$83.00 per month as an
20 allowable expense against a recipient's income when determining
21 Medicaid eligibility and patient pay amounts.

22 Sec. 1607. (1) The department shall immediately presume that
23 an applicant for Medicaid whose qualifying condition is pregnancy
24 is eligible for Medicaid coverage, unless the preponderance of
25 evidence in the applicant's application indicates otherwise. The
26 applicant who is qualified as described in this subsection is
27 allowed to select or remain with the Medicaid participating
28 obstetrician of the applicant's choice.

29 (2) Each qualifying applicant is entitled to receive all



1 medically necessary obstetrical and prenatal care without
2 preauthorization from a health plan. All claims submitted for
3 payment for obstetrical and prenatal care must be paid at the
4 Medicaid fee-for-service rate if a contract does not exist between
5 the Medicaid participating obstetrical or prenatal care provider
6 and the managed care plan. The applicant must receive a listing of
7 Medicaid physicians and managed care plans in the immediate
8 vicinity of the applicant's residence.

9 (3) If an applicant, presumed to be eligible for Medicaid
10 under subsection (1), is subsequently found to be ineligible, a
11 Medicaid physician or managed care plan that has been providing
12 pregnancy services to the applicant is entitled to reimbursement
13 for the services until the Medicaid physician or managed care plan
14 is notified by the department that the applicant was found to be
15 ineligible for Medicaid.

16 (4) If the preponderance of evidence in an application under
17 subsection (1) indicates that the applicant is not eligible for
18 Medicaid, the department shall refer the applicant to the nearest
19 public health clinic or similar entity as a potential source for
20 receiving pregnancy-related services.

21 (5) The department shall develop an enrollment process for
22 applicants covered under this section that facilitates the
23 selection of a managed care plan at the time of application.

24 (6) The department shall require that Medicaid managed care
25 plans enroll women whose qualifying condition for Medicaid is
26 pregnancy.

27 (7) The department shall encourage physicians to provide an
28 applicant whose qualifying condition for Medicaid is pregnancy with
29 a referral to a Medicaid participating dentist at the applicant's



1 first pregnancy-related appointment.

2 Sec. 1611. (1) For care provided to Medicaid recipients with
3 other third-party sources of payment, Medicaid reimbursement shall
4 not exceed, in combination with such other resources, including
5 Medicare, those amounts established for Medicaid-only patients. The
6 Medicaid payment rate shall be accepted as payment in full. Other
7 than an approved Medicaid copayment, no portion of a provider's
8 charge shall be billed to the recipient or any person acting on
9 behalf of the recipient. This section does not affect the level of
10 payment from a third-party source other than the Medicaid program.
11 The department shall require a nonenrolled provider to accept
12 Medicaid payments as payment in full.

13 (2) Notwithstanding subsection (1), if a hospital service is
14 provided to a dual Medicare/Medicaid recipient with only Medicare
15 part B coverage, the Medicaid reimbursement must equal, when
16 combined with a payment for Medicare or other third-party source of
17 payment, the amount established for a Medicaid-only patient,
18 including a capital payment.

19 Sec. 1620. (1) If a Medicaid claim is a fee-for-service
20 Medicaid claim, the professional dispensing fee for a drug that is
21 listed as a medication on the Michigan pharmaceutical products list
22 is \$20.02 or the pharmacy's submitted dispensing fee, whichever is
23 less.

24 (2) If a Medicaid claim is a fee-for-service Medicaid claim,
25 the professional dispensing fee for a drug that is not listed as a
26 specialty medication on the Michigan pharmaceutical products list
27 is as follows:

28 (a) If the drug is indicated as preferred on the department's
29 preferred drug list, \$10.80 or the pharmacy's submitted dispensing



1 fee, whichever is less.

2 (b) If the drug is not on the department's preferred drug
3 list, \$10.64 or the pharmacy's submitted dispensing fee, whichever
4 is less.

5 (c) If the drug is indicated as nonpreferred on the
6 department's preferred drug list, \$9.00 or the pharmacy's submitted
7 dispensing fee, whichever is less.

8 Sec. 1621. From the funds appropriated in part 1 for health
9 plan services, Healthy Michigan plan, and pharmaceutical services,
10 the department shall utilize generic and biosimilar pharmaceuticals
11 on this state's preferred drug list for both Medicaid managed care
12 and fee-for-service.

13 Sec. 1622. From the funds appropriated in part 1 for health
14 plan services, Healthy Michigan plan, and pharmaceutical services
15 the department shall incorporate any applicable aspects of federal
16 Executive Order No. 14297, 90 Fed. Reg. 20749 (May 12, 2025)
17 "Delivering Most-Favored-Nation Prescription Drug Pricing to
18 American Patients", for both Medicaid managed care and fee-for-
19 service pharmaceutical products specifically seeking most-favored-
20 nation pricing for high-cost pharmaceuticals on this state's
21 preferred drug list.

22 Sec. 1626. (1) Not later than January 15 of the current fiscal
23 year, each pharmacy benefit manager that receives reimbursements
24 directly, through a department-administered fee-for-services
25 contract, or through a Medicaid health plan, from the funds
26 appropriated in part 1 for health services must submit all of the
27 following information to the department for the previous fiscal
28 year:

29 (a) The total number of prescriptions that were dispensed.



1 (b) The aggregate fiscal year paid pharmacy claims repriced
2 using the wholesale acquisition cost for each drug on its
3 formulary.

4 (c) The aggregate amount of rebates, discounts, and price
5 concessions that the pharmacy benefit manager received for each
6 drug on its formulary. The aggregate amount of rebates must include
7 any utilization discounts the pharmacy benefit manager received
8 from a manufacturer.

9 (d) The aggregate amount of administrative fees that the
10 pharmacy benefit manager received from all pharmaceutical
11 manufacturers.

12 (e) The aggregate amount identified in subdivisions (b) and
13 (c) that were retained by the pharmacy benefit manager and did not
14 pass through to the department or to the Medicaid health plan.

15 (f) The aggregate amount of reimbursements the pharmacy
16 benefit manager paid to contracting pharmacies.

17 (g) Any other information considered necessary by the
18 department.

19 (2) Not later than March 1 of the current fiscal year, the
20 department shall submit a report including the information provided
21 under subsection (1) to the standard report recipients.

22 (3) Any nonaggregated information submitted under this section
23 is confidential and must not be disclosed to any person by the
24 department. The information described in this subsection is not a
25 public record of the department.

26 Sec. 1628. From the funds appropriated in part 1 for hospital
27 services and therapy and Healthy Michigan plan, the department
28 shall continue to allocate \$3,000,000.00 in general fund/general
29 purpose revenue and any associated federal match to maintain the



1 Medicaid reimbursement rate for dental services provided at
2 ambulatory surgical centers and outpatient hospitals. The funding
3 provided in this section must be used to maintain the minimum rate
4 of reimbursement for dental services provided in ambulatory
5 surgical centers at \$1,495.00 and maintain the minimum rate of
6 reimbursement for dental services provided in outpatient hospitals
7 at \$2,300.00.

8 Sec. 1629. The department shall utilize maximum allowable cost
9 pricing for generic drugs that is based on wholesaler pricing to
10 providers. The wholesaler pricing must be based on the price
11 available from at least 2 wholesalers who deliver drugs in this
12 state.

13 Sec. 1630. Not later than April 1 of the current fiscal year,
14 from the funds appropriated in part 1 for Medicaid dental services,
15 the department shall submit a report to the standard report
16 recipients on the dental service benefit. The report must cover all
17 of the following areas:

18 (a) Information on the implementation of the Adult Medicaid
19 dental benefit redesign including all of the following information:

20 (i) The number of dental providers, by Medicaid health plan in
21 this state, who provided 1 or more Medicaid dental services in the
22 fiscal year ending September 30, 2022, and the number of additional
23 providers who were added in the previous fiscal year, with a
24 delineation in the reported numbers based on the average payment
25 per visit and before and after the implementation of the Adult
26 Medicaid dental benefit redesign.

27 (ii) The status of enhanced care coordination.

28 (iii) The array of covered dental benefits and services before
29 the Adult Medicaid dental benefit redesign and how the available



benefits and services changed or expanded after the Adult Medicaid dental benefit redesign.

(b) Information on the Healthy Kids Dental program including all of the following information:

(i) The number of children enrolled in the Healthy Kids Dental program who visited the dentist in the previous fiscal year broken down by dental benefit manager.

(ii) The number of dentists who accept payment from the Healthy Kids Dental program broken down by dental benefit manager.

(iii) The annual change in dental utilization of children enrolled in the Healthy Kids Dental program broken down by dental benefit manager.

(iv) Service expenditures for the Healthy Kids Dental program broken down by dental benefit manager.

(v) Administrative expenditures for the Healthy Kids Dental program broken down by dental benefit manager.

Sec. 1631. (1) The department shall require copayments on dental, podiatric, and vision services provided to Medicaid recipients, except as prohibited by federal law or a law of this state.

(2) Except as otherwise prohibited by federal law or a law of this state, the department shall require Medicaid recipients to pay the following copayments:

(a) Two dollars for a physician office visit.

(b) Three dollars for a hospital emergency room visit.

(c) Fifty dollars for the first day of an inpatient hospital stay.

(d) Two dollars for an outpatient hospital visit.

(e) One dollar for a generic drug or any drug indicated as



1 preferred on the department's preferred drug list and \$3.00 for a
2 brand-name drug not indicated as preferred on the department's
3 preferred drug list.

4 Sec. 1633. (1) The department shall provide adequate guidance
5 to insurance providers to ensure compliance with section 71117 of
6 Public Law 119-21.

7 (2) For the first quarterly insurance provider assessment
8 payment due after the end of the transition period provided on
9 November 14, 2025 in CMS correspondence regarding a revised
10 insurance provider assessment tax structure, the department and the
11 department of treasury shall work with insurance providers to
12 establish a reasonable time period for submission of that payment.

13 Sec. 1640. From the funds appropriated in part 1, the
14 department shall maintain the rate increase for the home help
15 individual caregiver rate and the home help agency provider rate
16 specified in the department's Medicaid provider letters L 24-66, L
17 24-67, and L 24-74.

18 Sec. 1641. An institutional provider that is required to
19 submit a cost report under the Medicaid program shall submit cost
20 reports completed in full not more than 5 months after the end of
21 the institutional provider's fiscal year.

22 Sec. 1644. (1) From the funds appropriated in part 1, the
23 department shall maintain wages at a level not less than the amount
24 in effect the previous fiscal year. This funding must include all
25 costs incurred by the employer, including, but not limited to,
26 payroll taxes, due to the wage increase. As used in this
27 subsection, "direct care workers" means a registered professional
28 nurse, licensed practical nurse, competency-evaluated nursing
29 assistant, and respiratory therapist.



1 (2) From the funds appropriated in part 1, the department
2 shall maintain wages at a level not less than the amount in effect
3 during the previous fiscal year for direct care workers who are
4 employed by licensed adult foster care facilities and licensed
5 homes for the aged and who provide Medicaid-funded fee-for-service
6 personal care services that were not eligible for any direct care
7 worker pay adjustment under Medicaid-funded managed care. This
8 funding must include all costs incurred by the employer, including,
9 but not limited to, payroll taxes, due to the wage increase.

10 Sec. 1645. (1) From the funds appropriated in part 1, the
11 department shall maintain the wages of eligible nonclinical staff
12 employed by skilled nursing facilities. The funding must include
13 all costs incurred by the employer, including payroll taxes, due to
14 prior wage increases.

15 (2) The nonclinical staff eligible for the wages described in
16 subsection (1) are those whose costs are reported in the following
17 job classifications in nursing facility institutional cost reports
18 shared with the department:

- 19 (a) Other housekeeping.
- 20 (b) Other maintenance worker.
- 21 (c) Other plant operations.
- 22 (d) Other laundry.
- 23 (e) Dining room assistants.
- 24 (f) Other dietary workers.
- 25 (g) Other medical records.
- 26 (h) Other social services.
- 27 (i) Other diversion therapy.
- 28 (j) Beauty and barber.
- 29 (k) Gift, flower, coffee, and canteen worker.



1 Sec. 1646. From the funds appropriated in part 1, the
2 department shall maintain the Medicaid reimbursement rates for
3 orthotic and prosthetic providers in place in the previous fiscal
4 year.

5 Sec. 1647. By February 1 of the current fiscal year, the
6 department shall submit a comprehensive report to the standard
7 report recipients detailing the use of all home and community-based
8 services funds received under section 9817 of Public Law 117-2. The
9 report must include, but is not limited to, all of the following:

10 (a) Total funds received and total expenditures by fiscal
11 year.

12 (b) Expenditures by category and by vendor or grantee.

13 (c) Program accomplishments and progress.

14 (d) Any unspent balances and projected future spending.

15 (e) A list of active contracts and grants associated with home
16 and community-based services funding.

17 Sec. 1657. (1) The department shall not make reimbursement for
18 Medicaid to screen and stabilize a Medicaid recipient, including
19 stabilization of a psychiatric crisis, in a hospital emergency
20 room, contingent on obtaining prior authorization from the
21 recipient's HMO. If the recipient is discharged from the emergency
22 room, the hospital shall notify the recipient's HMO within 24 hours
23 of the diagnosis and treatment received.

24 (2) If the treating hospital determines that the recipient
25 will require further medical service or hospitalization beyond the
26 point of stabilization, that hospital shall receive authorization
27 from the recipient's HMO prior to admitting the recipient.

28 (3) Subsections (1) and (2) do not require an alteration to an
29 existing agreement between an HMO and its contracting hospitals and



1 do not require an HMO to reimburse for services that are not
2 considered to be medically necessary.

3 Sec. 1662. (1) From the funds appropriated in part 1, the
4 department shall require an annual external quality review of each
5 contracting HMO. The external quality review must analyze and
6 evaluate aggregated information on quality, timeliness, and access
7 to health care services that the HMO or its contractors furnish to
8 Medicaid beneficiaries. The department shall create a report
9 containing each quality review required under this subsection.

10 (2) The department shall require Medicaid HMOs to provide
11 EPSDT utilization data through the encounter data system, and HEDIS
12 well child health measures in accordance with the National
13 Committee for Quality Assurance prescribed methodology.

14 (3) The department shall submit a copy of the analysis of the
15 Medicaid HMO annual audited reports on HEDIS and the report under
16 subsection (1) to the standard report recipients within 30 days
17 after the department's receipt of the final information required
18 from the contractors.

19 Sec. 1670. (1) The appropriation in part 1 for the MICHild
20 program is to be used to provide comprehensive health care to all
21 children under age 19 who reside in families with an income at or
22 below 212% of the federal poverty level, who are uninsured and have
23 not had coverage by other comprehensive health insurance within 6
24 months of applying for MICHild benefits, and who are residents of
25 this state. The department shall develop detailed eligibility
26 criteria through the behavioral and physical health and aging
27 services administration public concurrence process. The eligibility
28 criteria must be consistent with the provisions of this part and
29 part 1.



1 (2) The department shall provide up to 1 year of continuous
2 eligibility to a child eligible for the MIChild program unless the
3 child reaches age 19.

4 Sec. 1677. From the funds appropriated in part 1 for the
5 MIChild program, the department shall provide, at a minimum, all
6 benefits available under the Michigan benchmark plan that are
7 delivered through contracted providers and consistent with federal
8 law, including, but not limited to, the following medically
9 necessary services:

10 (a) Inpatient mental health services, other than substance use
11 disorder treatment services, including services furnished in a
12 state-operated mental hospital and residential or other 24-hour
13 therapeutically planned structured services.

14 (b) Outpatient mental health services, other than substance
15 use disorder services, including services furnished in a mental
16 hospital operated by this state and community-based services.

17 (c) Durable medical equipment and prosthetic and orthotic
18 devices.

19 (d) Dental services as outlined in the approved MIChild state
20 plan.

21 (e) Substance use disorder treatment services that may include
22 inpatient, outpatient, and residential substance use disorder
23 treatment services.

24 (f) Care management services for mental health diagnoses.

25 (g) Physical therapy, occupational therapy, and services for
26 individuals with speech, hearing, and language disorders.

27 (h) Emergency ambulance services.

28 Sec. 1682. (1) In addition to the appropriations in part 1,
29 the department is authorized to receive and spend penalty money



1 received as the result of noncompliance with Medicaid certification
2 regulations. Penalty money, characterized as private funds,
3 received by the department shall increase authorizations and
4 allotments in the long-term care accounts.

5 (2) Any unexpended penalty money, at the end of the year, must
6 carry forward to the following year.

7 (3) Not later than March 1 of the current fiscal year, the
8 department shall report to the standard report recipients, and to
9 the house and senate appropriations subcommittees on the department
10 of licensing and regulatory affairs budget, on penalty money
11 received by the department as described in subsection (1). The
12 report must include, but is not limited to, the following
13 information:

14 (a) The amount of penalty monies received by the department in
15 the previous fiscal year listed by the assessed entity.

16 (b) A list of the entities that were assessed penalties in the
17 previous fiscal year with the rationale for each penalty.

18 Sec. 1692. (1) The department is authorized to pursue
19 reimbursement for eligible services provided in Michigan schools
20 from the federal Medicaid program. The department and the state
21 budget director are authorized to negotiate and enter into
22 agreements, together with the department of education, with local
23 and intermediate school districts regarding the sharing of federal
24 Medicaid services funds received for these services. The department
25 is authorized to receive and disburse funds to participating school
26 districts pursuant to agreements described in this subsection and
27 pursuant to federal law and a law of this state.

28 (2) From the funds appropriated in part 1 for health services
29 school-based services payments, the department is authorized to do



1 all of the following:

2 (a) Finance activities within the behavioral and physical
3 health and aging services administration related to eligible
4 services.

5 (b) Reimburse participating school districts pursuant to the
6 fund-sharing ratios negotiated in the state-local agreements
7 authorized in subsection (1).

8 (c) Offset general fund costs associated with the Medicaid
9 program.

10 Sec. 1694. From the funds appropriated in part 1 for special
11 Medicaid reimbursement, \$2,628,500.00 of general fund/general
12 purpose revenue and any associated federal match must be
13 distributed for poison control services to an academic health care
14 system that has a high volume of providing care to indigent
15 individuals.

16 Sec. 1697. The department shall require that Medicaid health
17 plans administering adult dental benefits maintain a network of
18 dental providers in sufficient numbers, mix, and geographic
19 locations throughout their respective service areas in order to
20 provide adequate dental care for Healthy Michigan plan enrollees.

21 Sec. 1700. Not later than December 1 of the current fiscal
22 year, the department shall report to the standard report recipients
23 on the distribution of funding provided, and the net benefit if the
24 special hospital payment is not financed with general fund/general
25 purpose revenue, to each eligible hospital during the previous
26 fiscal year from the following special hospital payments:

27 (a) GME.

28 (b) Special rural hospital payments provided under section
29 1802(1)(b) of this part.



1 (c) Lump-sum payments to rural hospitals for obstetrical care
2 provided under section 1802(1)(a) of this part.

3 Sec. 1702. From the funds appropriated in part 1, the
4 department shall allocate \$2,830,000.00 in general fund/general
5 purpose revenue and any associated federal match to maintain the
6 rates in place in the previous fiscal year for private duty nursing
7 services for Medicaid beneficiaries under the age of 21. These
8 additional funds must be used to attract and retain highly
9 qualified registered nurses and licensed practical nurses to
10 provide private duty nursing services so that medically fragile
11 individuals can be cared for in the most homelike setting possible.

12 Sec. 1757. The department shall obtain proof from all Medicaid
13 recipients that they are United States citizens or otherwise
14 legally residing in this country and that they are residents of
15 this state before approving Medicaid eligibility.

16 Sec. 1764. The department's contracted actuaries shall
17 annually certify whether rates paid to Medicaid health plans and
18 specialty PIHPs are actuarially sound in accordance with federal
19 requirements. The department shall provide to the standard report
20 recipients a copy of the rate certification required under this
21 section and the approval of rates paid to Medicaid health plans and
22 specialty PIHPs for any fiscal year not later than October 1 for
23 Medicaid capitation rate certifications and not later than November
24 15, February 15, May 15, and August 15 for any Medicaid capitation
25 rate amendments. Following the rate certification, the department
26 shall ensure that any new or revised policies with a substantive
27 impact on capitation rates are reviewed by the contracted actuaries
28 and considered for future rate amendments.

29 Sec. 1786. From the funds appropriated in part 1, the



1 department shall maintain Medicaid reimbursement for the
2 administration of injectable, nasal, and oral vaccines at \$23.03.

3 Sec. 1787. From the funds appropriated in part 1 for health
4 plan services, Healthy Michigan plan, and long-term care services,
5 the department shall maintain the Medicaid reimbursement rates in
6 place in the previous fiscal year for CPT codes 31579, 92507,
7 92508, 92520, 92521, 92522, 92523, 92524, 92526, 92597, 92607,
8 92608, 92609, 92610, 92630, 92633, 92700, 94010, 97129, 97130,
9 97533, 97799, G2250, G2251, and S9152.

10 Sec. 1788. From the funds appropriated in part 1, the
11 department shall provide Medicaid reimbursement rates, including
12 Medicaid reimbursements from the ambulance provider quality
13 assurance assessment, for ground ambulance services at not less
14 than 100% of the Medicare base rates for Locality 01 for ground
15 ambulance services in effect on January 1, 2023.

16 Sec. 1789. From the funds appropriated in part 1 for federally
17 qualified health centers, the department shall allocate not less
18 than \$11,300,000.00 in general fund/general purpose revenue and any
19 associated federal match to maintain Medicaid prospective payment
20 system reimbursement rates.

21 Sec. 1790. The department shall maintain the current
22 practitioner rates paid for CPT codes 90791 through 90899 for
23 psychiatric procedures through Medicaid fee-for-service and through
24 the comprehensive Medicaid health plans for psychiatric procedures
25 provided for Medicaid recipients under the age of 21.

26 Sec. 1791. From the funds appropriated in part 1 for health
27 plan services and physician services, the department shall provide
28 Medicaid reimbursement rates for neonatal services at 100% of the
29 Medicare rate received for those services in effect on the date the



1 services are provided to eligible Medicaid recipients. The neonatal
2 services and physician services eligible for reimbursement rates
3 under this section are described as CPT codes 99468, 99469, 99471,
4 99472, 99475, 99476, 99477, 99478, 99479, and 99480.

5 Sec. 1792. By April 30 of the current fiscal year, the
6 department shall evaluate pharmacy encounter data through the first
7 2 quarters of the fiscal year to determine, in consultation with
8 the Medicaid health plans, if rates must be recertified. By May 30
9 of the current fiscal year, the department shall report the
10 evaluation results to the standard report recipients and the
11 Medicaid health plans.

12 Sec. 1793. (1) The department shall follow the guidance of the
13 CMS regarding the medical loss ratio, capitation rate development,
14 community reinvestment, and other financial reporting.

15 (2) The department shall not require Medicaid health plans to
16 use investment income as part of its medical loss ratio, capitation
17 rate development, community reinvestment, and other financial
18 reporting.

19 Sec. 1794. (1) From the funds appropriated in part 1, the
20 department shall provide Medicaid reimbursements for hospital-based
21 substance use disorder peer-supports.

22 (2) Not later than March 1 of the current fiscal year, the
23 department shall report to the standard report recipients on the
24 statewide amounts and each hospital amount for hospital-based
25 substance use disorder peer-supports during the first quarter of
26 the current fiscal year, including for all of the following:

- 27 (a) The number of individuals served.
28 (b) The Medicaid reimbursement utilization.
29 (c) The total expenditures.



1 Sec. 1801. From the funds appropriated in part 1 for physician
2 services and health plan services, the department shall continue
3 the increase to Medicaid rates for primary care services provided
4 only by primary care providers. The department shall not provide
5 the increase to Medicaid rates under this section to primary care
6 providers whose primary practice is as a non-primary-care
7 subspecialty. As used in this section, "primary care provider"
8 means a physician, or a practitioner working in collaboration with
9 a physician, who is either licensed under part 170 or part 175 of
10 the public health code, 1978 PA 368, MCL 333.17001 to 333.17097 and
11 333.17501 to 333.17556, and who works as a primary care provider in
12 general practice or is board-eligible or certified with a specialty
13 designation of family medicine, general internal medicine, or
14 pediatric medicine, or is a provider who provides the department
15 with documentation of equivalency.

16 Sec. 1802. (1) From the funds appropriated in part 1 for
17 hospital services and therapy, the department shall provide for the
18 following:

19 (a) \$8,470,200.00 in general fund/general purpose revenue as
20 lump-sum payments to noncritical access hospitals that qualified
21 for rural hospital access payments in fiscal year 2013-2014
22 proportional to Medicaid deliveries performed during the fiscal
23 year before the previous fiscal year. Payment amounts must be based
24 on the volume of obstetrical care cases and newborn care cases for
25 all such cases billed by each qualified hospital in the most recent
26 year for which data is available. The department shall make
27 payments not later than January 1 of the current fiscal year. For
28 the current fiscal year, a hospital that met established occupied
29 bed criteria based on Medicaid cost reports as of the fiscal year



1 ending September 30, 2011, and that is located within a county with
2 a population of not more than 195,000 and within a city, village,
3 or township with a population of not more than 15,000, according to
4 the 2020 federal decennial census, is eligible.

5 (b) \$15,204,800.00 in general fund/general purpose revenue and
6 any associated federal match awarded as rural access payments to
7 noncritical access hospitals that meet criteria established by the
8 department for services to low-income rural residents. One of the
9 reimbursement components of the criteria established by the
10 department under this subsection must be assistance with labor and
11 delivery services.

12 (2) Payments under this section must be made by January 1 of
13 the current fiscal year.

14 (3) The department shall publish the distribution of payments
15 for the current fiscal year and the previous fiscal year.

16 Sec. 1804. The department may utilize the federal public
17 assistance reporting information system to continue to work to
18 identify Medicaid recipients who are veterans and who may be
19 eligible for federal veterans' health care benefits or other
20 benefits and shall continue to refer veterans to the department of
21 military and veterans affairs for assistance in securing additional
22 benefits.

23 Sec. 1809. (1) From the funds appropriated in part 1 for
24 health plan services and physician services, the department shall
25 allocate not less than \$565,200.00 of general fund/general purpose
26 revenue and any associated federal matching funds to provide
27 perinatal and gynecological services if perinatal and gynecological
28 services are provided by a perinatal or gynecological professional
29 who is licensed, registered, or otherwise authorized to practice in



1 this state, including, but not limited to, a licensed midwife
2 acting within the scope of the licensed midwife's license. Medicaid
3 reimbursement must be paid when gynecological or perinatal care
4 service is provided in a medical care facility, hospital or
5 freestanding birth center licensed under article 17 of the public
6 health code, 1978 PA 368, MCL 333.20101 to 333.22260, midwifery
7 care facility, or home setting. The rates paid to perinatal or
8 gynecological professionals described in this section must be the
9 same as those paid to other perinatal or gynecological
10 professionals, regardless of the location of those services.

11 (2) The perinatal or gynecological care services described in
12 subsection (1) must meet all of the following:

13 (a) Promote high-quality, cost-effective, and evidence-based
14 care.

15 (b) Promote high-value, evidence-based payment models.

16 (c) Prevent risk in subsequent pregnancies.

17 Sec. 1810. In advance of the annual rate setting development,
18 Medicaid health plans must be given at least 60 days to dispute and
19 correct any discarded encounter data before rates are certified.
20 The department shall notify each contracting Medicaid health plan
21 of any encounter data that have not been accepted for the purposes
22 of rate setting.

23 Sec. 1812. Not later than June 1 of the current fiscal year,
24 and using the most recent available cost reports, the department
25 shall complete a report of all direct and indirect costs associated
26 with residency training programs for each hospital that receives
27 funds appropriated in part 1 for graduate medical education or
28 through the MiDocs consortium. The report shall be submitted to the
29 standard report recipients.



1 Sec. 1820. (1) In order to avoid duplication of effort, if a
2 Medicaid health plan has been reviewed and accredited by a national
3 accrediting entity for health care services, the department shall
4 use applicable national accreditation review criteria to determine
5 compliance with corresponding requirements in this state.

6 (2) The department shall continue to comply with federal law
7 and laws of this state and shall not initiate an action that
8 negatively impacts beneficiary safety.

9 (3) As used in this section, "national accrediting entity"
10 means the National Committee for Quality Assurance, the URAC,
11 formerly known as the Utilization Review Accreditation Commission,
12 or another appropriate entity, as approved by the department.

13 Sec. 1831. From the funds appropriated in part 1 for hospital
14 services and therapy and prenatal care outreach and service
15 delivery support, the department shall allocate \$10,000,000.00 to
16 continue to support hospitals in this state to improve maternal
17 safety and outcomes by administering and expanding a data-driven
18 maternal safety and quality improvement initiative that is based on
19 interdisciplinary and consensus-based practices. The initiative
20 expansion must focus on mitigating pregnancy-associated injury and
21 death, work to improve outcomes for underserved groups, and address
22 problems related to substance use disorders.

23 Sec. 1837. The department shall continue, and expand where
24 appropriate, utilization of telemedicine and telepsychiatry as
25 strategies to increase access to services for Medicaid recipients.

26 Sec. 1846. From the funds appropriated in part 1 for graduate
27 medical education, the department shall distribute the funds with
28 an emphasis on the following health care workforce goals:

29 (a) The encouragement of the training of physicians in



1 specialties, including primary care, that are necessary to meet the
2 future needs of residents of this state.

3 (b) The training of physicians in settings that include
4 ambulatory sites and rural locations.

5 (c) The training of practitioners providing pediatric
6 psychiatry services.

7 Sec. 1850. The department may allow Medicaid health plans to
8 assist with maintaining eligibility through outreach activities to
9 ensure continuation of Medicaid eligibility and enrollment in
10 managed care. The assistance may include mailings, telephone
11 contact, or face-to-face contact with beneficiaries enrolled in the
12 individual Medicaid health plan. Medicaid health plans may offer
13 assistance in completing paperwork for beneficiaries enrolled in
14 the Medicaid health plan.

15 Sec. 1854. The funds appropriated in part 1 for PACE must
16 support a current fiscal year enrollment cap that is not less than
17 9,590.

18 Sec. 1855. From the funds appropriated in part 1 for PACE, to
19 the extent that funding is available in the PACE line item and
20 unused program slots are available, the department may do the
21 following:

22 (a) Increase the number of slots for a local and already-
23 established PACE if the local PACE has provided appropriate
24 documentation to the department indicating its ability to expand
25 capacity to provide services to additional PACE clients.

26 (b) Suspend the 10 member per month individual PACE enrollment
27 increase cap in order to allow unused and unobligated slots to be
28 allocated to address unmet demand for PACE services.

29 Sec. 1856. (1) From the funds appropriated in part 1 for



1 hospice services, \$5,000,000.00 shall be expended to provide room
2 and board for Medicaid-eligible individuals who meet hospice
3 eligibility requirements and receive services at Medicaid enrolled
4 hospice residences in this state. The department shall distribute
5 funds through grants based on the total beds located in all
6 eligible residences that have been providing these services as of
7 October 1, 2017. An eligible grant applicant may inform the
8 department of the applicant's request to reduce the grant amount
9 allocated for the applicant's residence and the funds must be
10 distributed proportionally to increase the total grant amount of
11 the remaining grant-eligible residences. Grant amounts shall be
12 paid out monthly with 1/12 of the total grant amount distributed
13 each month to the grantees.

14 (2) Not later than September 15 of the current fiscal year,
15 each Medicaid-enrolled hospice with a residence that receives funds
16 under this section shall provide a report to the department on the
17 utilization of the grant funding provided in subsection (1). The
18 report must be provided in a format prescribed by the department
19 and must include the following information:

20 (a) The number of patients served.

21 (b) The number of days served.

22 (c) The daily room and board rates for the patients served.

23 (d) If there is not sufficient funding to cover the total room
24 and board need, the number of patients who did not receive care due
25 to insufficient grant funding.

26 (3) If funds awarded under this section remain unused at the
27 end of the current fiscal year, the Medicaid-enrolled hospice with
28 a residence shall return those unused funds to this state.

29 Sec. 1859. The department shall partner with the Michigan



1 Association of Health Plans and Medicaid health plans to develop
2 and implement strategies for the use of information technology
3 services for Medicaid research activities. The department shall
4 make available state medical assistance program data, including
5 Medicaid behavioral data, to the Michigan Association of Health
6 Plans and Medicaid health plans or any vendor considered qualified
7 by the department to perform research activities consistent with
8 this state's goals of improving health; increasing the quality,
9 reliability, availability, and continuity of care; and reducing the
10 cost of care for the eligible population of Medicaid recipients.

11 Sec. 1862. From the funds appropriated in part 1, the
12 department shall maintain payment rates for Medicaid obstetrical
13 services at 95% of Medicare levels.

14 Sec. 1870. (1) From the funds appropriated in part 1 for
15 hospital services and therapy, the department shall allocate
16 \$6,400,000.00 in general fund/general purpose revenue plus any
17 contributions from public entities, up to \$5,000,000.00, and any
18 associated federal match to the MiDocs consortium to create new
19 primary care residency slots in underserved communities. The new
20 primary care residency slots must be in 1 of the following
21 specialties: family medicine, general internal medicine, general
22 pediatrics, general OB-GYN, psychiatry, or general surgery.

23 (2) The department shall seek any necessary approvals from CMS
24 to allow the department to implement the program described in this
25 section.

26 (3) Assistance with repayment of medical education loans, loan
27 interest payments, or scholarships provided by the MiDocs
28 consortium shall be contingent upon a minimum 2-year commitment to
29 practice in an underserved community in this state post-residency



1 and an agreement to forego any sub-specialty training for at least
2 2 years post-residency with the exception of a child and adolescent
3 psychiatry fellowship that must be integrated with a psychiatry
4 residency training program in a MiDocs consortium affiliated
5 institution.

6 (4) The MiDocs consortium shall work with the department to
7 integrate the Michigan inpatient psychiatric admissions discussion
8 (MIPAD) recommendations and, when possible, prioritize training
9 opportunities in state psychiatric hospitals and community mental
10 health organizations.

11 (5) The department shall maintain the MiDocs consortium
12 initiative advisory council to help support implementation of the
13 program described in this section, and to provide oversight. The
14 advisory council must be composed of the MiDocs consortium, the
15 Michigan Area Health Education Centers, the Michigan Primary Care
16 Association, the Michigan Center for Rural Health, the Michigan
17 Academy of Family Physicians, and any other appointees designated
18 by the department.

19 (6) Not later than September 1 of the current fiscal year, the
20 MiDocs consortium shall submit a report to the standard report
21 recipients that includes all of the following information:

22 (a) Audited financial statement of per-resident costs.

23 (b) Education and clinical quality data.

24 (c) Roster of trainees, including areas of specialty and
25 locations of training.

26 (d) Medicaid revenue by training site.

27 (7) The department shall monitor outcome and performance
28 measures for this program, including, but not limited to, the
29 following:



1 (a) Increasing this state's ability to recruit, train, and
2 retain primary care physicians and other select specialty
3 physicians in underserved communities.

4 (b) Maximizing training opportunities with community health
5 centers, rural critical access hospitals, solo or group private
6 practice physician practices, schools, and other community-based
7 clinics, in addition to the required training through rotations at
8 inpatient hospitals.

9 (c) Increasing the number of residency slots for family
10 medicine, general internal medicine, general pediatrics, general
11 OB-GYN, psychiatry, and general surgery.

12 (8) Unexpended and unencumbered funds up to a maximum
13 \$6,400,000.00 in general fund/general purpose revenue plus any
14 contributions from public entities, up to \$5,000,000.00, and any
15 associated federal match remaining in accounts appropriated in part
16 1 for hospital services and therapy are designated as work project
17 appropriations, and any unencumbered or unallotted funds must not
18 lapse at the end of the fiscal year and must be available for
19 expenditures for the MiDocs consortium to create new primary care
20 residency slots in underserved communities under this section until
21 the work project has been completed. All of the following are in
22 compliance with section 451a of the management and budget act, 1984
23 PA 431, MCL 18.1451a:

24 (a) The purpose of the work project is to fund the cost of the
25 MiDocs consortium to create new primary care residency slots in
26 underserved communities.

27 (b) The work project will be accomplished by contracting with
28 the MiDocs consortium to oversee the creation of new primary care
29 residency slots.



1 (c) The total estimated completion cost of the work project is
2 \$20,200,000.00.

3 (d) The tentative completion date for the work project is
4 September 30, 2031.

5 Sec. 1872. From the funds appropriated in part 1 for personal
6 care services, the department shall maintain the monthly Medicaid
7 personal care supplement paid to adult foster care facilities and
8 homes for the aged that provide personal care services to Medicaid
9 recipients in place during the previous fiscal year.

10 Sec. 1874. The department shall ensure, in counties where PACE
11 services are available, that PACE is included as an option in all
12 options counseling and enrollment brokering for aging services and
13 managed care programs, including, but not limited to, Area Agencies
14 on Aging, centers for independent living, and the MiChoice home and
15 community-based waiver. The department must include approved
16 marketing and discussion materials for options counseling.

17 Sec. 1879. Not later than May 15 of the current fiscal year,
18 the department shall submit to the standard report recipients a
19 report with Medicaid pharmaceutical information. The report shall
20 include, for the previous fiscal year, the total Medicaid
21 pharmaceutical costs and the total Medicaid pharmaceutical rebates.
22 The report must categorize the total Medicaid pharmaceutical costs
23 and total Medicaid pharmaceutical rebates recognized by the
24 contracted health plans and the department. In addition, the report
25 must also include all of the following information:

26 (a) The total estimated pharmaceutical benefit expenses
27 incurred by contracted health plans from the previous fiscal year
28 and through the first 2 quarters of the current fiscal year.

29 (b) The total estimated pharmaceutical benefit expenses



1 included in approved initial rates for contracted health plans from
2 the previous fiscal year and total estimated pharmaceutical benefit
3 expenses included in approved initial rates for contracted health
4 plans for the first 2 quarters of the current fiscal year.

5 (c) The total Medicaid pharmaceutical rebates received by the
6 department in the previous fiscal year and the single preferred
7 drug list supplemental rebates invoices in the previous fiscal
8 year.

9 (d) Information as to whether the average benefit expense for
10 the composite average across all rate cells and service categories
11 included in capitation rates, based on actual enrollment and
12 anticipated recoveries, for the previous fiscal year and through
13 the first 2 quarters of the current fiscal year exceeded the
14 reported contracted health plan's experience, adjusted for
15 completion over the same reporting periods.

16 (e) The following information related to the current Medicaid
17 pharmacy carve-out of pharmaceutical products as provided for in
18 section 109h of the social welfare act, 1939 PA 280, MCL 400.109h:

19 (i) The number of prescriptions paid by the department during
20 the previous fiscal year.

21 (ii) The total amount of expenditures for prescriptions paid by
22 the department during the previous fiscal year.

23 (iii) The number of and total expenditures for prescriptions
24 paid by the department for generic equivalents during the previous
25 fiscal year.

26 Sec. 1880. The department shall align all pharmacy-related
27 policies with the United States Food and Drug Administration
28 quality and clinical standards. Any single preferred drug list
29 utilization management criteria will be established in consultation



1 with the Medicaid health plans and the Michigan pharmacy and
2 therapeutics committee described in section 9705 of the public
3 health code, 1978 PA 368, MCL 333.9705, with consideration given to
4 applicable United States Food and Drug Administration dosing
5 guidelines, subsequent evidence-based literature or studies, and
6 current treatment guidelines.

7 Sec. 1888. The department shall establish contract performance
8 standards associated with the capitation withhold provisions for
9 Medicaid health plans at least 3 months before the implementation
10 of those standards. The determination of whether performance
11 standards have been met must be based primarily on recognized
12 concepts such as 1-year continuous enrollment and the health care
13 effectiveness data and information set, HEDIS, audited data.

14 Sec. 1889. All quality assurance assessment program revenue
15 collected under section 20161 of the public health code, 1978 PA
16 368, MCL 333.20161, must only be expended on services provided
17 under the Healthy Michigan plan, under 2013 PA 107, or the state
18 Medicaid program, under Title XIX and Title XXI.

19
20 **INFORMATION TECHNOLOGY**

21 Sec. 1901. (1) The department shall submit a report on a
22 semiannual basis to the standard report recipients that lists the
23 projects approved in the previous 6 months and provides the purpose
24 for approving each project including any federal, state, court, or
25 legislative requirement for each project.

26 (2) Once an award for an expansion of information technology
27 is made, the department shall submit a report to the standard
28 report recipients that provides the projected cost of the expansion
29 broken down by use and type of expense.



1 Sec. 1902. (1) From the funds appropriated in part 1 for
2 comprehensive child welfare information system, the department
3 shall submit a report not later than March 1 to the standard report
4 recipients. The report must include, but is not limited to, the
5 following:

6 (a) The total expenditures by fiscal year, from all sources,
7 on the development of the comprehensive child welfare information
8 system.

9 (b) The expenditure plan for the subsequent fiscal year for
10 the development, implementation, and maintenance of the
11 comprehensive child welfare information system.

12 (c) The details on upgrades, remediation of user-reported
13 issues, and other modifications to currently implemented modules of
14 the comprehensive child welfare information system that occurred
15 during the current fiscal year and are planned for the subsequent
16 fiscal year.

17 (d) The current timeline for the full implementation of the
18 comprehensive child welfare information system.

19 (2) The department shall continue to provide the report
20 described in subsection (1) after the implementation of the
21 comprehensive child welfare information system is complete and
22 operational.

23 Sec. 1903. (1) Not later than November 1 of the current fiscal
24 year, the department shall submit a report to the standard report
25 recipients that describes the status of an implementation plan
26 regarding the appropriation in part 1 to modernize the MiSACWIS.
27 The report must include, but is not limited to, an update on the
28 status of the settlement and efforts to bring the system in
29 compliance with the settlement and other federal guidelines set



1 forth by the United States Department of Health and Human Services
2 Administration for Children and Families.

3 (2) Not later than July 1 of the current fiscal year, the
4 department shall submit to the standard report recipients a report
5 on the department's efforts and recommendations to develop and
6 implement a simpler and more streamlined process for the annual
7 renewal of the licenses for family foster care homes, and the
8 development of a simpler and more efficient version of the
9 application form for renewal of the licenses for family foster care
10 homes.

11 (3) From the funds appropriated in part 1 for Michigan
12 statewide automated child welfare information system, the
13 department shall submit a report by not later than March 1 to the
14 standard report recipients. The report must include, but is not
15 limited to, the following:

16 (a) The current timeline for the phaseout of MiSACWIS and
17 MiSACWIS's replacement by the comprehensive child welfare
18 information system.

19 (b) Expenditures, from all funding sources, for maintenance,
20 upgrades, and remediation of user-reported issues in the previous
21 fiscal year.

22 (c) Any cost savings realized by decommissioning MiSACWIS.
23 Sec. 1906. From the funds appropriated in part 1 for
24 information technology services and projects, the department shall
25 allocate \$1,750,000.00 general fund/general purpose revenue, and
26 all associated federal matching revenue, to a public and private
27 nonprofit collaboration that is designated as this state's
28 statewide health information exchange by cooperative agreement, to
29 implement health information technology strategies for health



1 information exchange development, data management, and population
2 health at a statewide level.

3 Sec. 1909. (1) From the funds appropriated in part 1 for child
4 support automation, the department shall only encumber or expend
5 funds for the operation, maintenance, and improvements of the
6 Michigan child support enforcement system.

7 (2) From the funds appropriated in part 1 for bridges
8 information system, the department shall only encumber or expend
9 funds for the operation, maintenance, and improvements of Bridges
10 and MIBridges.

11 (3) From the funds appropriated in part 1 for Michigan
12 Medicaid information system, the department shall only encumber or
13 expend funds for the operation, maintenance, and improvements of
14 the community health automated Medicaid processing system.

15 (4) From the funds appropriated in part 1 for Michigan
16 statewide automated child welfare information system, the
17 department shall only encumber or expend funds for the operation,
18 maintenance, and improvements of MiSACWIS.

19 (5) From the funds appropriated in part 1 for comprehensive
20 child welfare information system, the department shall only
21 encumber or expend funds for the operation, maintenance, and
22 improvements to the comprehensive child welfare information system.

23 (6) From the funds appropriated in part 1 for comprehensive
24 child welfare information system, the department shall continue
25 development of a new information system to replace MiSACWIS
26 consistent with the plan provided by the department to the United
27 States District Court for Eastern District of Michigan as a part of
28 the settlement. The development of the comprehensive child welfare
29 information system must adhere to department of technology,



management, and budget and information technology investment fund (ITIF) policies and practices, including use of the state unified information technology environment methodology and agile development. The project team shall also participate in and comply with the enterprise portfolio management office process and product quality assurance. To ensure full transparency, the project must be included in the ITIF portfolio for executive, legislative, and external reporting purposes. As a component of the ITIF portfolio, the project is subject to governance and oversight by the information technology investment management board.

ONE-TIME APPROPRIATIONS

Sec. 1930. (1) From the funds appropriated in part 1 for opioid response activities, the department shall allocate \$75,500,000.00 from the Michigan opioid healing and recovery fund created under section 3 of the Michigan trust fund act, 2000 PA 489, MCL 12.253, to programs and services to address the opioid crisis in a manner consistent with the opioid judgment, settlement, or compromise of claims pertaining to violations, or alleged violations, of law related to the manufacture, marketing, distribution, dispensing, or sale of opioids. The funds must be allocated as follows:

(a) \$4,000,000.00 for public health awareness and education campaigns.

(b) \$8,000,000.00 for evidence-based treatment programming, as follows:

(i) \$6,000,000.00 for workforce development programming.

(ii) \$2,000,000.00 for access to treatment in jails.

(c) \$41,000,000.00 for recovery investments, as follows:



1 (i) \$10,000,000.00 as competitive grants for recovery community
2 organizations.

3 (ii) \$17,000,000.00 for recovery and permanent housing
4 developments.

5 (iii) \$4,000,000.00 to support transportation for individuals in
6 recovery.

7 (iv) \$10,000,000.00 for job training.

8 (d) \$5,000,000.00 for wraparound services to address social
9 determinants impacting health, education, and economic stability.

10 (e) \$4,500,000.00 to support families impacted by the opioid
11 epidemic, as follows:

12 (i) \$1,500,000.00 for prenatal and perinatal care coordination
13 and social supports.

14 (ii) \$1,500,000.00 for family-based treatment and recovery
15 programs.

16 (iii) \$1,500,000.00 for evidence-based home visiting programs.

17 (f) \$1,000,000.00 to support local governments.

18 (g) \$2,000,000.00 for law enforcement training.

19 (h) \$10,000,000.00 for diversion programs.

20 (2) If any allocations remain after the completion of the
21 projects listed in subsection (1)(a) to (h), the department may
22 expend remaining funds for additional opioid response activities
23 that are consistent with the purposes outlined in this section.

24 Sec. 1931. Unexpended and unencumbered funds appropriated in
25 part 1 for nursing facility staffing initiative are designated as
26 work project appropriations. Any unencumbered or unallotted funds
27 do not lapse at the end of the fiscal year and are available for
28 projects under this section until the work project has been
29 completed. The following is in compliance with section 451a of the



1 management and budget act, 1984 PA 431, MCL 18.1451a:

2 (a) The purpose of the work project is to address chronic
3 workforce shortages in nursing homes.

4 (b) The work project will be accomplished using resources or
5 contracts with service providers, or both.

6 (c) The total estimated completion cost of the work project is
7 \$10,000,000.00.

8 (d) The tentative completion date for the work project is
9 September 30, 2031.

10 Sec. 1932. Unexpended and unencumbered funds appropriated as
11 one-time in part 1 for health services administration are
12 designated as work project appropriations. Any unencumbered or
13 unallotted funds do not lapse at the end of the fiscal year and are
14 available for projects under this section until the work project
15 has been completed. The following is in compliance with section
16 451a of the management and budget act, 1984 PA 431, MCL 18.1451a:

17 (a) The purpose of the work project is to support the home
18 help caregiver council and related activities, fulfilling the
19 requirements of the home help caregiver council act, 2024 PA 144,
20 MCL 400.801 to 400.805.

21 (b) The work project will be accomplished using state
22 resources or contracts with service providers or other entities.

23 (c) The total estimated completion cost of the work project is
24 \$4,000,000.00.

25 (d) The tentative completion date for the work project is
26 September 30, 2031.

27 Sec. 1934. From the funds appropriated in part 1 for uterine
28 fibroid study, the department shall allocate \$250,000.00 to
29 commission a study to evaluate options to expand services for the



1 detection and treatment of uterine fibroids, increase awareness of
2 services for the detection and treatment of uterine fibroids, and
3 increase access to services for the detection and treatment of
4 uterine fibroids. The department shall provide the study
5 commissioned under this section to the standard report recipients
6 not later than 30 days after the department receives the study.

7 Sec. 1936. From the funds appropriated in part 1 for Michigan
8 agricultural surplus system, the department shall allocate
9 \$12,045,000.00 for procuring and distributing the Michigan
10 agricultural surplus system to distribute surplus produce to low-
11 income residents of this state.

12 Sec. 1938. Not later than June 1 of the current fiscal year,
13 the department shall submit a report to the standard report
14 recipients on outcomes of the funds provided in part 1 to the
15 Michigan Clinical Consultation and Care program (MC3). The outcomes
16 reported must include, but are not limited to, the number of same-
17 day telephone consultations with primary care providers and the
18 number of local resource recommendations made to primary care
19 providers who are providing medical care to patients who need
20 behavioral health services.

21 Sec. 1940. From the funds appropriated in part 1 for Michigan
22 clinical consultation and care, the department shall allocate at
23 least \$325,000.00 to address needs in a city in which a declaration
24 of emergency was issued because of drinking water contamination.

25 Sec. 1942. From the funds appropriated in part 1 for local
26 office staff travel, the department shall allocate up to
27 \$100,000.00 annually toward reimbursing the out-of-pocket costs of
28 county board members and county department directors to attend
29 statewide meetings of the Michigan County Social Services



1 Association.

2 Sec. 1956. (1) From the funds appropriated in part 1 for
3 permanent supportive housing, the department shall allocate
4 \$5,000,000.00 to expand supportive housing services. Organizations
5 that received funding under section 1983 of article 6 of 2023 PA
6 166, section 701 of article 16 of 2024 PA 121, or section 1956 of
7 article 6 of 2025 PA 22 are eligible to apply for and receive
8 funding under this section. The funds must be used for services to
9 households living in supportive housing who need additional
10 services to maintain stability and currently homeless households
11 moving into supportive housing.

12 (2) From the funds appropriated in this section, no more than
13 8% may be allocated as grants to organizations providing permanent
14 supportive housing for capacity building necessary to develop and
15 sustain high-quality service delivery and to build administrative
16 capacity to seek Medicaid reimbursement for eligible services.

17 (3) From the funds appropriated in this section, at least 1%
18 must be allocated for technical assistance and capacity building to
19 support organizations providing permanent supportive housing
20 services.

21 (4) Funds appropriated in part 1 for permanent supportive
22 housing are considered work project funds, do not lapse at the
23 close of the fiscal year, and are available for expenditures for
24 projects under this section until the projects have been completed.
25 The following are in compliance with section 451a of the management
26 and budget act, 1984 PA 431, MCL 18.1451a:

27 (a) The purpose of the work project is to provide funding for
28 grants to eligible entities to provide permanent supportive housing
29 services for eligible households.



1 (b) The work project will be accomplished through partnerships
2 with community-based agencies that provide supportive housing
3 services, the Michigan state housing development authority, and
4 local governments.

5 (c) The total estimated cost of the work project is
6 \$5,000,000.00.

7 (d) The tentative completion date for the work project is
8 September 30, 2031.

9 Sec. 1965. (1) From the funds appropriated in part 1 for water
10 affordability, the department shall allocate \$10,000,000.00 as
11 grants to qualified providers to assist eligible residents who have
12 a financial burden, have accumulated a balance on their water
13 utility bill, have had their water service shut off, and/or are at
14 risk of having their water service shut off. Eligible expenditures
15 from these funds must be income-based and must include all of the
16 following:

17 (a) Restoring residential water service.

18 (b) Paying down water bills currently in arrears.

19 (c) Supporting reasonable water affordability plans that are
20 based on an individual's ability to pay, including capped payments
21 based on household income to prevent accumulating a balance on
22 future water bills and funding to qualified providers to cover the
23 remaining cost of service.

24 (d) Protecting participating residents from water shutoffs.

25 (2) To be considered a qualified provider under this section,
26 the provider must be 1 of the following:

27 (a) A community water system.

28 (b) A community action agency.

29 (c) A nonprofit, community-based organization organized under



1 the laws of this state that is exempt from federal income tax under
2 section 501(c)(3) of the internal revenue code of 1986, 26 USC 501,
3 with a history of doing utility assistance work.

4 (3) Qualified providers receiving grants under this section
5 may spend not more than 3% of the total grant award for
6 administrative services related to the implementation of this
7 section.

8 (4) Qualified providers receiving grants under this section
9 shall report to the department by September 30 of the current
10 fiscal year on outcomes and performance measures for the program,
11 including, but not limited to, all of the following:

12 (a) The total grant award received by the qualified provider.

13 (b) The percentage of the grant award that was used for
14 administrative costs.

15 (c) The total dollars spent broken down by type of assistance
16 provided.

17 (d) The number of individuals helped broken down by type of
18 assistance provided.

19 (e) The number of individual applicants denied assistance.

20 (5) Upon receipt of the information required under subsection
21 (4), the department shall compile and forward the report to the
22 standard report recipients.

23 (6) The unexpended funds appropriated in part 1 for water
24 affordability are designated as a work project appropriation, and
25 any unencumbered or unallotted funds shall not lapse at the end of
26 the fiscal year and shall be available for expenditures for
27 projects under this section until the projects have been completed.
28 The following is in compliance with section 451a of the management
29 and budget act, 1984 PA 431, MCL 18.1451a:



(a) The purpose of the project is to provide grants to qualified providers that assist eligible residents who have a financial burden, have accumulated a balance on their water utility bill, have had their water service shut off, and/or are at risk of having their water service shut off.

(b) The project will be accomplished through competitive grants to qualified providers.

(c) The total estimated cost of the project is \$10,000,000.00.

(d) The tentative completion date is September 30, 2031.